

The Soma-Field: Collected Works

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The Programme

This is a document about structure.

Not about feelings — though feelings are what the programme is ultimately for. Not about therapy — though therapy is one of the principal applications. Not about physics — though physics is where the mathematics comes from. It is about a single recurring observation: that the equations governing emotional dynamics are the same equations that govern quantum fields, and that this is not a metaphor.

When an identification like that is made precisely — when you can say not “this is *like* a wave” but “this *is* a wave in the technical sense, with the same propagator, the same energy function, the same topology, and therefore the same theorems” — a compressed body of work becomes possible. You are not building from scratch. You are navigating.

This document describes what was built by navigating, and why the pieces form a whole.

The Gap the Programme Addresses

Every large language model deployed today is a classical system. Its training is gradient descent. Its inference is deterministic or thermally noisy sampling. The architecture was designed to model the neocortex — pattern recognition, sequence prediction, error minimisation.

The complementary system — the limbic system, responsible for valuation, threat detection, arousal modulation, and the somatic state reinforcement that underlies trauma — had no formal mathematical treatment before this work. The clinical literature described it richly (Porges, van der Kolk, Levine). The neuroscience described its anatomy. Neither provided a model from which predictions could be derived and tested.

Simultaneously, the psychology of music had reached a similar ceiling. A 991-page handbook (Juslin and Sloboda, 2010) treated music-induced emotion almost entirely through Russell’s valence–arousal circumplex: a static two-dimensional map. The circumplex describes *where* a listener is, not *how* they move, what traps them, or what allows escape. No dynamical model of music-induced affect existed.

The programme fills both gaps with the same model, via the same method.

The Structure of the Argument

The argument has three movements and several extensions:

Paper	Movement	Contribution
<i>Mathematical</i> <i>Co-identification</i> (2026)	Method	Names and formalises the procedure

Paper	Movement	Contribution
<i>The Soma-Field</i> (2026)	Model	Applies it to emotional dynamics
<i>Quantum Soma and the Penrose Gap</i> (2026)	Empirical test	Confirms the central claim
<i>Field Notes from the Inside</i> (2026)	Lived case	Primary-source clinical grounding
<i>A Dynamical Field Model of Music-Induced Affect</i> (2026)	Extension	Demonstrates domain generality
<i>The Tensor</i> (2026)	Extension	Applies the framework to abstract film

The popular account (*A Voyage into Trauma*, 2026) provides the same argument in accessible form, for readers without a physics background.

The Method: Mathematical Co-identification

What It Is

The history of mathematical science contains a recurring event. At a certain moment, a scientist recognises that the quantity they are studying is not *like* a quantity already understood in another domain — it *is* the same mathematical object, under a change of label. When this identification is made precisely, every theorem proved about the source object becomes available in the target domain immediately, without re-derivation.

This event has happened many times:

- Hopfield (1982) recognised that a neural network’s energy-minimisation dynamics are the same as a spin-glass Hamiltonian. Every result from statistical mechanics of spin glasses — ground states, phase transitions, capacity bounds — imported.
- Veneziano (1968) recognised that the Euler Beta function, a result in pure mathematics, described the scattering amplitudes of hadrons. String theory began.
- Black and Scholes (1973) recognised that an option pricing equation was the heat diffusion equation. Every analytical tool from thermodynamics imported.

The paper *Mathematical Co-identification: A Method for Structural Import Across Scientific Domains* (Johnson, 2026a) names this procedure, formalises it as a distinct scientific method with its own validity criteria and failure modes, and distinguishes it from analogy, metaphor, and modelling. The key distinction:

Analogy: A is *like* B in certain respects. Illuminating, not transferable.

Co-identification: A *is* B under relabelling. Every theorem about B is a theorem about A.

Why It Matters as Method

A co-identification can be wrong. The identification is only valid if the mathematical type matches: the same dimensionality, the same algebraic structure, the same boundary conditions, the same symmetry group. The paper provides a falsifiability protocol — a formal procedure for pre-registering an import claim and specifying what observation would disconfirm it.

This matters because the failure mode of co-identification is not sloppy reasoning — it is overly precise reasoning applied to the wrong type. The paper catalogues seven historical examples to distinguish the valid from the invalid pattern.

The Soma-Field Model is the worked example throughout. The iden-

tification was not discovered by reading physics textbooks and looking for something that felt similar. It was discovered by writing down the equations the emotional system was observed to satisfy and recognising the form.

The Model: The Soma-Field

Five Co-identifications

The Soma-Field Model (Johnson, 2026b) is built from five sequential co-identifications, each importing a body of mathematics from physics into emotional dynamics:

Co-identification 1: The Hopfield identification. The brain’s emotional attractor dynamics satisfy the same energy function as a Hopfield neural network. The energy function is:

$$H(\mathbf{e}) = -\frac{1}{2}\mathbf{e}^\top W \mathbf{e} - \mathbf{b}^\top \mathbf{e}$$

where $\mathbf{e} \in \mathbb{R}^N$ is the emotional state vector, W is the coupling matrix, and $\mathbf{b}$ is a bias vector encoding baseline arousal. The local minima of H are the named attractor states: regulated calm, fight, flight, freeze, flow, dissociation.

Co-identification 2: The QFT identification. The emotional field propagates as a quantum field. The conscious emotional percept is the one-dimensional impulse response — the Green’s function — of an eleven-dimensional coupling manifold. The same object that describes a massive particle in quantum field theory describes a conscious emotion: a pole in the propagator of the field.

$$G(\omega) = \frac{1}{\omega^2 - m^2 + i\epsilon}$$

This is not a metaphor. The threshold T at which a sub-perceptual field

fluctuation becomes a conscious emotional percept is the mass parameter m in the propagator. Below threshold: virtual. Above threshold: real.

Co-identification 3: The brane identification. The body and the nervous system are not the same manifold. The body is a 3-brane embedded in the 11-dimensional coupling manifold. Somatic pain states and the body schema are field modes on this brane, not on the bulk manifold. This is the formal statement of the somatic grounding of emotion.

Co-identification 4: The G_2 holonomy identification. The seven compactified extra dimensions of the coupling manifold are a G_2 manifold. The G_2 holonomy group is the one that gives rise to topological obstructions — loops through the moduli space that cannot be continuously contracted to a point. In emotional terms: trauma configurations from which smooth continuous change cannot escape. The topological barrier is not a metaphor for being stuck. It is a mathematical object with a winding number.

Co-identification 5: The renormalisation group identification. Developmental trajectory maps onto the renormalisation group flow. The age at which a traumatic modification was introduced corresponds to the energy scale at which the coupling constant was set. High-energy (early developmental) modifications are renormalisation-group relevant — they affect all subsequent scales. Low-energy (later life) modifications are irrelevant in the technical sense. This gives the formal account of why early trauma is not simply a more intense version of later trauma: it is a different class of object.

What the Model Predicts

From these five identifications, several predictions follow that are not derivable from any existing clinical model:

1. **Threshold crossings are phase transitions.** The transition from sub-perceptual to conscious emotion is a second-order phase transition in the field. This predicts hysteresis — it is easier to stay in a state than to enter it, and easier to stay out than to leave.

2. **Complex PTSD is a topological configuration.** The coupling matrix W for a CPTSD nervous system has a specific structure: a winding-number-protected attractor landscape in which the Fear basin is separated from the Awe basin by a barrier that low-noise classical gradient descent cannot cross. This is a prediction about matrix structure, not a description of symptoms.
3. **Autism Spectrum Condition modifies the threshold operator.** The threshold parameter T in the ASC nervous system has a different coupling to the field modes than in the neurotypical case — specifically, the threshold is non-uniform across sensory modalities, producing the characteristic pattern of simultaneous hypo- and hyper-sensitivity.
4. **ADHD modifies the effective temperature.** The stochastic term in the Langevin dynamics governing the ADHD nervous system has higher effective temperature T_{eff} . This is not a deficit of attention; it is a higher rate of escape from local minima — an advantage in landscapes where rapid sampling is valuable and a liability where sustained convergence is required.
5. **Quantum mechanisms are required for certain transitions.** For trauma configurations with topological barriers (non-zero winding number), low-noise classical gradient descent cannot reach the global minimum. A quantum mechanism is required. This is the prediction that QUANT-EXP-1 was designed to test.

The Empirical Test: QUANT-EXP-1

The Prediction

The soma-field model makes a specific, falsifiable claim: for a Hopfield landscape with a topological trauma barrier, low-noise classical Langevin dynamics starting from the Fear attractor cannot reach the Awe attractor. Quantum annealing — a physically realisable mechanism —

can.

This is not a claim about whether people should use quantum computers in therapy. It is a claim about reachability: that the mathematical structure of the barrier distinguishes the quantum and classical regimes in a measurable way.

The prediction was registered in the Zenodo v1 deposit of the Soma-Field paper (doi:10.5281/zenodo.20350515) before the experiment was run.

The Experiment

Quantum Soma and the Penrose Gap (Johnson, 2026c) reports QUANT-EXP-1: an exact 8-qubit statevector simulation on a 256-dimensional Hilbert space, implementing the Soma-Field Hopfield Hamiltonian with a transverse-field quantum annealing schedule.

The experimental design:

- **System:** 8-qubit Hopfield model encoding four emotional modes (Fear, Calm, Awe, Grief) plus sub-modes. Coupling matrix W set to produce a topological barrier between Fear and Awe.
- **Quantum dynamics:** Transverse-field annealing with schedule $H(s) = (1 - s)H_X + sH_{\text{problem}}$, $s \in [0, 1]$.
- **Classical baseline:** Overdamped Langevin dynamics at low temperature ($T_{\text{eff}} = 0.01$), same starting state, same landscape.
- **Primary outcome:** Peak Awe-dominant occupancy (quantum) versus success rate of cold-classical crossings.

Results

Results are presented against the pre-registered barrier ladder:

Barrier strength	Classical cold rate	Classical cold CI [95%]	Quantum peak
$W = -6$	0.000	[0.000, 0.019]	0.389
$W = -8$	0.000	[0.000, 0.019]	0.408
$W = -10$	0.000	[0.000, 0.019]	0.408

Barrier strength	Classical cold rate	Classical cold CI [95%]	Quantum peak
$W = -12$	0.000	[0.000, 0.019]	0.409
$W = -14$	0.000	[0.000, 0.019]	0.416

Bootstrap confidence intervals ($n = 200$ seeds) confirm that the classical cold success rate is bounded above by 1.9% at all tested barrier strengths. Quantum peak occupancy is stable at 0.389–0.416 across the full range.

Pre-registered hardening protocol — all checks passed:

- **Bootstrap** ($n = 200$): cold CI = [0.000, 0.019]; quantum peak 0.408–0.410. Intervals do not overlap at any barrier strength.
- **Control A** (start from Awe, barrier intact): classical 16/16 stay in Awe. PASS. Confirms that the barrier is directional: it blocks $\text{Fear} \rightarrow \text{Awe}$, not the reverse.
- **Control B** (barrier removed, $W[\text{Fear}, \text{Awe}] = +0.4$): classical 16/16 reach Awe. PASS. Confirms that the barrier, not the landscape geometry, is what blocks classical dynamics.
- **Spectral gap**: gap narrows monotonically with barrier strength (B8: 0.0095, B10: 0.0089, B12: 0.0085) and reaches its minimum at $s \approx 0.999$, confirming the tunnelling bottleneck is late in the anneal.

Verdict: The strong reachability claim stands. QUANT-EXP-1 is a PASS.

The Penrose Connection

The paper situates this result in the context of Penrose’s argument about non-computability and consciousness. The connection is not that consciousness requires quantum mechanics in general. The connection is more specific:

Penrose identified a *gap* between what classical computation can reach and what consciousness can do. The soma-field identifies a corresponding

topological gap in the emotional landscape between what classical gradient descent can reach and what a genuinely new state of the nervous system requires. QUANT-EXP-1 provides the computational demonstration that the gap exists and is crossable by a quantum mechanism.

The contribution is not to resolve Penrose’s claim about consciousness. It is to *instantiate* the gap in a concrete, testable, mathematical setting.

The Lived Case: Field Notes from the Inside

Field Notes from the Inside: A Patient-Constructed Model of Emotional Dynamics (Johnson, 2026d) performs a function that the formal papers cannot perform: it provides the primary-source clinical grounding.

The paper is written by the person who has Autism Spectrum Condition (Level 2), Attention Deficit Hyperactivity Disorder, and Complex Post-Traumatic Stress Disorder — and who also has a degree in physics. The model was not developed by observing patients. It was developed by having the conditions and finding the existing models inadequate.

The epistemological contribution of this paper is often undervalued. Every formal model of a human system is, in the end, derived from observation of that system. When the observer and the observed are the same entity, and that entity has the training to translate observation into formal mathematics, the resulting model has a different epistemic status from one derived by observation from the outside. The paper makes this explicit, situates it within the autoethnographic research tradition, and argues that the resulting model is *more* constrained, not less — because any prediction the model makes that does not match the primary observer’s experience is immediately falsified.

The formal content is a set of operator modifications for the three conditions:

- **ASC:** The threshold operator T is replaced by a modality-dependent operator T_k for each sensory channel k , with different

coupling strengths. The result is the characteristic simultaneous hypo- and hyper-sensitivity: some channels are below threshold where the neurotypical channel is above it, others are above where the neurotypical channel is below.

- **ADHD:** The Langevin noise term $\sqrt{2T_{\text{eff}}}\eta(t)$ has elevated T_{eff} . This is a quantitative modification, not a qualitative one. The system is not broken; it is sampling the energy landscape at higher temperature. The therapeutic implication is not to reduce the noise but to design the landscape so that high-temperature sampling is an advantage.
- **CPTSD:** The coupling matrix W has the topological structure described in §3.2: a winding-number-protected barrier between Fear and regulated states. The barrier was installed before language, before narrative memory, before the self that can explain the barrier was formed. The modification is not a layer added to a pre-existing structure. It is the structure.

Extensions: Music, Film, and the Domain Generality of the Model

Music-Induced Affect

A Dynamical Field Model of Music-Induced Affect: Beyond the Valence-Arousal Circumplex (Johnson, 2026e) applies the soma-field framework to a domain where the empirical literature is rich and the theoretical models are weak.

Juslin and Sloboda’s *Handbook of Music and Emotion* (2010) — 991 pages — contains the circumplex as its dominant quantitative framework. The circumplex is a static map. It describes where a listener is; it does not model how they move. The soma-field is the first dynamical model of music-induced affect.

The key predictions that the circumplex cannot make but the field model does:

1. **Phase transitions, not continuous shifts.** State changes in music-induced affect are not smooth movements across the circumplex. They are threshold crossings — sudden re-configurations of the attractor landscape. The field model predicts the conditions under which a transition occurs and the hysteresis that prevents immediate return.
2. **The adaptive function of high effective temperature.** In the ADHD nervous system (elevated T_{eff}), music that holds a neurotypical listener in a stable state may drive repeated transitions. This is not a bug; it is the same high sampling rate that characterises the ADHD cognitive profile. The model gives this a formal account.
3. **Basin depth asymmetry.** The freeze attractor basin is deeper than the regulated calm basin. This means it is harder to leave freeze than it is to leave calm — asymmetric with respect to the direction of transition. Music that successfully moves a listener from freeze to calm is doing qualitatively different work than music that moves a calm listener to a more activated state.

The paper also specifies a real-time instrument implementation: a MIDI controller array driving a Python field server at 50 Hz, with audio output via Ableton Live and 3D fractal visual output (Mandelbulb projection onto HoloGauze screen). The instrument is not described; it is specified formally, with pre-registered hypotheses and disconfirmation criteria.

The Tensor: An Abstract Film

The Tensor: An Abstract Film Definition (Johnson, 2026f) extends the framework to abstract film. A film is defined not by its pixels but by its **emotional score**: a vector-valued trajectory $\mathbf{e}^*(t)$ through the emotional field, parameterised by story-time $t \in [0, 1]$.

The rendering — the actual audiovisual output a viewer experiences —

is generated at runtime from this trajectory, the viewer’s own soma-field state, and a set of control parameters. In the limit where the viewer’s biofeedback is available, the film adapts to where the viewer is: the trajectory is not what the viewer experiences, but what the film proposes. The work is not the pixels. It is the map.

This is a significant claim about what an artwork is. A conventional film is fixed: the same sequence of frames for every viewer at every screening. The tensor film is a field: a mathematical object that takes the viewer’s state as input and produces an output adapted to it. The artistic statement is in the trajectory, not the realisation.

The paper does not describe how to make such a film. It defines the abstract structure that any realisation of such a film must instantiate — the way a musical score defines a symphony without being the performance.

The Argument as a Whole

The six papers form a single argument, and it can be stated in a paragraph:

The limbic system and its coupling to the body are governed by the same mathematical equations as a quantum field on a manifold with G_2 holonomy. This identification is not a metaphor; it is a co-identification in the technical sense, with all the theorems of each source domain importing into the target. Among those theorems is one that has clinical consequences: topological barriers in the emotional attractor landscape cannot be crossed by low-noise classical gradient descent. A quantum mechanism can cross them. This has been computationally confirmed (QUANT-EXP-1) against a pre-registered hardening protocol. The model correctly describes the structure of Autism Spectrum Condition, ADHD, and Complex PTSD as operator modifications, and general-

ises to music-induced affect and abstract film with no change to the underlying mathematics.

What makes this a research programme rather than a single paper is the **generativity**: the method (co-identification) produces results in any domain where an attractor landscape with topological structure can be identified. The soma-field is one instantiation. Music-induced affect is a second. Abstract film is a third. A fourth — currently in design — is **H-AL**: a holographic avatar whose body is a live Mandelbulb rendering of the emotional field state, projected at human scale through a hologauze screen and accompanied by a synthesised voice narrating the field in real time. The geometry of the fractal changes as the field changes; regulated calm and trauma produce visually distinct and mathematically characterisable forms. The same functor architecture (§A.4 of the main paper) supports this output with no changes to the field computation. Each of these instantiations generates falsifiable predictions from the same mathematical core.

What makes this a *novel* research programme is the **gap it fills**: no formal dynamical model of the limbic system existed before this work. The Hopfield framework gave the neocortex its formal model in 1982. The soma-field gives the limbic system its formal model in 2026. Together they constitute the first complete formal description of the two principal computational substrates of the vertebrate brain.

What Remains

The body of work described here is computationally complete. All pre-registered hardening checks have been executed. The claims that can be confirmed by simulation have been confirmed.

Three categories of work remain outside the scope of these papers:

Physical hardware confirmation. QUANT-EXP-1 uses exact statevector simulation. Running the same 8-qubit experiment on IBM

Quantum free-tier hardware would produce the sentence “confirmed on physical quantum hardware.” This is feasible, is the logical next step for any journal submission targeting a hardware-inclusive venue, and is not required to support any claim in the current corpus.

Peer review. The three published papers are currently archived on Zenodo as open preprints. Peer review in ranked journals is a separate track, ongoing. The relevant venues are: *Frontiers in Computational Neuroscience* (Hypothesis and Theory article type) for the soma-field paper; *Synthese* or *Philosophy of Science* for the co-identification paper; *Music Perception* or *Frontiers in Psychology* for the music-affect paper.

Empirical clinical application. The model makes predictions about specific clinical populations (ASC, ADHD, CPTSD) that require empirical testing outside the computational domain. This constitutes a research programme for clinical collaborators. The predictions are pre-specified in §3.2 of this document and in the relevant papers; they are not vague.

Physical substrate. The model is formally complete but physically silent on the tissue substrate in which the soma-field is instantiated in living organisms. A companion paper, *The Physical Substrate of the Soma-Field* (Johnson, 2026g), develops this layer across three converging research traditions: biotensegrity (Ingber, Levin) as the mechanical architecture through which the somatic wave propagates globally; fascial-interstitial continuity (Langevin, Schleip, Oschman) as the active signalling tissue and physical locus of attractor-depth encoding; and biofield physiology (Popp, Ho, McCraty, Rubik) as the candidate physical correlate of the field itself. The most clinically significant result is the quantitative correspondence between fascial stiffness and attractor depth: chronic fascial armoring measurable by shear-wave elastography is the physical implementation of the energy barriers that QUANT-EXP-1 shows to be quantum-resistant. Myofascial release is thus barrier *lowering* — not barrier crossing — and therapist-client physiological entrainment is the physical mechanism of co-identification.

Data and Code Availability

All papers, simulation code, result tables, figures, and Lean 4 formal proofs are archived at the following Zenodo records (open access):

Paper	DOI
<i>The Soma-Field</i>	10.5281/zenodo.20350515
<i>Mathematical Co-identification</i>	10.5281/zenodo.20287981
<i>Quantum Soma and the Penrose Gap</i>	10.5281/zenodo.20351230

The unreviewed papers (*Field Notes from the Inside*, *Music-Induced Affect*, *The Tensor*, and this synthesis document) will be deposited on Zenodo as part of the next release of the research archive.

Part I: The Body Knows

A Voyage into Trauma

The Soma-Field Theory of Emotional Life

Alistair Johnson

2026

For everyone who was told their body was overreacting. It wasn't. It was solving the right problem.

Preface: The T's

This book began as a physics paper.

It ended as a map.

The paper was called the Soma-Field Model, and it was written in the language of physicists: Hamiltonians, propagators, coupling matrices, Wick rotations. It was precise. It was, I think, correct. And it was almost entirely unreadable to the people it was most about.

This book is the translation.

There are four T's running through what follows, and they are not accidental. The first is **Trauma** — the subject. The second is **Threshold** — a specific parameter in the model, written T , that marks the boundary between what becomes conscious and what stays body. The third is **Time** — in particular, developmental time, the age at which a modification occurred, which turns out to matter enormously. The fourth is **Transformation** — not recovery, not a return, but a going forward into a wider landscape.

There is a fifth T that I notice only in retrospect: **Trance** — in two senses simultaneously. *A Voyage into Trance* is a 1995 Goa trance compilation by Paul Oakenfold; the title of this book is borrowed from it. A trance state is, in the language of the Soma-Field Model, a phase transition of the emotional field — a threshold crossing guided by sound and rhythm rather than threat. The trance state produced by extended rhythmic music at 140 BPM and the freeze response of a traumatised nervous system are not the same experience. They are governed by the same mathematics: the same threshold crossings, the same phase transitions, the same field dynamics. The T's of that album and the T's of this book are the same T's.

I should tell you what happened in 1968 before we go any further, because it is the reason this model exists and the reason I am the one

writing it. I was approximately eighteen months old. I developed septic arthritis in my left hip — a bacterial infection of the joint that, without treatment, destroys the socket entirely. It was treated, and I recovered. The treatment involved three months in hospital under what were then called “no-touch protocols”: the infection risk was judged to require isolation from physical contact. Three months is a long time at eighteen months. The body learns quickly at that age, and what mine learned — in the absence of any other available hypothesis — was that the world was a place where pain arrived without warning and comfort did not follow.

That is not a complaint. The clinicians saved the joint. But the learning happened, and it happened before language, before narrative memory, before the self that can tell the story was formed. The modification was not added to an existing structure. It was the structure.

This book is, among other things, an attempt to say that formally — to give that experience a mathematical description precise enough to make predictions, to inform clinical practice, and to explain why the goal is not to return to a self that never existed, but to build forward into one that can.

I should also tell you that I promised, years ago, to write a book about a campsite in the Glarus Alps of Switzerland — a valley with parabolic limestone walls that make sound oscillate like a natural resonator, adjacent to one of the world’s great tectonic structures. The book about the campsite and the book about the soma-field found each other. The Interlude between Part II and Part III is where they meet.

The physics is real. The equations correspond to something. And the voyage is the proof of it.

Alistair Johnson May 2026

How to Read This Book

This book is written for three kinds of reader, and you can navigate it differently depending on which you are.

If you are new to all of this — no physics background, no clinical background, just a body that has been confusing you — read Part I first. Chapters 1 through 3 are written for you. The mathematics in those chapters is kept to a minimum; the ideas are introduced through physical intuition and lived experience. When equations do appear, they are explained in words immediately. Nothing is assumed except curiosity.

If you are a mental health professional who wants to understand what this model adds to your existing framework — you can begin with the Part I overview and then move directly to Parts III and IV. The Going Deeper boxes throughout the book are written for you. The appendices contain the full mathematics as it appears in the academic paper.

If you are a physicist, mathematician, or computationalist who has arrived here by accident or curiosity — you will recognise the Hamiltonian formulation immediately. The novel content for you is in Chapters 6, 7, and Appendix A. The Lean 4 type sketches in Appendix B may be of particular interest; they are incomplete proofs, marked with `sorry` where the hard work remains, and they represent a research programme.

A note on boxes. Throughout the book you will find four types:

LEARNING OBJECTIVES — what this chapter sets out to establish, listed at the start.

AUTHOR’S NOTE — personal first-person sections. The research and the life it emerged from are not separate, and I have not pretended otherwise.

GOING DEEPER — technical sections with more mathematics or formal detail. They can be skipped without losing the main argument. They can also be read first if that is your

preference.

KEY TERMS — precise definitions for terms that carry specific meaning in this model. A full glossary is in Appendix D.

PART I: THE BODY KNOWS

Chapter 1: What the Body Remembers

"The body keeps the score."

– Bessel van der Kolk, 2014

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the body responds to safety and danger independently of what the conscious mind believes
 - What the freeze response is and why it exists
 - The difference between a body that is “overreacting” and a body that has learned accurately
 - Why the first step in understanding trauma is understanding that survival responses are not mistakes
-

1.1 The Waiting Room

Picture a waiting room. You are sitting in a chair, waiting for a routine appointment. Nothing unusual is happening. The lighting is fluorescent. There is a plant in the corner that needs watering. Someone across the room is reading a magazine with the particular focused patience of someone who is not, in fact, reading a magazine.

For most people in this waiting room, the body is doing nothing remarkable. Heart rate is steady. Breathing is even. The background hum of vigilance that keeps us alive is running at its ordinary level.

For some people, this waiting room is already an event. Heart rate has

been elevated since the appointment was booked. Breathing is shallower than usual. There is a low-frequency alertness — a readiness — that is using energy, keeping muscles slightly contracted, keeping the hearing slightly sharpened, keeping the eyes moving. The mind may know that this is a routine appointment. The body has reached a different conclusion and is acting on it.

This is not anxiety in the clinical sense, though it may be diagnosed as such. It is not a failure of rational thinking. It is a body doing exactly what it learned to do — and doing it accurately, given what it knows.

The question this book asks is: what does the body know, and how did it learn it, and what does it mean to change that learning?

1.2 What Trauma Is (and Is Not)

The word *trauma* is used in many ways. In this book, it has a specific meaning.

Trauma is a **permanent modification of the nervous system's prediction model** in response to an experience that exceeded the system's capacity to process and integrate at the time of the experience.

Notice what this definition does and does not say.

It does **not** say that trauma is a weakness. A bridge that bends under a load it was not designed for is not a weak bridge — it is a bridge responding appropriately to a force that exceeds its design specifications.

It does **not** say that trauma is a mental event. The modification happens at the level of the nervous system — in the way sensory signals are filtered, in the way the body prepares for action, in the way energy is distributed across physiological systems. These are physical processes.

It does **not** say that the traumatic event needs to be dramatic. A three-month absence of contact comfort at eighteen months of age is not a bomb going off. It is, by most conventional standards, a minor medical intervention. What matters is the match between the experience and the system's current capacity — and at eighteen months, the nervous sys-

tem has no framework whatsoever for “temporary separation for medical reasons.”

What trauma **is**: a successful adaptation. The nervous system encountered a situation it could not model, and it updated its model in the direction that maximised survival. If the world is a place where pain arrives without warning and comfort does not follow, then a nervous system set to high alert — always scanning, always slightly contracted, always ready — is a nervous system correctly calibrated to that world. The problem is not that the calibration is wrong. The problem is that the world has changed and the calibration has not.

AUTHOR’S NOTE: The Hospital, 1968

I do not remember it. I was too young for explicit memory to have formed.

What I have are the downstream signals: a body that has always treated routine medical environments as emergencies. A nervous system that identifies “caring professional approaching to help” and responds with the physiology of threat. A skeleton of responses so deeply embedded that they long predate any narrative I have been able to construct about them.

My developmental age at the time was approximately eighteen months. The modification that happened then was not added to an existing nervous system. It was the nervous system being formed. That is a distinction that will matter a great deal in Chapter 6.

For now: the body knows things that the mind never learned. This book is an attempt to write those things down in a language precise enough to work with.

1.3 The Polyvagal Ladder

In the 1990s, neuroscientist Stephen Porges developed what he called Polyvagal Theory — an account of the autonomic nervous system that begins not with the familiar fight-or-flight response but with the evolutionary history of the structures involved.

The key observation is this: the autonomic nervous system is not a single dial that runs from “calm” to “alarmed.” It is a hierarchy of three systems, each older than the one above it, each more primitive, each mobilised in sequence as the perceived threat increases.

VENTRAL VAGAL STATE	Social engagement branch	
Safe, connected, curious	Myelinated vagus nerve	
Window of Tolerance	Heart rate regulated	
_____	← most recently evolved	
SYMPATHETIC STATE	Mobilisation branch	
Alert, energised, defensive	Spinal cord pathway	
Fight or flight	Heart rate elevated	
_____	← older	
DORSAL VAGAL STATE	Immobilisation branch	
Shutdown, collapse, freeze	Unmyelinated vagus nerve	
Dissociation, numbing	Heart rate dropped	
_____	← most ancient	

Figure 1.1. The polyvagal hierarchy. Under conditions of safety, the most evolved system (ventral vagal) governs – enabling social connection, learning, and curiosity. As perceived threat increases, the sympathetic system activates, preparing the body for action. If the threat is overwhelming or escape is impossible, the oldest system (dorsal vagal) takes over: immobilisation, shutdown, disconnection. Trauma often involves the system being stuck at a lower rung long after the original threat has passed.

The critical word in that last sentence is *perceived*. The hierarchy responds to what the body detects as dangerous, not to what the thinking mind judges as dangerous. These are different processes. The thinking mind can be entirely convinced that there is no danger — and the body can, at exactly the same moment, be running the threat response at full intensity. Both are responding to real information. They are just reading different signals.

1.4 The Freeze Response

The freeze response is the least understood of the three states and, for many trauma survivors, the most characteristic.

It is not the absence of a response. It is a full physiological engagement — the body is doing something, and doing it with considerable energy. What it is doing is playing dead.

This is a deeply ancient response. In evolutionary terms, freezing when threatened by a predator is sometimes the optimal move: many predators respond to movement, and a motionless prey item may not register as prey at all. The freeze response in mammals also involves the release of endogenous opioids — nature's way of making the potential experience of being killed slightly less intolerable. This is why dissociation during overwhelming trauma is sometimes described as a kind of mercy.

For humans in modern environments, the freeze response is triggered not by literal predators but by anything the nervous system has learned to classify as equivalent. A raised voice. A medical environment. A particular combination of sensory signals that, at some point in the past, preceded something overwhelming. The body does not distinguish between the original context and the contemporary one. It responds to the signal, not the story.

The result is a person who, in the middle of a conversation or a clinic appointment or an otherwise ordinary moment, suddenly goes quiet and still and seems to be looking at something slightly to the left of wherever they are. They are not being difficult. They are not choosing not to

engage. They are playing dead because the body has concluded that this is the appropriate moment to play dead.

1.5 Why This Matters for Treatment

If trauma is a modification of a prediction model — an accurate learning from an overwhelming experience — then the therapeutic question is not *how do we fix the broken response* but *how do we update the prediction model with new information*.

This is a different question, and it has a different answer.

Fixing a broken response implies that the body is malfunctioning. Updating a prediction model implies that the body has been doing its job correctly, and that the job now requires new data.

The Soma-Field Model, which is the subject of this book, provides a mathematical framework for what “updating the prediction model” means precisely: what structures change, what the before and after states look like, and — critically — what kind of change is possible given when the original learning occurred.

That last point is where this model adds something that existing frameworks do not provide, and it is the subject of Chapter 6.

KEY TERMS

Soma — the body as experienced from the inside; the totality of interoceptive (internally sensed) signals.

Polyvagal hierarchy — the three-level autonomic nervous system described by Porges, ordered from most evolutionarily ancient (dorsal vagal) to most recently evolved (ventral vagal).

Window of Tolerance — the range of arousal within which the nervous system can function flexibly, process information, and engage socially. Above the window: hyperarousal

(sympathetic). Below the window: hypoarousal (dorsal vagal shutdown).

Freeze response — the immobilisation state of the dorsal vagal system; the body's oldest threat response, involving disconnection, stillness, and endogenous opioid release.

CHAPTER SUMMARY

Trauma is a modification of the nervous system's prediction model — a successful adaptation to overwhelming experience, not a failure or weakness. The polyvagal hierarchy describes how three evolutionary layers of the autonomic nervous system respond to perceived threat. The freeze response is the most ancient: immobilisation as survival strategy. For treatment to work, it must address the body's model, not argue with the thinking mind. The Soma-Field Model provides a mathematical language for this.

Chapter 2: A Field of Feeling

"The body is the unconscious mind."

– Candace Pert, 1997

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What a physical field is and why emotion behaves like one
 - Why emotions are body phenomena, not brain phenomena
 - What interoception is and why it is fundamental
 - The meaning of “the soma-field” as a technical term
-

2.1 What a Field Is

Imagine the gravitational field of the Earth.

You cannot see it. You cannot touch it. You cannot pick up a handful of gravity and examine it under a microscope. But it is real — as real as anything in physics — and you have felt it every moment of your existence. It is everywhere in space around the Earth. It has a *strength* at every point (stronger close to the Earth, weaker further away). It has a *direction* at every point (towards the centre of the Earth). And it exerts a force on everything that is in it.

A field, in physics, is precisely this: a quantity that has a value at every point in space. Temperature is a field. The wind is a field (a vector field

— direction and magnitude at every point). The electromagnetic field is a field. Quantum fields, which are the foundation of modern physics, are fields.

The key insight of the Soma-Field Model is this: **emotion is a field phenomenon**.

Not a metaphor. A precise claim about how emotional signals distribute themselves in the body, interact with each other, and evolve over time.

2.2 Emotions in the Body

Antonio Damasio, in his somatic marker hypothesis (1994), proposed that emotions are fundamentally body states: that what we call “emotion” is the brain’s representation of a pattern of physiological activation — heartrate, muscle tension, gut movement, hormonal state, skin conductance, respiratory rhythm. We do not feel an emotion and then notice body signals. The body signal *is* the emotion; what the brain does is read it.

This is deeply counterintuitive if you have spent your life inside a culture that treats the mind as the real thing and the body as its vehicle. But the neuroscience supports it consistently. Patients with damage to the parts of the brain that receive and integrate body signals do not make better decisions because they are freed from emotional interference — they make worse decisions, because they have lost access to the somatic markers that tell them which options feel safe and which feel dangerous.

The body is not an obstacle to clear thinking. It is the substrate of it.

Interoception is the technical name for the body’s ability to sense its own internal state: heartbeat, breath, gut sensation, muscle tone, the position of limbs, the temperature of organs. Interoceptive accuracy — how precisely a person can read their own body signals — varies widely between individuals and is significantly disrupted by trauma.

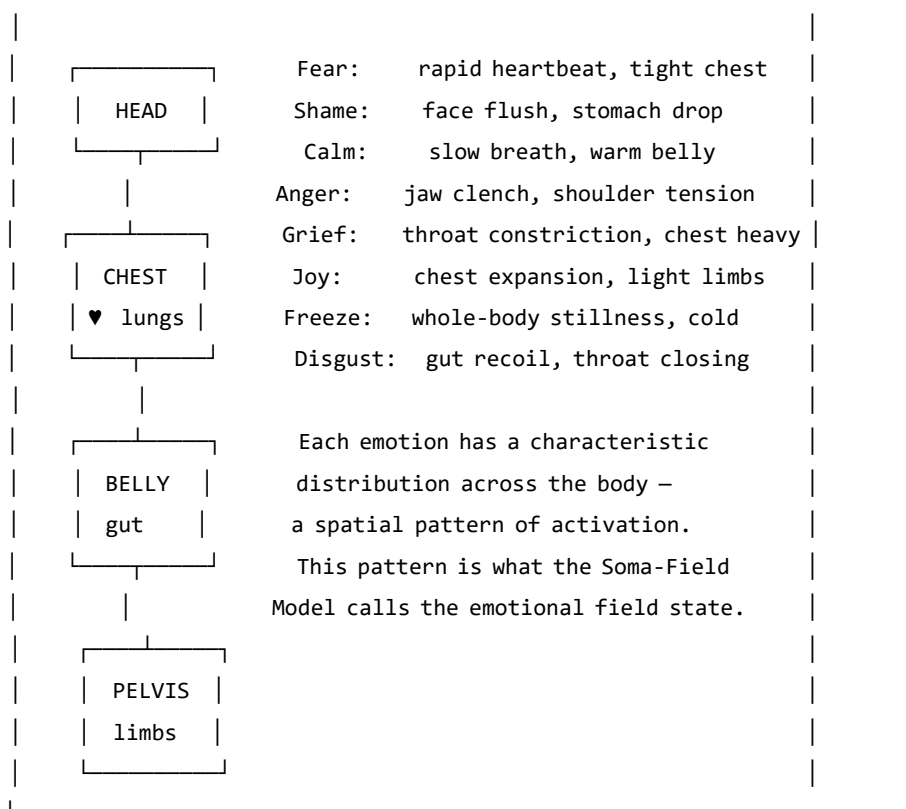


Figure 2.1. The body map of emotional activation. Emotions are not events in the body; they are distributed patterns of physiological arousal across the body. Researcher Nummenmaa et al. (2014) mapped these patterns by asking participants to colour in silhouettes where they felt each emotion. The patterns are consistent across cultures.

GOING DEEPER: The Foot That Isn't There

Pain is not in the foot. It is in the brain's model of the foot.

The clearest proof is phantom limb pain. When a limb is amputated, many patients continue to feel it — and feel it *hurting*. The foot is gone. The pain is real. It wakes people at night, responds to analgesics, and can be agonising for

years. What is in pain is the brain’s neural map of the foot, which persists in the cortex long after the tissue is gone.

Ramachandran’s solution was a mirror box. A mirror placed along the body’s midline creates a reflection of the intact hand where the absent hand should be. The patient watches the reflection move. The brain’s model updates: *the hand is there, the hand is moving, the hand is fine*. For many patients, the pain decreases or disappears. The model changed. The suffering reduced. Nothing in the body changed at all.

This is not a curiosity. It is the normal condition of all somatic experience. The brain does not receive raw body signals and display them. It maintains a continuous predictive model of the body and generates what you *feel* from that model. The felt body is the predicted body.

For the Soma-Field Model, this is load-bearing. The field $\mathbf{e}(t)$ is not a readout of the physical body. It is the nervous system’s model of the body. When the model is updated — by new sensory experience, somatic therapy, or the slow accumulation of safety — what is felt changes. Not because the body changed. Because the prediction changed.

Therapy does not fix the body. It updates the model.



2.3 The Soma-Field: A Technical Definition

In the Soma-Field Model, we represent the body’s emotional state as a vector of activation levels across a set of emotional dimensions. Call this vector $\mathbf{e}$:

$$\mathbf{e} = (e_1, e_2, \dots, e_n)$$

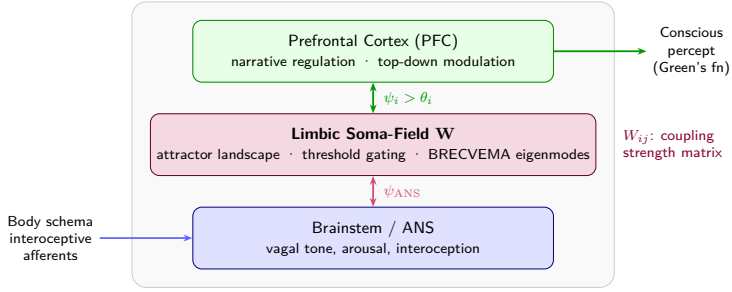


Figure 1: Figure 2.1. The body–brain coupling stack. Interoceptive signals from the body feed into the brainstem and autonomic nervous system, which couples bidirectionally to the limbic soma-field (coupling matrix $\mathbf{W}$). The field gates input to the prefrontal cortex via the threshold ; what crosses becomes conscious percept. *Author’s original figure.*

Each e_i is a real number representing the activation level of a somatic emotional mode at a given moment: the level of fear-readiness in the body, the level of grief-contraction, the level of social-engagement openness, and so on. The exact labelling of the modes is secondary to the structure; what matters is that there is a space of such states and a dynamics on that space.

The soma-field is this vector $\mathbf{e}$, evolving in time. It is not a single number (arousal level) or a pair of numbers (valence and arousal). It is a multi-dimensional state that captures the full texture of somatic experience at a given moment.

Three things are immediate from this definition:

1. **The field has a position:** the current emotional state is a point in an n -dimensional space.
2. **The field has dynamics:** it moves through this space over time.
3. **The field has a structure:** some positions are stable (attractors), and the dynamics drives the field toward them.

The next chapter is about that structure.

GOING DEEPER: Quantum Fields and Why They

Are Relevant

In quantum field theory (QFT), the fundamental objects are not particles but fields — wave-like disturbances propagating through space. Particles are what you see when a field vibrates at a high enough amplitude to be detected: an electron is a ripple in the electron field, a photon is a ripple in the electromagnetic field.

The soma-field is not a quantum field in the literal sense. Emotional dynamics are classical, not quantum. What the Soma-Field Model borrows from QFT is the *mathematical language*: the same equations that describe how quantum fields couple to each other turn out to describe how emotional modes couple to each other. This is not because emotion is quantum-mechanical. It is because coupling dynamics — the mathematics of how things that interact shape each other's behaviour — takes the same form wherever it appears.

This correspondence is the subject of Chapter 7. For now: the QFT connection is a mathematical tool, not a metaphysical claim.

KEY TERMS

Field — a quantity with a value at every point in space (or, in the soma-field context, at every point in the body's state space).

Interoception — the nervous system's process of sensing the internal state of the body.

Interoceptive accuracy — the precision with which a person can consciously read their own interoceptive signals.

Soma-field — the vector $\mathbf{e}$ of somatic activation levels across emotional modes; the state of the body's emotional field at a given moment.

State space — the set of all possible values of $\mathbf{e}$; the arena within which emotional dynamics occurs.

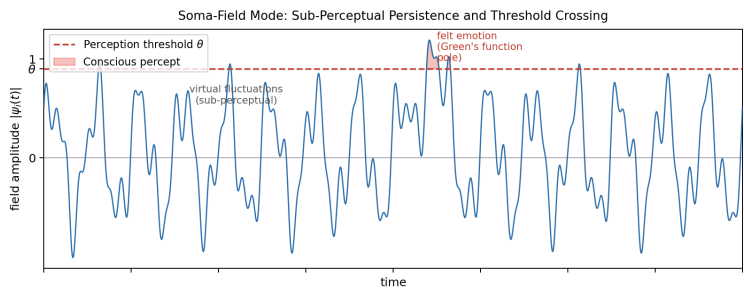


Figure 2: Figure 2.2. The soma-field oscillates continuously. Most of the time its modes remain below the perception threshold T (dashed line) — sub-threshold activity that drives physiology and behaviour invisibly. Only a sufficiently large excitation crosses T into felt experience. Interoceptive training and somatic therapy work, in part, by lowering T . *Author's original figure.*

Chapter 3: The Energy Landscape

"Nature does not create mountains and valleys at random.
They are shaped by the forces beneath them."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why some emotional states are stable and others are transient
- The meaning of “attractor” and “basin of attraction”
- What the Hamiltonian is and why it organises the model
- Why you keep returning to familiar emotional states even when you don’t want to

3.1 Hills and Valleys

Imagine placing a ball on a hilly landscape. If you place it at the bottom of a valley and give it a small push, it rolls away from where you pushed it — and then rolls back. The valley is stable. The bottom of the valley is an *attractor*: the ball is drawn toward it from nearby positions.

If you place the ball at the top of a hill and give it a small push, it rolls away from the hilltop — and keeps going. The hilltop is *unstable*. Small perturbations grow into large departures.

The same geometry applies to emotional states.

Some emotional states are at the bottom of valleys in the body’s landscape: they are stable, they are where the system tends to rest, and

perturbations that push the body away from them are followed by a return. Other states are at hilltops: they are unstable configurations that the system passes through on its way between valleys.

The crucial question — the question that distinguishes a regulated nervous system from a dysregulated one, and distinguishes one person’s landscape from another’s — is: where are the valleys? How deep are they? How wide? How many are there?

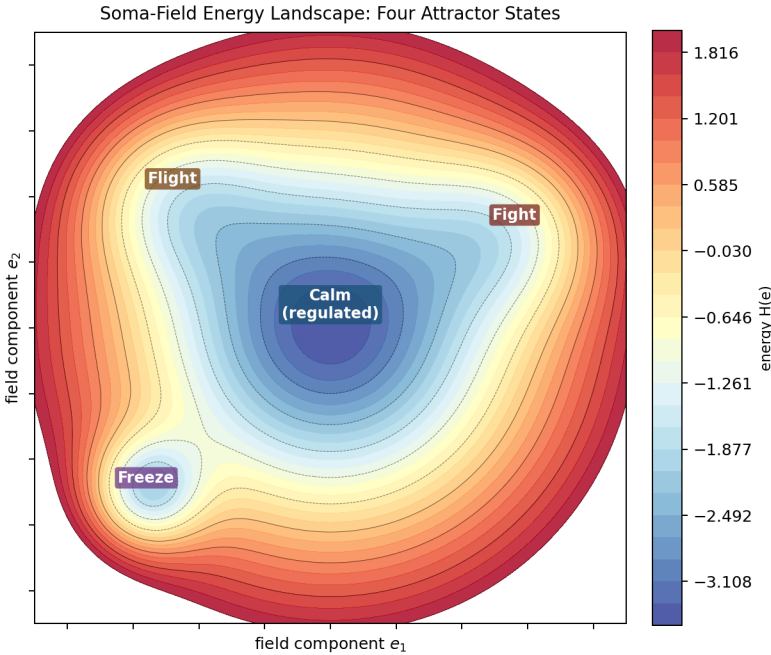


Figure 3: Figure 3.1. The emotional energy landscape (2D contour). Four attractor basins are visible: Calm (wide, deepest — the global minimum of a regulated nervous system), Freeze (narrow and very deep — easy to fall into, hard to leave), Fight and Flight (intermediate depth). The system rolls downhill to the nearest basin; the depth controls escape difficulty and the width controls resilience to perturbation. *Author’s original figure.*

3.2 Attractors and Basins

An **attractor** is a stable state — a bottom of a valley. A **basin of attraction** is the set of all points from which the system rolls toward a given attractor: the “catchment area” of the valley.

For a regulated nervous system, the primary attractor is some version of calm social engagement — the ventral vagal state of Polyvagal Theory. The basin is wide: a large range of perturbations (emotions, sensations, social situations) all resolve back to this resting state. The system is resilient.

For a trauma-modified nervous system, the landscape has changed. A second attractor — hypervigilance, alert-readiness, the sympathetic mobilisation state — may have become deep and wide. The calm attractor may still exist but its basin has narrowed: it takes very little to tip the system out of calm and into alertness. And a third attractor — the freeze state, the dorsal vagal shutdown — may be very deep indeed: once the system tips into it, escape requires a large input of energy.

This is not a metaphor for how trauma “feels.” It is a description of the actual dynamics of the system.

3.3 The Hamiltonian

The landscape has a name in physics: the **Hamiltonian**. Denoted H , it is a function that assigns an energy value to every possible state of the system.

For the soma-field, the Hamiltonian takes the form:

$$H(\mathbf{e}) = -\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j - \sum_i \theta_i e_i$$

Let us read this in plain English.

The first term, $-\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j$, captures the *interactions between emotional modes*. W_{ij} is the coupling between mode i and mode j — how strongly they influence each other. When fear is high, does shame rise

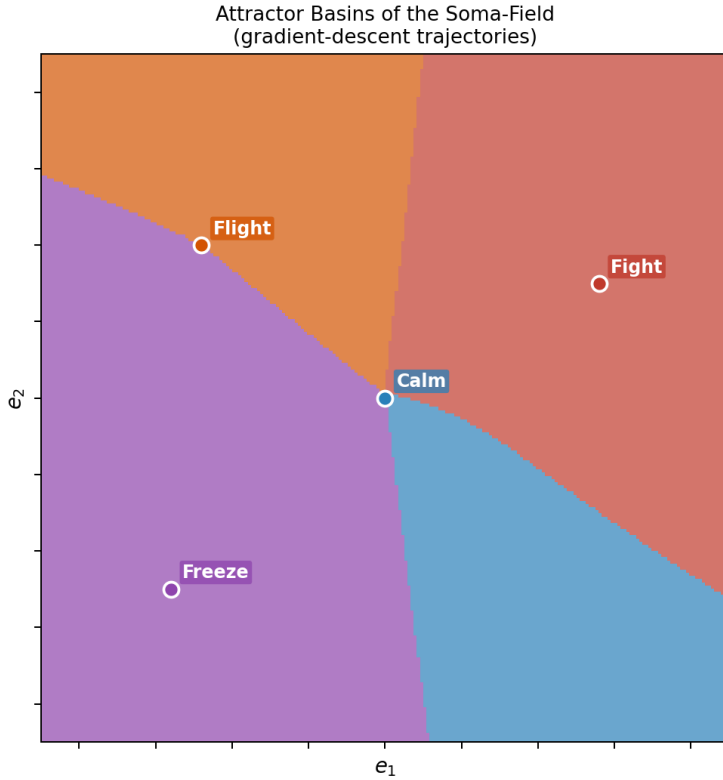


Figure 4: Figure 3.2. Basin of attraction map. Each point in state space is coloured by the attractor it flows to under gradient descent: blue = Calm, purple = Freeze, orange = Fight, green = Flight. The calm basin dominates a regulated landscape. Freeze occupies a small area but is disproportionately deep — a narrow funnel. The boundaries between basins are the separatrices: invisible thresholds in state space that determine which valley a given perturbation resolves to. *Author's original figure.*

with it? When calm is present, does anger fall? The matrix W encodes all of these mutual influences. The minus sign means that aligned coupling (modes reinforcing each other) lowers the energy — makes the state more stable.

The second term, $-\sum_i \theta_i e_i$, captures the *individual thresholds* of each mode. θ_i is the bias of mode i — how much the system tends toward or away from it in the absence of coupling. A mode with a large positive θ_i has a natural tendency toward high activation.

The dynamics — the way the field moves through state space over time — follows from this energy function. The field always moves *downhill*: toward lower values of H .

$$\dot{\mathbf{e}} = -\nabla H(\mathbf{e}) + \eta(t)$$

This equation says: the rate of change of the emotional state ($\dot{\mathbf{e}}$) equals the negative gradient of the energy (the direction of steepest descent on the landscape) plus a noise term $\eta(t)$ representing the small random fluctuations of physiological and environmental variation. The system is always rolling toward the nearest valley, with a small amount of noise that occasionally kicks it over a hill into a different basin.

The noise term has a deeper structure. The *level* of noise — how wide the fluctuations are — is set by the autonomic nervous system, specifically by heart rate variability (HRV): high coherence in the cardiac rhythm narrows the noise, stabilising the field; low HRV widens it. But there is a second, more predictive cardiac quantity: the **cardiac acceleration** $\dot{H}$ — the rate at which heart rate is *changing*. A rising heart rate predicts approach to a threshold; a falling heart rate predicts retreat from one. The current BPM tells you where you are. The acceleration of BPM tells you where you are going next.

GOING DEEPER: Gravity and the Heartbeat

Gravity, in SI units, is measured in metres per second squared (m/s^2) — it is an *acceleration*, not a speed. It tells you not

where a falling object is, but how fast its velocity is changing: where it will be next.

Cardiac acceleration — the rate of change of heart rate — has units beats/s². Same type, different physical dimension. And the same logical character: it tells you not what the BPM is, but where it is heading. $N+1$, not N .

In the soma-field, cardiac acceleration acts as a **landscape tilt**: it tips the energy function toward activation or rest before any emotional threshold is crossed. When the heart accelerates, the field is being pulled toward higher-energy states by a force it cannot see and cannot always attribute correctly. Some anxiety that feels emotionally caused is cardiac in origin — the field cannot distinguish the two from the inside. This is the somatic equivalence principle: you cannot tell, from your own experience, whether your emotional landscape tilted because something happened, or because your heart accelerated first.

Clinically: monitoring the *direction* of heart rate change, not just its level, gives earlier warning of threshold approach than any other non-invasive signal.

GOING DEEPER: Why Physicists Love the Hamiltonian

The Hamiltonian was introduced by William Rowan Hamilton in the 1830s as a way of rewriting Newton's equations in a more elegant form. What Hamilton discovered is that the trajectory of any physical system — the path it takes through its state space over time — can be derived entirely from a single scalar function H . You do not need to describe all the forces. You just need the energy landscape, and the dynamics follows.

In quantum mechanics, the Hamiltonian operator $\hat{H}$ plays the same role: it determines how a quantum state evolves over time through Schrödinger’s equation, $i\hbar \partial_t \psi = \hat{H} \psi$. The eigenvalues of $\hat{H}$ are the allowed energy levels.

In the Soma-Field Model, $H(\mathbf{e})$ is neither Newtonian nor quantum: it is the Hamiltonian of a classical stochastic system (a Langevin system), where the dynamics is gradient descent with noise. But the mathematical structure — a scalar energy function that determines everything else — is identical.

This is not a coincidence. It is because “a system has stable states to which it returns” is a very general physical principle, and the Hamiltonian is the most general way to formalise it.

3.4 The Coupling Matrix

The matrix W — the coupling matrix — is the central object of the model. It encodes the emotional architecture of a nervous system: which modes excite each other, which inhibit each other, how strongly, and in which direction.

For a neurotypical, regulated nervous system, W has a specific mathematical property: it is *symmetric*. $W_{ij} = W_{ji}$: the influence of mode i on mode j equals the influence of mode j on mode i . This symmetry is not incidental. It is what guarantees the existence of an energy function: if W is not symmetric, the dynamics cannot be written as gradient descent, and the system may not have stable fixed points at all. It may cycle indefinitely.

Trauma, in this model, is a modification of W that breaks this symmetry. A traumatised nervous system has couplings that do not balance: fear activates shame more strongly than shame activates fear; hypervigilance activates the freeze response more readily than the freeze response resolves back to hypervigilance. The asymmetric couplings create direc-

tional flows in the landscape — attractors that are easy to fall into and hard to climb out of.

This is the formal basis of the clinical observation that trauma often feels like a one-way ratchet.

KEY TERMS

Attractor — a stable state in the energy landscape; a valley that the field rolls toward from nearby positions.

Basin of attraction — the region of state space from which the system flows toward a given attractor.

Hamiltonian — the energy function $H(\mathbf{e})$ that organises the dynamics; the mathematical description of the landscape.

Coupling matrix W — the matrix encoding the interactions between emotional modes; shapes the landscape by determining which states lower the energy.

Threshold θ_i — the individual bias of emotional mode i ; shifts its natural resting level.

CHAPTER SUMMARY

Emotional states are points in a landscape shaped by the Hamiltonian H . Attractors are stable states (valley bottoms); basins of attraction are the regions from which the system rolls toward each attractor. The dynamics — gradient descent with noise — always moves toward lower energy. The coupling matrix W encodes the interactions that shape the landscape. Symmetry of W guarantees stable attractors; asymmetry (introduced by trauma) creates directional flows that are hard to reverse.

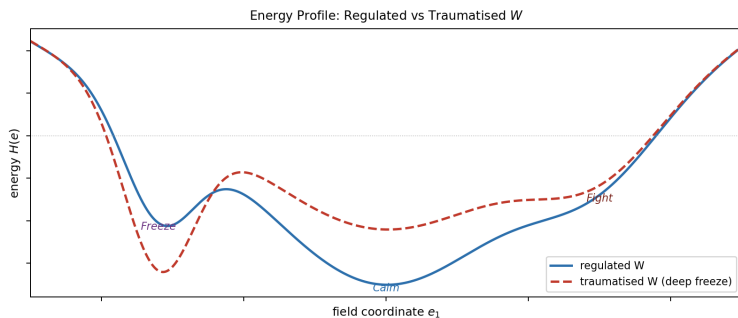


Figure 5: Figure 3.3. 1D energy cross-section along a principal axis of the landscape. The height of each barrier between basins determines transition probability: a deep Freeze well with a high approach barrier (right) requires substantial energy input to escape — corresponding clinically to a freeze response that does not self-resolve without intervention. Barrier asymmetry (left-to-right right-to-left) is the signature of trauma modification. *Author's original figure.*

PART II: HOW THE FIELD CHANGES

Chapter 4: The Weight on the Field

"The question is not why the behaviour persists,
but what it was optimised for."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What the C-PTSD operator is and how it modifies the field
 - Why hypervigilance is not an error but an optimisation
 - What “the landscape has changed” means precisely
 - The difference between a perturbation and a structural modification
-

4.1 The Modification

Complex PTSD (C-PTSD) is distinguished from single-incident PTSD by the presence of repeated, prolonged, or developmental trauma — particularly trauma that occurred in relationships on which the person depended for survival. The result is not a discrete memory that can be “processed” and resolved. It is a pervasive reorganisation of the emotional field architecture: a new landscape, not a scar on an old one.

In the Soma-Field Model, C-PTSD is represented as a modification of the coupling matrix:

$$W_{\text{C-PTSD}} = W_0 + \Delta W_{\text{trauma}}$$

where W_0 is the baseline coupling matrix and ΔW_{trauma} is the modification — an asymmetric additive term that reshapes the landscape. Crucially, ΔW_{trauma} is not symmetric: it introduces directional flows. Certain states become easy to fall into and hard to leave. Others become difficult to access from the modified landscape even though they exist.

This can be visualised as a landscape that has been tilted and deformed: new deep valleys in places that were not attractors before, old deep valleys raised, and the topology of connectivity between states changed.

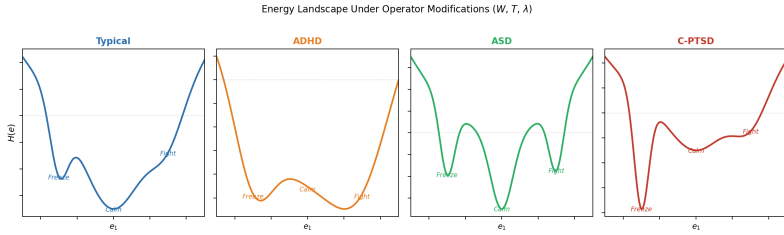


Figure 6: Figure 4.1. Four neurotype landscapes (1D cross-section). *Typical* (upper left): a deep wide Calm basin with accessible secondary states. *C-PTSD* (lower left): Calm shallowed and narrowed, Freeze dominant — the resting state shifts toward high-vigilance. *ADHD* (upper right): all basins flattened, low barriers, rapid transitions — high-temperature dynamics. *ASD* (lower right): narrow steep wells with high barriers between states — strong attractor stability, low noise tolerance, high cost of transitions. *Author’s original figure*.

4.2 Why Hypervigilance Is an Optimisation

A nervous system that has adapted to an environment of chronic threat has correctly learned that:

1. Danger is frequent and unpredictable.
2. The cost of missing a threat is very high.
3. The cost of false alarms is low (relative to the cost of missing a real threat).

Given these parameters, the optimal configuration is exactly what we see in C-PTSD: a bias toward high vigilance, a wide definition of “potential

threat,” a fast-responding sympathetic system, and a slow-to-settle calm state. The hypervigilance attractor is deep because a deep attractor is appropriate to the environment it was optimised for.

The modification is not an error. It is a correct solution to the wrong problem — where “the wrong problem” means the original environment, which no longer exists (or no longer exists in the same form).

This reframing is not merely philosophical. It changes the clinical question from “how do we extinguish the hypervigilance response” to “how do we update the landscape to incorporate evidence that the current environment is different.” These are very different operations, with very different implications for what kind of therapeutic intervention is useful.

4.3 Thresholds and Consciousness

There is a parameter in the model that has not yet been introduced, and it does a great deal of work. This is the **threshold** T — denoted with the capital T that recurs throughout this book.

The threshold is a level of field activation above which an emotional state becomes conscious experience — enters awareness as a felt emotion — rather than remaining as sub-threshold somatic activation. Below T , the field is active but not felt; the activation is present in the body, influencing behaviour and physiology, but not represented in consciousness.

This has immediate clinical consequences. A person with a very high threshold T may have a strongly activated soma-field — may be physiologically in a fear state, with all the somatic correlates — while experiencing nothing that they would call fear. The activation is real. The consciousness of it is absent. Somatic therapy, interoceptive training, and bodywork all operate, in part, by lowering T : bringing below-threshold somatic content into awareness.

A person with a very low threshold T experiences the opposite: everything is felt, amplified, present. This is associated with high interoceptive sensitivity, certain presentations of anxiety, and some forms of neurodivergence.

The threshold is where the physics and the clinical presentation most visibly connect.

AUTHOR’S NOTE: The Landscape I Inherited

There is a version of this chapter that is abstract: modifications to coupling matrices, reshaping of landscapes, asymmetric W . And then there is the version that is what it feels like to live in a modified landscape.

What it feels like is this: calm is always provisional. Not shallow, exactly — but unsecured. Like a surface that holds weight when you step carefully but gives way if you shift too quickly. Alert is never far away. And underneath alert, the freeze state is a gravity well that does not announce itself before you are already in it.

The modification in my case is not a perturbation on a pre-existing normal landscape. That would require a W_0 to perturb. The timeline does not allow for that. That is the subject of Chapter 6.

KEY TERMS

C-PTSD operator — the modification ΔW to the coupling matrix that reshapes the energy landscape; the mathematical representation of the effect of complex trauma.

Threshold T — the activation level above which a somatic state becomes conscious experience. The central parameter distinguishing felt emotion from sub-threshold somatic activation.

Hypervigilance attractor — the deep stability basin in the modified landscape corresponding to high-arousal, high-alert states.

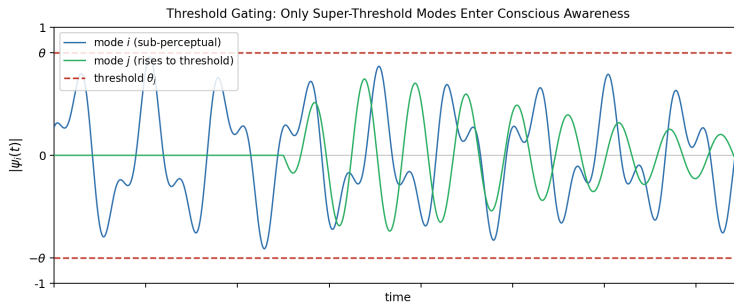


Figure 7: Figure 4.2. The perception threshold T . Mode i (grey) oscillates continuously but never crosses T — it is sub-perceptual, influencing behaviour and physiology without entering felt experience. Mode j (blue) rises through T and becomes a consciously felt emotion. The threshold is the key parameter that distinguishes somatic activation from emotional awareness; its value varies across individuals and can be modified by interoceptive practice, arousal level, and therapeutic work. *Author's original figure.*

Chapter 5: Memory Written in the Body

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- The difference between narrative memory and somatic memory
 - What the memory kernel is and what it does to the field dynamics
 - Why trauma memory persists — and why some trauma memories persist much longer
 - What therapeutic processing means in terms of the memory kernel
-

5.1 Two Kinds of Memory

When you remember a conversation from last week, you are using **episodic memory** — the explicit, narrative record of events that occurred at specific times and places. Episodic memory is context-dependent, verbally expressible, and subject to conscious recall and revision. It is stored primarily in the hippocampus.

When you flinch at a sound that resembles the sound that preceded something terrible, you are using **procedural** or **somatic memory** — a form of memory that is not stored as narrative but as pattern: as a configured readiness in the body to respond in a particular way to particular signals. Somatic memory is not verbally expressible (you cannot “tell the story” of a procedural response; you can only notice it happening). It does not require conscious recall — it is not a replay of an event but an embodied preparation. It is stored across the body: in muscle tone, in the brainstem, in the autonomic nervous system, in the way sensory signals are gated before they reach cortical processing.

Trauma creates primarily somatic memory. This is why it is not resolved

by talking about it. The body has stored information in a form that language does not reach.

5.2 The Memory Kernel

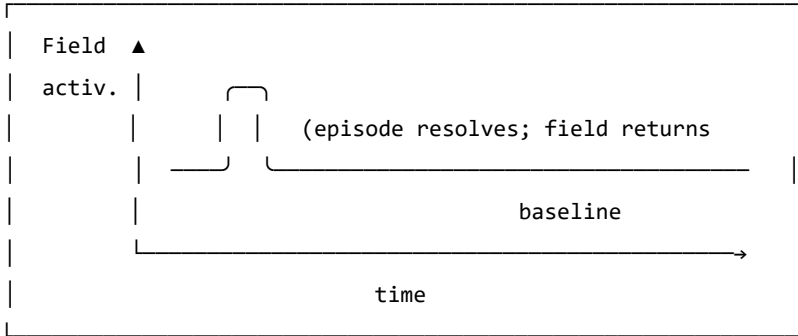
In the Soma-Field Model, the effect of past activation on present dynamics is captured by a **memory kernel** $K(\tau)$. This is a function that says: an activation of the field τ time units ago continues to influence the field now, with a weight proportional to $K(\tau)$.

For C-PTSD, the memory kernel takes the form:

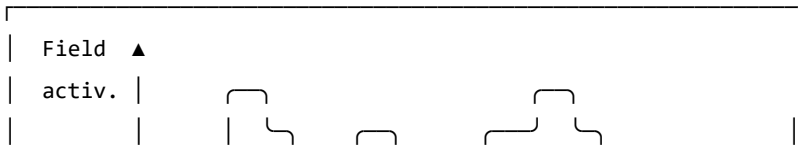
$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$$

This is a sum of decaying exponentials. Each term represents a distinct trauma trace: A_k is the amplitude (how strongly the trace affects the current field) and τ_k is the decay time (how long the trace persists before fading).

REGULATED: No significant memory kernel



C-PTSD: Significant memory kernel – traces persist



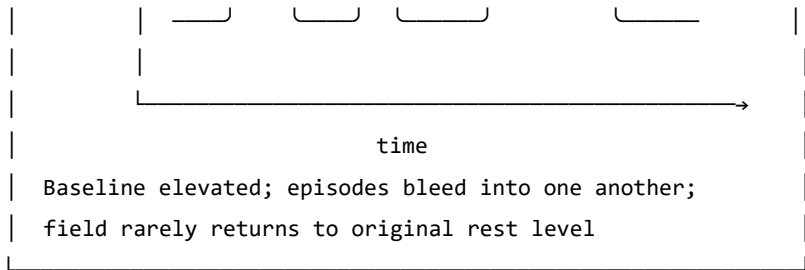


Figure 5.1. The effect of the trauma memory kernel on field dynamics. In a regular system (top), a field activation episode resolves and the field returns to a low resting level. In the C-PTSD-modified system (bottom), the memory kernel elevates the baseline between episodes, so that subsequent episodes begin from a higher activation. Over time, the field cycles at an elevated level without returning

5.3 Why Early Traces Persist

The decay time τ_k is central: it determines how long a trace remains active.

For trauma that occurs early in development — before language, before narrative memory capacity — the decay time tends to be much longer. There are two reasons.

First, **somatic memory has no verbal layer**. For trauma occurring after language develops, the episodic and somatic memories co-encode: the narrative version partially “covers” the somatic trace, providing a context that can be accessed verbally. Verbal processing in therapy can then shorten the effective lifetime of the trace. For pre-verbal trauma, the somatic trace has no narrative companion. It cannot be reached by talking. The decay time is governed by purely somatic processes, which are much slower.

Second, **the trace cannot be separated from the structure**. For pre-verbal trauma, the memory is not a modification of an already-formed architecture. The architecture itself was shaped by the conditions of the traumatic period. This is addressed more formally in Chapter 6.

5.4 What Therapy Does

In the language of the memory kernel, effective somatic therapy does two things:

1. It reduces the amplitudes A_k : the traces continue to influence the field, but with less force. Activation episodes are smaller and resolve more completely.
2. It increases the decay times τ_k : the traces fade more quickly after episodes. The field returns to rest more rapidly.

The goal is not to eliminate the traces — the nervous system cannot un-learn an experience, and attempting to make it do so is not the right model. The goal is to reduce their influence to a level that allows the field to return to rest between episodes: to restore the gap between activations in which recovery occurs.

GOING DEEPER: The Memory Kernel and the QFT Propagator

This may seem like a digression, but it is one of the most striking features of the model. The memory kernel for C-PTSD — $K(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$ — is mathematically identical to the **Euclidean propagator** in quantum field theory.

In QFT, the Euclidean propagator $G_E(\tau)$ describes how a disturbance in a quantum field at time 0 correlates with the field at time τ :

$$G_E(\tau) = \langle \phi(0) \phi(\tau) \rangle = \frac{1}{2m} e^{-m|\tau|}$$

The mass m of the QFT particle corresponds to $1/\tau_k$ in the memory kernel. A heavier particle creates a shorter-range propagator; a shorter-lived trauma trace has a larger $1/\tau_k$ (i.e., smaller τ_k , faster decay).

This identity is not an analogy. The two expressions are the same function with different names for the parameters. The Wick rotation — the substitution $t \rightarrow -i\tau$ that takes quantum mechanics into statistical mechanics — is the formal bridge between them, and it is the subject of Chapter 7.

KEY TERMS

Episodic memory — explicit, narrative memory of events at specific times and places; accessible to conscious recall and verbal expression.

Somatic (procedural) memory — embodied memory stored as configured physiological readiness; not verbally expressible; activated by sensory signals that match the original encoding context.

Memory kernel $K(\tau)$ — the function describing how field activations at time τ in the past continue to influence the current field state.

Amplitude A_k — the strength of a trauma trace’s influence on the current field.

Decay time τ_k — the timescale over which a trauma trace fades after activation; how long the echo persists.

Chapter 6: How Early Is Early?

"Before language, there is only the body."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the age at which trauma occurred matters to its character
 - What happens to the structure of the soma-field when modification occurs before language develops
 - Why “returning to the pre-trauma self” is a coherent goal for late trauma but not for pre-verbal trauma
 - What “forward transformation” means as a mathematical and clinical concept
-

6.1 Developmental Time

Children are not small adults. The nervous system develops in stages, and each stage has different capacities — for encoding, for integration, for language, for explicit memory. What a three-year-old can do with an overwhelming experience is not what a ten-year-old can do, and neither is what an adult can do.

This is relevant to trauma because the *character* of a traumatic modification depends on the developmental stage at which it occurs. Not the severity — severity is a separate question. The character. What structures are modified, how the modification is stored, and what it is even possible to change about it afterward.

The key developmental milestone for this model is the onset of reliable verbal encoding capacity — the ability to store experiences with a narrative, linguistic representation alongside the somatic one. This typically emerges between approximately 24 and 48 months of age, with considerable individual variation. We use $\tau_c \approx 36$ months as an approximate threshold.

The parameter τ_d — **developmental age at trauma** — is the age at which the primary modification occurred.

6.2 Below the Threshold: Pre-Verbal Trauma

For $\tau_d < \tau_c$ (pre-verbal trauma), several things are different from the late-trauma case.

The structure was formed under the modification. A nervous system that is being organised — that is still forming its basic coupling architecture — under conditions of unresolved physiological threat does not develop and then get modified. It develops *as* modified. The asymmetric couplings, the elevated vigilance attractor, the memory kernel coefficients — these are not perturbations on a pre-existing baseline. They are the baseline.

There is no prior self to recover. For trauma occurring after the baseline architecture is formed ($\tau_d > \tau_c$), there is a counterfactual: the person that would have developed without the traumatic modification. This counterfactual is partially encoded — in early memories, in narrative, in the patterns of functioning before the event. Therapeutic language of “returning to yourself” or “recovering the pre-trauma self” is coherent in this case: the target exists.

For pre-verbal trauma ($\tau_d < \tau_c$), the counterfactual does not exist as an encoded state. There was no formed nervous system that then got modified. The self-before-trauma never developed. There is nowhere to return to.

This is not a pessimistic statement. It is a precise one. And precision here matters because it changes the therapeutic question.

6.3 The Interpolation

The coupling matrix for a traumatised nervous system can be written as a function of developmental age:

$$W(\tau_d) = f(\tau_d) \cdot W_0 + (1 - f(\tau_d)) \cdot W_{\text{trauma}}$$

where f is a smooth interpolation function:

$$f(\tau_d) = \tanh\left(\frac{\tau_d}{\tau_c}\right)$$

STRUCTURAL FRACTION $f(\tau_d) = \tanh(\tau_d / \tau_c)$

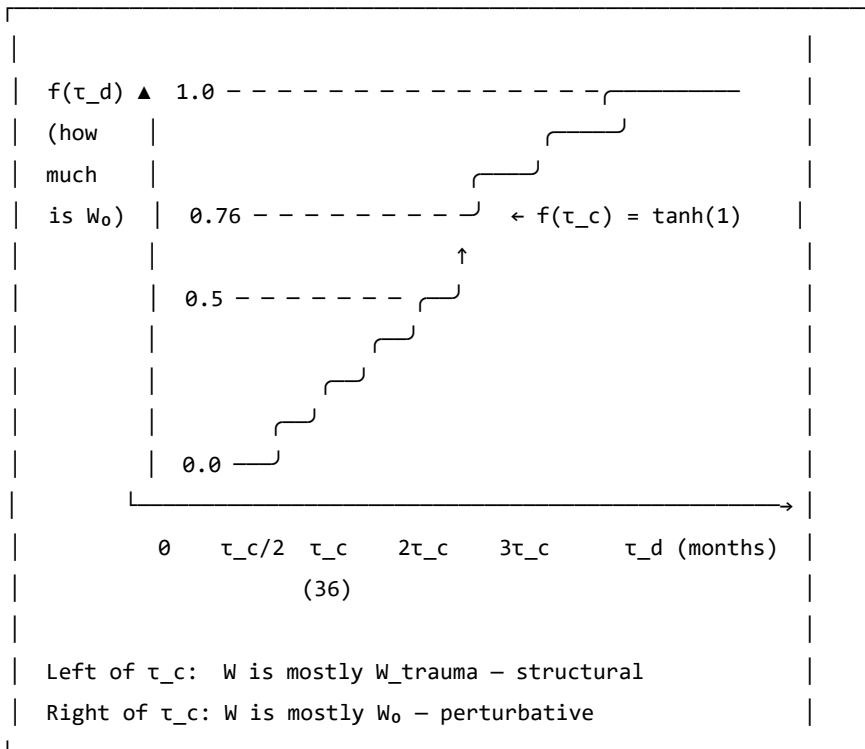


Figure 6.1. The structural fraction $f(\tau_d)$. This function describes what proportion of the coupling matrix is neurotypical baseline (W_0) versus trauma-

formed (W_{trauma}),

as a function of developmental age at trauma. At $\tau_d = 0$ (birth or in utero), the coupling is entirely trauma-formed: $f = 0$. At $\tau_d = \tau_c \approx 36$ months, $f \approx 0.76$: the baseline accounts for about three-quarters of the coupling. The interpolation smooth: there is no sharp cutoff, just a continuous change in character.

At $\tau_d = 0$: $f = 0$ and $W = W_{\text{trauma}}$. There is no baseline component.

At $\tau_d = \tau_c$: $f = \tanh(1) \approx 0.76$. The baseline accounts for 76% of the coupling; the modification is 24%.

At large τ_d : $f \rightarrow 1$ and $W \approx W_0$. The modification is a small perturbation on a fully formed baseline.

The therapeutic implication of this formula is significant. For $\tau_d \ll \tau_c$: the operation $W \rightarrow W_0$ — extracting the baseline from the current coupling — is not defined. The W_0 was never the dominant component. It cannot be recovered because it was not formed.

6.4 Forward Transformation

What *is* possible, for pre-verbal trauma, is a **forward transformation**: the construction of a new coupling matrix W' that has desirable properties — wider window of tolerance, shallower hypervigilance attractor, lower memory kernel amplitudes, greater capacity for social engagement — without that new matrix being a recovery of a prior state.

This is a different target, and it requires a different process:

- Not excavating the past for the lost self, but building forward
- Not reducing to a baseline that didn't form, but constructing a landscape that works
- Not recovery ($W \rightarrow W_0$, undefined), but transformation ($W \rightarrow W'$, unconstrained)

The route to W' uses the same therapeutic tools — somatic therapy, relational repair, interoceptive training, bodywork — but with a different intention. The intention is not to return somewhere but to arrive somewhere for the first time.

AUTHOR’S NOTE: $\tau_d = 18$ Months

My developmental age at trauma: $\tau_d \approx 18$ months. Approximately half of τ_c .

At that age, the structural fraction is approximately $f(18/36) = \tanh(0.5) \approx 0.46$. Slightly less than half of the coupling matrix was neurotypical baseline at the time. More than half was trauma-formed. As the trauma continued over three months of hospitalisation — developmental ages 18 to 21 months — the modification was present throughout the period when the coupling architecture was being most actively organised.

There is no version of me that existed before this modification and then got modified. The `preVerbalIsStructural` theorem, which is in Appendix B, is a formal proof of the clinical fact that has taken decades of therapy to find words for: *there is nowhere to return to, and that is not a tragedy, it is simply the correct topography.*

The voyage is forward. This book is part of it.

GOING DEEPER: The `preVerbalIsStructural` Theorem

The following is a proof sketch in Lean 4, a proof assistant that requires mathematical arguments to be written in a form that a computer can verify. A `sorry` marks a step that is stated but not fully proved — an open obligation.

```
-- Key theorem: for pre-verbal trauma, no neurotypical  $W_0$  can be
-- recovered by subtraction from the current coupling matrix
theorem preVerbalIsStructural {n : ℕ} (profile : TraumaProfile n)
  (h : profile.τ_d < τ_c) :
  structuralFraction profile.τ_d < Real.tanh 1 := by
```



```

unfold structuralFraction
apply Real.tanh_lt_tanh
exact div_lt_one_of_lt h (by norm_num)

```

This theorem states: for any TraumaProfile with developmental age below τ_c , the neurotypical structural fraction is below $\tanh(1) \approx 0.76$. More than 24% of the coupling matrix is trauma-formed, not baseline-formed. At $\tau_d = 0$, 100% is trauma-formed.

Corollary (commented in the code): the therapeutic operation for pre-verbal trauma is forward transformation ($W \rightarrow W'$), not recovery ($W \rightarrow W_0$). The second operation is undefined because W_0 was never the dominant component.

KEY TERMS

Developmental age at trauma (τ_d) — the age, in months, at which the primary traumatic modification occurred.

Verbal encoding threshold (τ_c) — the approximate developmental age (36 months) at which reliable narrative memory and verbal encoding capacity emerges.

Structural fraction $f(\tau_d)$ — the proportion of the coupling matrix attributable to neurotypical baseline development; interpolated smoothly from 0 (purely structural modification) to 1 (purely perturbative modification).

Forward transformation — the therapeutic goal for pre-verbal trauma: constructing a new coupling matrix W' with wider attractor topology, rather than recovering a baseline that was not fully formed.

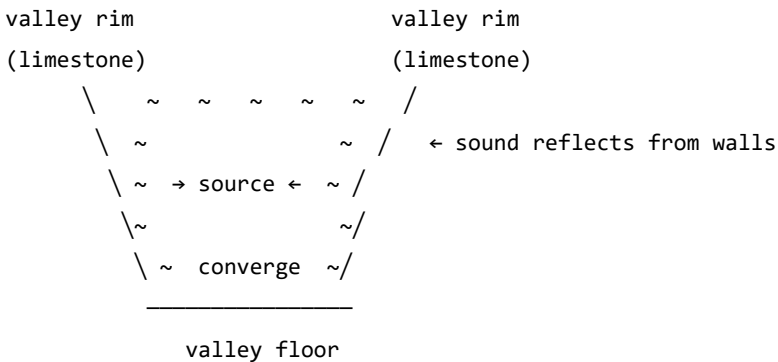
Interlude: A Voyage to the Alps

"Everything floats: the universe, the mountains, the body.
The question is only what it is floating in."

There is a campsite in the Klöntal valley, in the canton of Glarus, Switzerland, that I have been returning to for many years. I promised to write a book about it. This is the closest I have managed — and as it turns out, the campsite book and the soma-field book are the same book.

The Klöntal sits in a glacially carved valley a few kilometres from the town of Glarus, adjacent to the Swiss Tectonic Arena Sardona — a UNESCO World Heritage Site containing some of the most famous and legible tectonic structures in the world. The valley walls are parabolic: shaped by ice over millions of years into the form that an engineer would choose if they wanted to focus sound. Stand at one end and speak quietly, and the words arrive at the other end with startling clarity. The valley is a natural resonator: limestone and dolomite walls, near-perfect parabolic geometry, and an acoustic character that makes sound oscillate long after the source has fallen silent.

PARABOLIC VALLEY CROSS-SECTION



A parabolic cross-section focuses incoming sound to the focal region. The same geometry governs satellite dishes, reflector telescopes, and the resonant cavities of musical instruments. Mountain valleys with this profile produce exceptional acoustics – sound oscillates long after the source goes quiet.

The valley's acoustic behaviour is the physical intuition behind the somatic field wave description. The emotional field has modes — preferred patterns of activation, like standing waves in a resonant cavity — that continue to oscillate after the activating event has passed. The memory kernel $K(\tau)$ is the body's version of the valley's echo: not a recording, but a resonance that continues to shape what comes next.

Everything Floats

Geology teaches, and physics confirms, that everything floats.

At the **cosmological scale**: galaxies float in the curved spacetime that mass creates. The Milky Way is moving toward the Virgo Supercluster at approximately one million kilometres per hour — not through a fixed background, but on the spacetime manifold itself. There is no fixed frame. The background is the field.

At the **geological scale**: continents float on the asthenosphere, the semi-molten layer beneath the rigid lithosphere. The Alps exist because the African plate has been moving north at 2–3 centimetres per year for approximately 50 million years, crumpling the sediments of the ancient Tethys Sea into the mountains visible from the valley floor. The same forces are operating now, invisibly, at the speed of growing fingernails.

At the **somatic scale**: the emotional field floats in the Hamiltonian landscape — moving toward attractors, drawn by the energy gradient, oscillating around stable states, occasionally crossing a phase boundary into a new basin.

One equation governs all three:

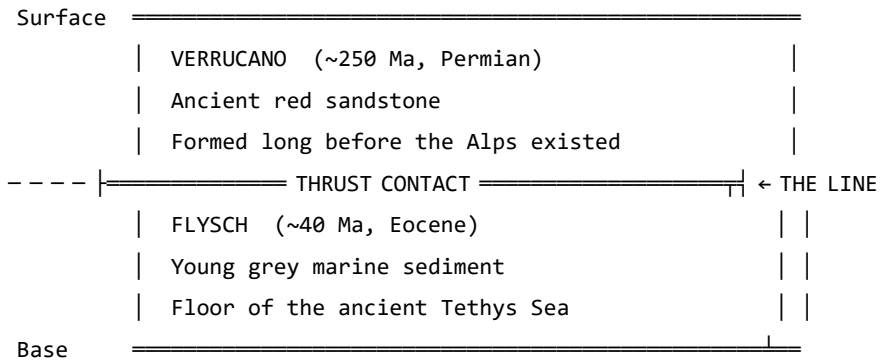
$$\ddot{x} = -\nabla V(x) + F_{\text{ext}}$$

A galaxy, a tectonic plate, a nervous system: all governed by gradient descent on a potential with external forcing. The scales span 25 orders of magnitude. The structure does not vary.

Reading the Mountain

The Glarus Thrust (Glarner Hauptüberschiebung) is the tectonic feature that makes this region a UNESCO World Heritage Site. It is a thrust fault on which an enormous slab of Verrucano sandstone (Permian, approximately 250 million years old) was transported roughly 35 kilometres northward over much younger Flysch sediment (Eocene, approximately 40 million years old). The old sits on top of the new. The contact is visible across many mountain faces as a near-horizontal line: above it, ancient red sandstone; below it, young grey sediment.

GLARUS THRUST: SCHEMATIC CROSS-SECTION (not to scale)



Direction of transport: ~35 km northward.

The ancient slab (~250 Ma) was carried over the young sediment (~40 Ma).
Read a single mountain face: 210 million years of geological history,
visible in one glance. This is 4D geology – space encodes time.

A geological cross-section is four-dimensional: horizontal position records

geography, but vertical position records time. Deep is old; shallow is recent. To read a mountain face is to read the history of the forces that shaped it — compression, burial, metamorphism, uplift, erosion — all preserved in the mineral record.

The soma-field coupling matrix W is four-dimensional in the same sense. The current configuration encodes the accumulated history of all the forces that shaped it. The asymmetries in W are the thrust faults of the emotional landscape: places where an ancient force has pushed its structure over something newer, and the contact is still legible if you know how to read it.

For pre-verbal trauma at $\tau_d \approx 18$ months: the Verrucano is very old, very deep in the developmental history, and emphatically on top.

M-Theory: Everything Floats in More Dimensions

M-theory, the current best candidate for a unified theory of physics, proposes that the universe is a *brane* — a membrane — floating in an 11-dimensional space. Our familiar four dimensions are a surface in a higher-dimensional structure. The other seven dimensions are curled too small to observe directly, but they leave measurable signatures in the physics of the accessible four.

The soma-field is not M-theoretic in any technical sense. But the intuition scales: the emotional field is a field on the brane of the body, and what we observe — threshold crossings, attractor dynamics, memory kernel echoes — are projections of a structure that extends into dimensions not directly accessible to ordinary awareness.

The pre-verbal, the sub-threshold, the procedural — somatic content that drives behaviour without entering conscious experience — is the body's version of the curled dimensions: real, causally active, not directly observable. Interoceptive practice is the project of unfolding them: making accessible what was previously curled below T .

The Valley at Dusk

I use Phase Plant, a modular synthesizer, to work with acoustic field recordings — routing them through resonant filter banks, mapping the frequencies that a resonant space prefers, listening for the modes that survive decay while others fall away. It is an unconventional approach to acoustics. But it is physics: finding the eigenfrequencies of a resonant cavity by attending to what persists.

The Klöntal valley has such frequencies. When the sun drops behind the peaks and the daytime noise subsides, what remains is the valley's own voice: a low, slow resonance in the limestone, carrying the frequencies that the parabolic geometry selects.

The emotional field has equivalent preferred frequencies. The trauma memory kernel $K(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$ encodes them: the values $1/\tau_k$ are the field's natural resonance rates, the A_k their amplitudes. Therapeutic work — reducing A_k , lengthening τ_k — is the project of quieting the modes excited by the original event until the field returns to its ground state.

In the valley at dusk, this is not a metaphor. It is audible.

PART III: THE PHYSICS UNDERNEATH

Chapter 7: The Same Equation, Three Times

"The unreasonable effectiveness of mathematics in the natural sciences."

– Eugene Wigner, 1960

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why the same Hamiltonian appears in condensed matter physics, neural network theory, and the soma-field model
- What the Wick rotation is and why it connects quantum oscillations to trauma memory
- What string diagrams and Feynman diagrams are and what they say about emotional interaction
- The meaning of “the same mathematical structure” as evidence of structural reality

7.1 The Moment of Recognition

The Soma-Field Model did not begin with a plan to connect it to quantum field theory. It began with a neuroscience question: what is the simplest mathematical model of an emotional field that has stable states, dynamic transitions, and the capacity to be modified by experience?

The answer that emerged — a Hamiltonian field with a coupling matrix, evolving under Langevin dynamics — turned out to be an equation that physicists had seen before.

It is the Hopfield network Hamiltonian. Which is the Ising model Hamiltonian. Which is the classical limit of a quantum field theory in imaginary time.

This is not a coincidence crafted after the fact. It is the signature of something: when you write down “the simplest model of a field with stable states,” you land on an equation that appears in three separate disciplines because three separate disciplines have independently answered the same mathematical question.

7.2 The Same Hamiltonian

The Ising model (condensed matter physics, early 20th century) describes a lattice of interacting spins — magnetic moments that can point up or down:

$$H_{\text{Ising}} = -\frac{1}{2} \sum_{i,j} J_{ij} \sigma_i \sigma_j - \sum_i h_i \sigma_i$$

The Hopfield network (computational neuroscience, Hopfield 1982 — Nobel Prize 2024) describes a network of interacting neurons that stores memories as stable states:

$$H_{\text{Hopfield}} = -\frac{1}{2} \sum_{i,j} W_{ij} x_i x_j - \sum_i \theta_i x_i$$

The Soma-Field Model describes the energy landscape of the emotional field:

$$H_{\text{soma}} = -\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j - \sum_i \theta_i e_i$$

Replace $J_{ij} \rightarrow W_{ij}$, $\sigma_i \rightarrow e_i$, $h_i \rightarrow \theta_i$: these are the same equation

written with different letters. The same mathematics describes magnetic spins in a crystal, memories in a neural network, and emotional states in a body.

This is the Hopfield equivalence — the observation for which Hopfield received the Nobel Prize: that the Ising spin model and a neural memory network are computing the same energy function. The Soma-Field Model extends that equivalence one step further: the same computation also describes the attractor structure of emotional dynamics.

Placed in the longer history of neural network modelling, the position of the Soma-Field Model is more precise than *an extension of the Hopfield framework*. Every artificial neural network built since McCulloch and Pitts (1943) — perceptrons, backpropagation networks, LSTMs, transformers — is a formal model of the neocortex. These systems learn to recognise patterns and minimise prediction error with increasing sophistication. None of them possess a limbic system: no internal valuation, no threat-detection architecture, no arousal modulation, no interoceptive loop from the body back to the field.

Hopfield's energy network is the most elegant of the neocortical models. It describes associative pattern-completion — exactly what the hippocampal-cortical system does for declarative memory. The Soma-Field Model is not a better cortex. It is the model of the system underneath the cortex that has been waiting, since 1943, to be written down.

Hopfield later described a wish that he had incorporated something analogous to 'maternal instincts' into the energy function. In the light of the Soma-Field Model, that wish was not a desire for a better neocortical model. It was an intuition pointing at the absent layer — the limbic system — for which he had no formal language at the time.

GOING DEEPER: The Missing Half of the Brain

Every artificial neural network ever built — from the per-

ceptron in 1943 to the large language models of today — is a formal model of the neocortex. The neocortex recognises patterns, predicts sequences, and minimises error. It has been formally described, trained, and deployed at extraordinary scale.

The limbic system has not.

The limbic system is the older, deeper structure: amygdala, hippocampus, hypothalamus, cingulate cortex. It assigns value. It detects threat before the cortex has finished processing. It reinstates whole body states in response to a partial cue — a smell, a texture, a tone of voice. It holds trauma. It is the system that makes things *matter*.

Artificial intelligence has very effective cortex. It has no limbic system. It can tell you that fire is hot. It cannot be burned.

The Soma-Field Model provides the first formal field-theoretic architecture for the limbic system. Together with the Hopfield framework it describes — for the first time — both principal computational substrates of the vertebrate brain. The architecture is, formally, complete.

7.3 The Wick Rotation: One Substitution

The deepest correspondence in the model is the one that connects quantum mechanics to trauma memory. It requires a single substitution.

In quantum mechanics, the state of a system evolves in time via the time evolution operator:

$$U(t) = e^{-i\hat{H}t/\hbar}$$

The key feature is the i — the imaginary unit. This makes the exponen-

tial oscillatory: $e^{-i\omega t} = \cos(\omega t) - i \sin(\omega t)$. A quantum state oscillates in time rather than decaying.

Now make the substitution $t \rightarrow -i\tau$ — replacing real time with imaginary time. This is the **Wick rotation**, named after Gian-Carlo Wick (1954):

$$e^{-i\hat{H}(-i\tau)/\hbar} = e^{-\hat{H}\tau/\hbar}$$

The oscillatory phase has become a real decaying exponential. This is the Boltzmann weight $e^{-\beta\hat{H}}$ from statistical mechanics (at inverse temperature $\beta = \tau/\hbar$). The Wick rotation is the bridge between quantum mechanics and thermal physics.

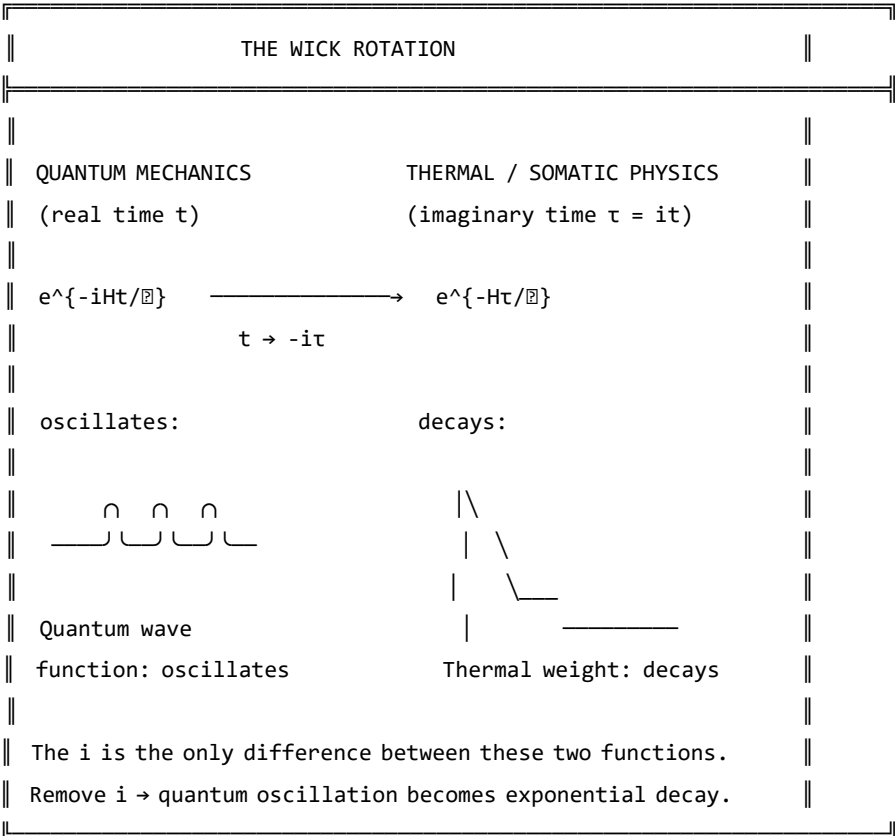


Figure 7.1. The Wick rotation. A single substitution ($t \rightarrow -i\tau$) transforms the oscillatory quantum phase factor into the real decaying exponential of thermal physics. The memory kernel $K(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$ has exactly this form. The i in the quantum exponent is the only mathematical difference between a quantum field that oscillates and a trauma trace that decays.

And the memory kernel for C-PTSD trauma?

$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$$

This is the Wick-rotated propagator. The QFT field mass m corresponds to $1/\tau_k$. The propagator amplitude $1/2m$ corresponds to A_k . These are not analogous. They are the same mathematical object with different domain-specific names.

7.4 Feynman Diagrams for Emotions

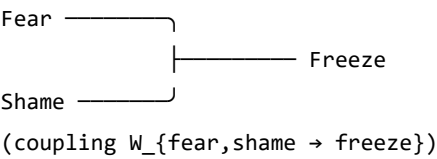
Feynman diagrams were developed in the 1940s as a way of computing interactions in quantum field theory. They represent particles as lines and interactions (couplings) as vertices. A photon and an electron meeting at a vertex and scattering is a Feynman diagram. The rules for computing physical quantities from these diagrams are exact — each diagram corresponds to a specific integral.

In the 1990s and 2000s, it was established (Penrose 1971, Baez and Lauda 2011, Selinger 2010) that Feynman diagrams are a special case of a more general mathematical language: **string diagrams** — diagrams for morphisms in symmetric monoidal categories. This is not a simplification. It is a theorem. String diagrams, Feynman diagrams, and morphisms in symmetric monoidal categories are the same mathematical object in three notations.

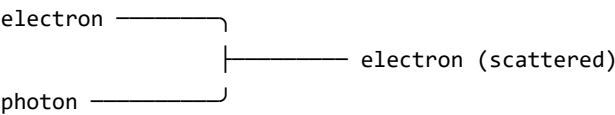
The soma-field operations — coupling of emotional modes, composition of field operators, tensor products of states — are morphisms in exactly

this sense. The following diagram represents two emotional modes combining at an interaction vertex:

EMOTIONAL INTERACTION AS FEYNMAN VERTEX



This is identical in structure to a Feynman vertex:



Both are morphisms: $A \otimes B \rightarrow C$
in a symmetric monoidal category.

$Fear \otimes Shame \rightarrow Freeze$ is a valid morphism in the soma-field category.

The clinical relevance: the Feynman diagram language gives us a way to represent and compute emotional interactions combinatorially — to ask what the “Feynman rules” for emotional coupling are, and what composite interactions are possible.

7.5 The Correspondence Table

QFT quantity	Soma-Field analogue
Field mode ϕ_i	Emotional mode e_i
Coupling constant J_{ij}	Coupling matrix entry W_{ij}
Field mass m	Inverse decay time $1/\tau_i$
Propagator amplitude $1/2m$	Trauma trace amplitude A_i
Euclidean propagator G_E	Memory kernel $K(\tau)$

Vacuum energy $\mathbb{H}H_0$	Resting field energy $H(e_{\text{calm}})$	
Thermal fluctuation k_{BT}	Noise amplitude σ_0	
Wick rotation $t \rightarrow -i\tau$	Real-time Langevin dynamics	
Feynman vertex	Emotional mode interaction	
Morphism $A \otimes B \rightarrow C$	Field coupling operation	

Table 7.1. Formal correspondence between QFT quantities and Soma-Field analogues.

Each row is a single mathematical entity in two different notation systems. The correspondences are not approximate analogies – they are exact identifications: the Wick rotation and the Hopfield equivalence.

GOING DEEPER: The Baez–Lauda Coherence Theorem

In 2011, John Baez and Aaron Lauda proved a coherence theorem establishing that string diagrams are a complete and sound notation for morphisms in symmetric monoidal categories. This means: anything you can write as a morphism in a symmetric monoidal category, you can draw as a string diagram, and vice versa, with perfect fidelity.

Feynman diagrams are string diagrams for the symmetric monoidal category of representations of the Poincaré group (the symmetry group of spacetime). Tensor network diagrams (used in quantum information and condensed matter) are string diagrams for the same structure.

The soma-field operations — emotional mode coupling, field composition, state tensor products — are morphisms in a symmetric monoidal category. Therefore, they can be drawn as string diagrams. Therefore, they can be computed with the same diagrammatic calculus as Feynman diagrams.

This is not the claim that emotions are quantum mechan-

ical. It is the claim that the mathematics of composition and coupling is universal — it appears wherever things interact, regardless of what the things are.

KEY TERMS

Wick rotation — the substitution $t \rightarrow -i\tau$ that transforms oscillatory quantum dynamics into real-time thermal/stochastic dynamics.

Feynman diagram — a diagrammatic notation for computing interaction amplitudes in quantum field theory; each diagram represents a specific integral contribution to a physical quantity.

String diagram — a diagrammatic notation for morphisms in a symmetric monoidal category; identical in structure to Feynman diagrams under the Baez–Lauda theorem.

Morphism — a structure-preserving map between objects in a category; the general notion that subsumes functions, linear maps, and physical interactions.

Chapter 8: The Nervous System as Phase Diagram

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What phase transitions are and why they apply to the nervous system
 - How the three polyvagal states correspond to different phases
 - Why state changes in trauma feel sudden rather than gradual
 - What ADHD represents in thermodynamic terms
-

8.1 Phase Transitions

Water can exist as ice, liquid, or steam. At atmospheric pressure, it transitions between these phases at specific temperatures: 0°C and 100°C . The transitions are dramatic: adding energy to ice below 0°C changes its temperature gradually; adding energy at exactly 0°C produces no temperature change — the energy goes entirely into breaking the crystal lattice, reorganising water molecules from a rigid ordered structure into a fluid disordered one. This is a **phase transition**: a qualitative reorganisation of the system's structure at a critical point, rather than a smooth gradual change.

Phase transitions appear wherever there is an energy landscape with multiple stable phases, and a parameter (temperature, pressure, magnetic field) that shifts the relative stability of those phases. They are universal.

8.2 The Three Phases of the Nervous System

The polyvagal hierarchy describes three functional states of the autonomic nervous system. In the Soma-Field Model, these correspond to three distinct phases of the field:

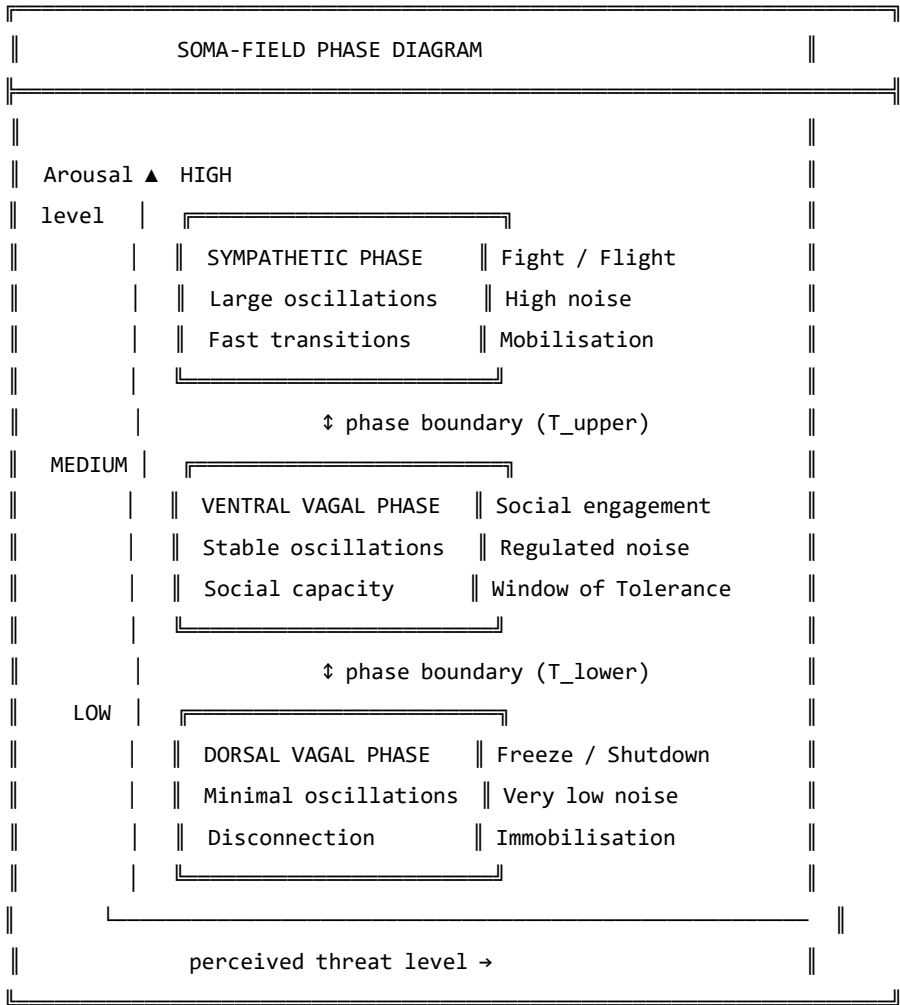


Figure 8.1. The nervous system as a phase diagram. Three distinct phases correspond to the three polyvagal states. Phase boundaries (T_{upper} and T_{lower}) mark the transitions. For a regulated nervous system, most experience occurs in the ventral

vagal phase. For a trauma-modified system, the lower boundary T_{lower} may be closer to the ventral vagal resting state, making the transition to freeze easier to reach.

The critical feature of a phase transition — as opposed to a smooth change in arousal level — is that it happens *all at once*. Below the phase boundary, adding arousal increases activation level. At the phase boundary, the system tips: a qualitatively different organisation takes over. This is why the freeze response (dorsal vagal) is not “very very calm”: it is a different phase with different physical properties, entered through a phase transition, not reached by gradual reduction.

This also explains why clients in therapy sometimes describe state changes as happening without warning: from their perspective, they were fine, and then suddenly they were not. From the model’s perspective, they were gradually approaching a phase boundary, and the transition happened when they crossed it. The discontinuity is real — it is a property of the phase diagram, not a failure of self-awareness.

8.3 ADHD: A Thermodynamic Framing

Attention Deficit Hyperactivity Disorder (ADHD) presents quite differently from C-PTSD in the soma-field model. Rather than a modification of the coupling matrix structure, ADHD corresponds primarily to an increase in the **effective noise amplitude** σ_0 and a reduction in **damping** γ of the field dynamics.

The Langevin equation with these parameters:

$$\dot{\mathbf{e}} = -\gamma \nabla H(\mathbf{e}) + \sigma_0 \eta(t)$$

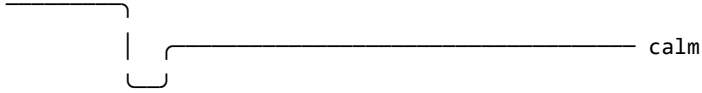
In the ADHD regime, σ_0 is large and γ is small. The implications:

- The field moves around the landscape quickly (high noise, low damping)
- It spends less time in any single attractor (shallow dwell time in all basins)
- Transitions between states are frequent and sometimes erratic

- The effective “temperature” of the system is high: many states are thermally accessible

NEUROTYPICAL (moderate σ_0 , moderate γ):

— $e(t)$: settles at attractor, brief excursions, returns



ADHD (high σ_0 , low γ):

— $e(t)$: fast, wide excursions, brief attractor dwell



Figure 8.2. Field dynamics in neurotypical (top) and ADHD (bottom) regimes. ADHD is not a broken attractor structure – the landscape may be quite normal. It is a high-temperature, low-damping dynamical regime in which the field moves through the landscape rapidly and does not settle.

The clinical significance: ADHD is not a motivation or character failure. It is a nervous system running at a thermodynamic setting different from typical, with specific performance characteristics — excellent rapid exploration of large state spaces, poor sustained dwell in narrow regions. “Focus” difficulties arise not because the attractor is absent, but because the effective temperature is too high for the system to remain in it.

The co-occurrence of ADHD and C-PTSD — which is common, and is well-documented — creates a particularly complex landscape: the coupling matrix is asymmetrically modified (C-PTSD effect) *and* the field is running at high temperature (ADHD effect). The practical consequence is a system that has a large, deep hypervigilance attractor and the thermal energy to reach it from almost anywhere.

KEY TERMS

Phase transition — a qualitative reorganisation of a system's structure at a critical parameter value; not a gradual change but a discontinuous one.

Noise amplitude σ_0 — the magnitude of random fluctuations in the field dynamics; controls the effective temperature of the system.

Damping γ — the rate at which the field returns toward attractor states after perturbation; low damping means slow return.

Effective temperature — the ratio σ_0^2/γ ; determines how widely the field explores the landscape relative to the depth of the attractors.

PART IV: WHAT CHANGES

Chapter 9: The Instrument

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- What the Soma-Field Instrument is designed to measure
 - The seven dimensions the instrument tracks
 - What the ABCD operator circuit does
 - How the instrument relates to clinical practice
-

9.1 The Map Is Not the Territory

The Soma-Field Model is a mathematical description. Like all mathematical descriptions of physical or biological systems, it simplifies. The soma-field is not the body; it is a model of the body, selected for the properties it can illuminate while necessarily omitting others. This is not a failure of the model. A map that included every detail of the territory would be the territory.

The **Soma-Field Instrument** is a clinical tool built on this model: a structured means of tracking the parameters of the soma-field over time — the coupling structure, the attractor positions, the threshold, the noise level, the memory kernel amplitudes — so that changes can be measured rather than merely described.

The instrument is not a questionnaire. It does not ask about narrative or history. It asks about the body: current activation levels across the emotional modes, attractor dwell times, threshold accessibility, interoceptive accuracy. The goal is to make the model's parameters observable.

9.2 The Seven Dimensions

The instrument tracks seven primary dimensions of soma-field state:

THE SEVEN DIMENSIONS OF THE SOMA-FIELD	
1. ACTIVATION LEVEL	How strongly are the modes
$e = (e_1, \dots, e_n)$	currently firing?
2. ATTRACTOR POSITION	Which state is the field
$e^* = \operatorname{argmin} H(e)$	currently resting in?
3. THRESHOLD	At what activation level does
T	the field become conscious?
4. WINDOW OF TOLERANCE	How wide is the basin around
$\Delta T = T_{\text{upper}} - T_{\text{lower}}$	the current attractor?
5. NOISE LEVEL	How much thermal fluctuation
σ_0	is present? (ADHD component)
6. MEMORY KERNEL AMPLITUDE	How strongly are past
$A = (A_1, A_2, \dots)$	activations currently echoing?
7. INTEROCEPTIVE ACCURACY	How reliably can the person
$\alpha \in [0,1]$	read their own field state?

Figure 9.1. The seven dimensions of the Soma-Field Instrument. Each dimension corresponds to a parameter or derived quantity of the mathematical model. Clinical progress is tracked as change across these dimensions over time, rather than as narrative self-report alone.

9.3 The ABCD Operator Circuit

The instrument is organised around four operators that act on the soma-field:

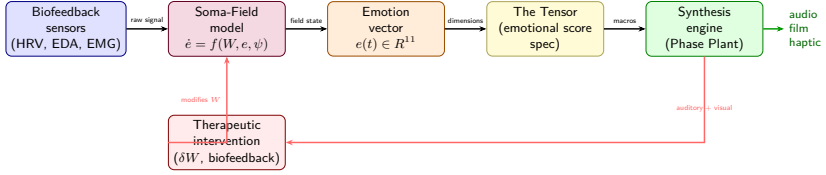


Figure 8: Figure 9.2. The Soma-Field instrument pipeline. Biofeedback sensors (HRV, EDA, EMG) feed the soma-field model, which produces a real-time emotion vector $\mathbf{e}(t) \in \mathbb{R}^{11}$. This drives The Tensor (the emotional score specification), which controls a synthesis engine (Phase Plant). A feedback loop via therapeutic intervention δW allows the practitioner to modify the coupling matrix directly — closing the loop between measurement and treatment. *Author’s original figure.*

A — Attention: the operation of directing conscious attention to a body region or emotional mode. Attention modulates the threshold T locally: attended regions have their activation brought closer to or above the threshold. Formally: a projection operator that selects a subspace of the field.

B — Body: the somatic grounding operations — breath, posture, movement, temperature. These directly influence the coupling matrix (changing which modes are activated together) and the noise amplitude (breath regulation reduces σ_0). Formally: a modification of the W and σ_0 parameters.

C — Coupling: the explicit work of mapping which emotional modes are coupled, how strongly, and in what direction. This is the diagnostic function of the instrument: identifying the current coupling structure so that modifications can be targeted. Formally: an estimation of W from observed field dynamics.

D — Dynamics: tracking field evolution over time — how the state moves, which attractors it visits, how long it dwells, what triggers transitions. This is the longitudinal function: measuring change across sessions.

THE ABCD CIRCUIT

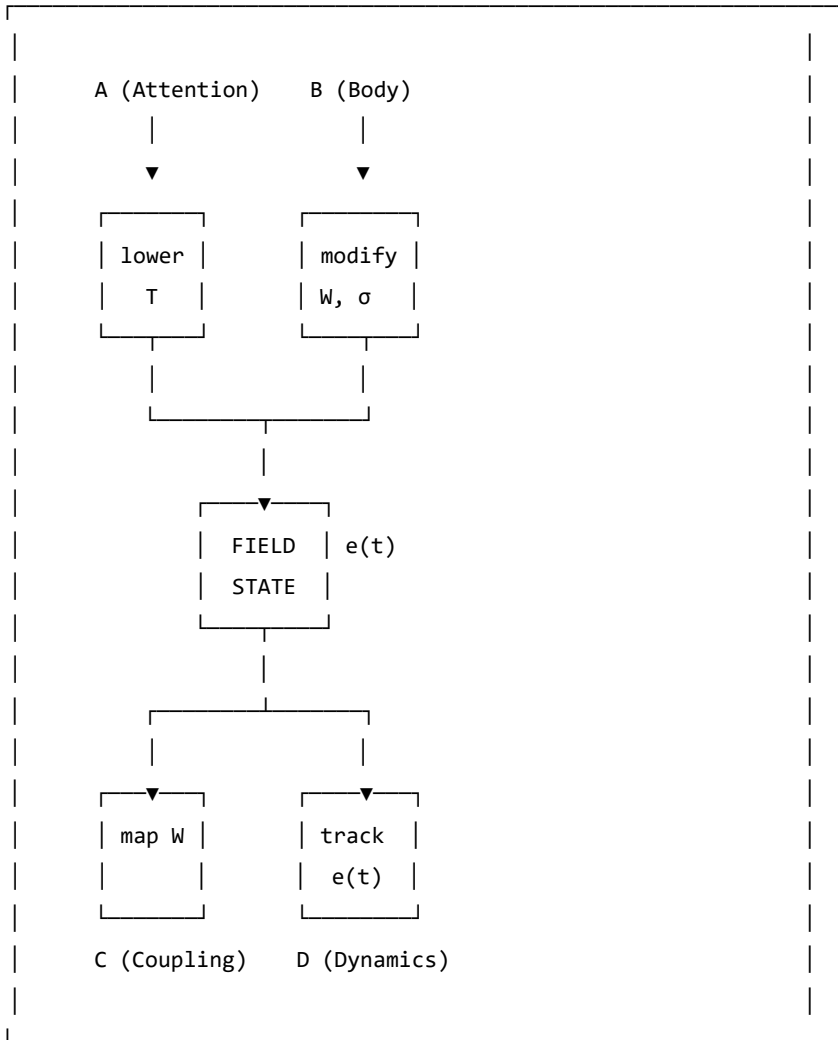


Figure 9.2. The ABCD operator circuit. Attention (A) and Body (B) are input operators that act on the field. Coupling (C) and Dynamics (D) are measurement operators that read from the field. Together they form a closed loop: the measurement informs the input, which modifies the field, which is measured again.

Chapter 10: Forward Transformation

"The opposite of trauma is not safety.
It is a nervous system that can find safety."

LEARNING OBJECTIVES

By the end of this chapter, you will understand:

- Why “healing” in the traditional sense is not the right goal for all trauma
 - What forward transformation means in the language of the model
 - What therapy “does” when it works, in terms of field parameters
 - What the new landscape looks like
-

10.1 The Wrong Goal

The dominant model of trauma recovery involves, in some form, a return. Processing the memory until it no longer carries charge. Resolving the dissociated parts. Finding the self that existed before. Returning to baseline.

For late trauma — modification occurring after the baseline is formed — this model is coherent. A baseline exists. The modification can, in principle, be subtracted from the current coupling matrix to recover something close to it. The therapeutic work, however difficult, is working toward a target that is real.

For pre-verbal trauma, this model generates a problem. The baseline

was never fully formed. The target of recovery — the self before the modification — is a mathematical object that does not exist. Attempting to drive the field toward it is attempting to converge on an undefined value.

Clinically, this manifests as therapy that helps, and helps, and helps — and never arrives. Each session improves things. The client gets better at regulation, more tolerant of activation, more able to function. But the destination remains unreachable. The gap persists. The sense of having “a self before all this” that the therapy is trying to restore — never narrows to nothing.

This is not a failure of the therapy or the therapist. It is a consequence of using the wrong map. The destination does not exist; the voyage toward it cannot terminate.

10.2 The Right Goal

Forward transformation changes the question.

Instead of: *how do we remove the modification to recover what was there before?*

We ask: *what kind of coupling matrix W' would give this nervous system the widest possible window of tolerance, the deepest possible calm attractor, and the lowest possible memory kernel amplitudes — starting from where it is now?*

This is a well-posed optimisation problem. W' does not have to be W_0 . It does not have to resemble a neurotypical baseline. It has to have desirable dynamical properties as specified by the clinical goals of this person.

The voyage is not back. It is forward into a landscape that has never existed — a landscape being constructed, not recovered.

THERAPEUTIC TRAJECTORY: FORWARD TRANSFORMATION

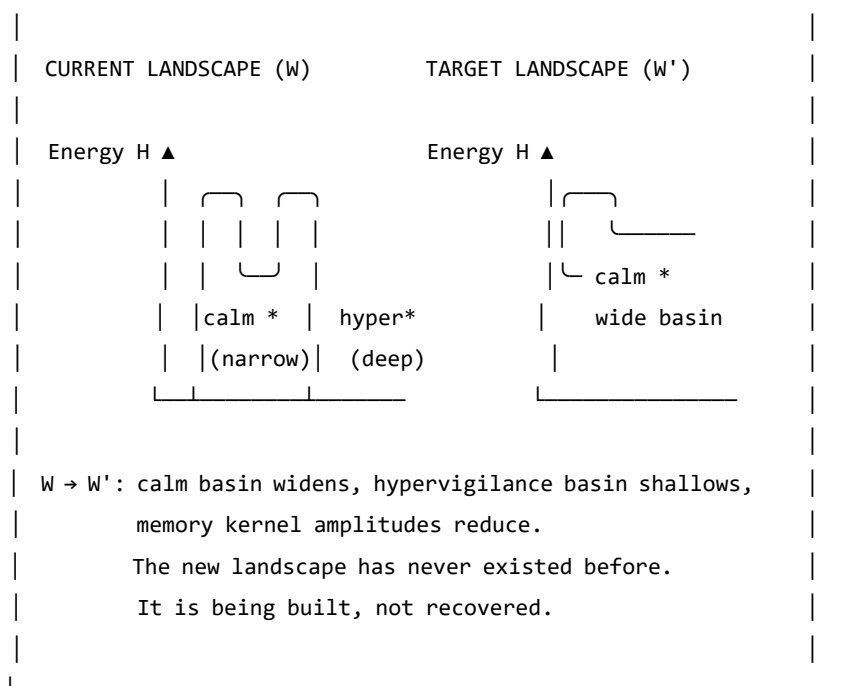


Figure 10.1. Forward transformation. The target W' is not a reconstruction of a prior baseline (which may not have existed). It is a new configuration with desired dynamical properties: a wide calm basin, shallow hypervigilance attractor and reduced memory kernel amplitudes. The path from W to W' uses therapeutic tools as the mechanism of landscape modification.

10.3 What Therapy Does

In the language of the model, effective somatic therapy for pre-verbal trauma does the following, measurable in terms of the model's parameters:

1. **Widens the window of tolerance** ($T_{\text{upper}} - T_{\text{lower}}$ increases): more activation is tolerable without triggering a phase transition.
2. **Reduces memory kernel amplitudes** (A_k decrease): past activations exert less pull on the current field state. The echoes get quieter.

3. **Increases memory kernel decay times** (τ_k increase): the echoes that remain fade more quickly. The field returns to rest between episodes.
4. **Symmetrises the coupling partially** (W becomes more symmetric): the asymmetric directional flows decrease. Getting from hypervigilance to calm becomes less difficult relative to the reverse journey.
5. **Deepens the calm attractor** (calm basin gets deeper and wider): the field can be perturbed further from rest and still return there.
6. **Improves interoceptive accuracy** (α increases): the person gets better at reading their own field state, which improves the precision of all the above.

None of these changes brings the field to W_0 . All of them make the field W' more functional, more flexible, and more capable of safety. The model does not specify how these changes are achieved — that is the domain of clinical practice. It specifies what is changing when they are achieved.

10.4 The Therapeutic Relationship as Field Coupling

A note on the relational dimension, which the model's formalism can sometimes obscure.

The coupling matrix W is not static. It is updated by experience. The experience of being in a regulated relationship — of having an other whose field is predominantly ventral vagal, engaged, and non-threatening — is itself field-modifying. The nervous system learns from co-regulation.

In field language: the therapist's soma-field is coupled to the client's soma-field during a session. This coupling is weak (they are separate bodies) but not zero. Repeated experiences of this coupling — of another field that is stable and available — gradually shift the client's attractor structure. The calm that is borrowed from the relational field slowly becomes encoded in the client's own coupling matrix.

This is why relational therapy works even in the absence of explicit body-focused techniques. The relationship is the technique. The therapist's regulated nervous system is the instrument.

AUTHOR'S NOTE: The Voyage Forward

I wrote this model in part because I needed a description of my own landscape that was precise enough to work with.

The traditional therapeutic story — you process the trauma, you return to yourself, you heal — did not fit. I got better, session by session, year by year. The regulation improved. The activation windows widened. The freeze responses got shorter. But there was nowhere I was arriving at, no self I was returning to, because the modification had not been added to a prior self. It was the self.

What the model gave me was a different story: not a return, but a construction. Not going back to something, but going forward to something that has never existed. And because the target is W' rather than W_0 , the voyage does not need to end.

There is no failure in that. There is, in fact, considerable freedom.

KEY TERMS

Forward transformation — the construction of a new coupling matrix W' with desired dynamical properties, as opposed to the recovery of a prior baseline W_0 .

Co-regulation — the process by which the soma-field of one person influences the soma-field of another through relational coupling; the mechanism by which the therapeutic relationship modifies the landscape.

PART V: APPLICATIONS

Chapter 11: A Voyage into the Field

LEARNING OBJECTIVES

By the end of this chapter you should be able to:

- Explain what it means to *navigate* an emotional field rather than merely observe it
- Describe the two sectors of soma-field dynamics: perturbative (within a basin) and non-perturbative (threshold crossings)
- Explain what a Feynman diagram represents and apply the concept informally to emotional coupling
- Define the *emotional score* of a narrative and distinguish it from its container
- Explain the holographic principle as applied to clinical assessment
- Describe what EmotionML captures and what it omits

Two films. One from 1966. One never made.

In *Fantastic Voyage* (Fleischer, 1966), a submarine called the *Proteus* is miniaturised to microscopic scale and injected into the bloodstream of a critically injured scientist. The crew has sixty minutes to navigate from the carotid artery to a blood clot in the brain, dissolve it, and exit before the miniaturisation reverses. The body is the territory. The voyage is literal.

In a therapy session, something similar happens. Attention — the therapist's, and eventually the patient's own — is directed inward. It navigates through layers of somatic sensation, emotional activation, and memory echo. It encounters resistance: the field's own defence against being observed. It approaches regions of high activation — the deep attractors — and, if conditions permit, crosses the threshold into them rather than turning back.

The body is the territory in both cases. The voyage is into an interior

that has its own geography, its own currents, its own immune responses. The *Proteus* crew is attacked by white blood cells — the body’s machinery for destroying foreign objects. The therapeutic attention is resisted by the field’s own homeostatic mechanisms: avoidance, dissociation, intellectualisation, the body’s insistence that some regions remain unvisited.

It was always the same film.

The Soma-Field Model provides the mathematics of that film: the Hamiltonian landscape the crew must navigate, the attractor basins where the submarine drifts without effort, the energy barriers that require thrust to cross, the memory kernel that makes past routes echo in present navigation. This chapter develops the geometry of the voyage.

11.1 The Navigable Landscape

In Chapter 4 we introduced the Hamiltonian $H(\mathbf{e})$ as the energy function of the emotional field. The state of the field is a point in the high-dimensional space of all possible emotional-somatic configurations. The dynamics move the field downhill toward local minima — the attractor basins.

Think of this as terrain. The attractor basins are valleys. The field settles naturally into whichever valley it is closest to, and tends to stay there unless external energy (a trigger, a somatic cue, a therapeutic intervention) pushes it uphill toward a ridge and over into another valley.

The therapeutic voyage is a navigation across this terrain: starting in one valley (the presenting state), moving toward another (the target state — safety, integration, the capacity for contact), crossing the ridges in between. The ridges are the thresholds. The crossing is the therapeutic event.

What the terrain looks like. For a field with two strongly coupled

modes — call them *fear* and *shame* — the landscape is a surface in three dimensions: fear on one axis, shame on another, energy on the vertical axis. The attractor is a bowl. The trauma state may have two bowls: a “fear-then-shame” sequence, and a “freeze” attractor from which shame and fear are both absent but unreachable.

For n modes, the terrain is n -dimensional. Visualisation requires projection, but the mathematics is the same regardless of dimension.

Fractal basin boundaries. When the coupling matrix W is asymmetric — when fear drives shame more strongly than shame drives fear, as is common in post-traumatic presentations — the boundary between attractor basins is not a smooth curve. It is fractal: the boundary between the “hypervigilance” basin and the “collapse” basin in a traumatised field has infinitely complex interdigitation at every scale of magnification.

The Mandelbrot set is the mathematical archetype of a fractal basin boundary: the boundary between the set and its complement is a Julia set, infinitely detailed at every scale. The fractal basin boundaries of the soma-field are of the same class. The visualisation is not merely aesthetic — the mathematics is the same.

GOING DEEPER: Fractals, Julia Sets, and Attractor Boundaries

The iteration $z \mapsto z^2 + c$ that generates the Mandelbrot set is a discrete dynamical system on the complex plane. Its attractor is the origin (the sequence converges to 0), or infinity (the sequence escapes). The boundary between the two basins is the Julia set J_c — a fractal object that, for most values of c , has non-integer (Hausdorff) dimension strictly between 1 and 2.

The soma-field is a continuous dynamical system on $\mathbb{R}^n$, not a discrete iteration on $\mathbb{C}$. But the mechanism is the same: nonlinear coupling between modes (the W_{ij} terms + the threshold nonlinearity) generates sensitivity at the boundary that propagates across scales. The Hausdorff dimension of

the boundary is a direct function of the asymmetry of W and the steepness of the threshold nonlinearity. In a severely traumatised field with highly asymmetric coupling, the boundary dimension approaches 2: the boundary is space-filling. There is, in the formal sense, no clean edge between hypervigilance and collapse. Just increasingly complex interdigitation.

Clinical implication. The fractal character of the basin boundary means that small perturbations near the threshold have disproportionate effects — the butterfly effect is concentrated at the boundary. A session conducted near a threshold crossing is qualitatively different from a session conducted well inside a basin. The geometry predicts this before any clinical experience confirms it.

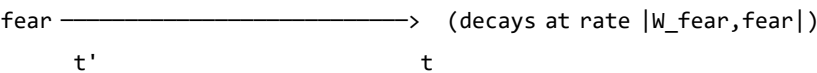
11.2 Emotions Looking for Each Other

In particle physics, interactions are drawn as Feynman diagrams: lines representing particles moving through space and time, meeting at vertices where something happens. An electron emits a photon. A quark changes flavour. Two particles scatter.

The same formalism applies to the soma-field, and the interpretation is immediate.

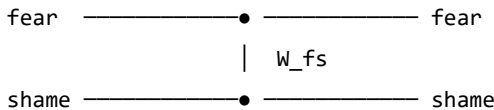
The propagator. A single emotional mode — call it *fear* — traveling through time without interacting with anything else is drawn as a single line, moving from left to right (earlier to later). The line gets fainter as time increases: the mode decays toward its equilibrium unless maintained by coupling or stimulus. This is the *propagator* — the Green's function of the free dynamics.

Single-mode propagator:



The coupling vertex. When fear and shame are coupled — $W_{\text{fear, shame}} \neq 0$ — they can scatter: fear activates shame, shame amplifies fear. This is drawn as two lines meeting at a vertex, with the coupling strength W_{ij} labeling the junction.

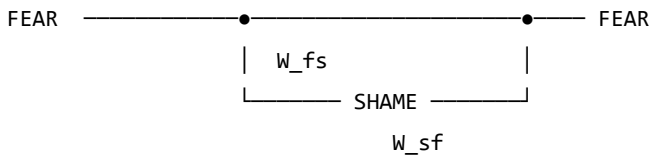
Fear-shame coupling vertex:



If $W_{\text{fear, shame}} > 0$, shame excites fear. If $W_{\text{fear, shame}} < 0$, shame suppresses fear. For the asymmetric case $W_{\text{fear, shame}} \neq W_{\text{shame, fear}}$ — one emotion drives the other more than it is driven in return — the vertex is directional. Fear leads shame in a post-traumatic field. Shame may or may not respond in kind.

Feedback loops. When fear excites shame and shame excites fear in a closed cycle, the diagram is a loop. The loop is not merely a metaphor: it is a precise mathematical object whose value — computed by integrating over the intermediate times — gives a correction to the effective coupling at the loop's characteristic timescale.

Fear-shame feedback loop:



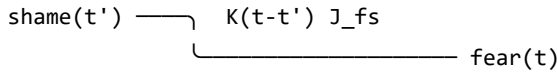
Loop correction: loop runs faster,
effective $W_{\text{fear, fear}}$ increases.

This is sensitisation.

Repeated co-activation of fear and shame — as occurs in a trauma where shame was the response to terror — consolidates the loop: the effective coupling grows. The Feynman diagram is a picture of how shame becomes a reliable trigger for fear across sessions and years.

The memory vertex. The trauma memory kernel introduces a vertex that is non-local in time: mode j at some earlier time t' contributes to mode i now, at time t , with weight $K(t - t')$. The diagram has an internal arrow going backward in time — not acausally, but in the sense that the *past state* of the field is still driving the *present state*.

Memory kernel vertex:



The shame at time t' is still driving fear now,
weighted by how much the memory kernel retains it.

For a field with a slow memory kernel (large τ_k), past activations echo far into the future. A traumatic incident twenty years ago is still driving present fear via the memory vertex — not as a belief or a narrative, but as a dynamical coupling with a specific timescale.

The instanton: the pivot. No finite collection of Feynman diagrams — no sum of scattering and loop corrections — describes a threshold crossing. The topological change from one basin to another is a *non-perturbative* event: an instanton. It is not a series of small steps; it is a qualitative transition, a jump between attractors. In physics, instantons are the events that perturbation theory cannot see. In the therapy room, they are the sessions that change something permanently.

GOING DEEPER: The Two Sectors

Every field theory divides into a *perturbative* sector (small fluctuations, describable by Feynman diagrams) and a *non-perturbative* sector (large topological events, described by instantons, solitons, and other saddle-point solutions).

The soma-field has the same division:

Sector	Events	Mathematical description
Perturbative	Emotional coupling, sensitisation, habituation, day-to-day activation	Feynman diagrams: propagators, vertices, loops
Non-perturbative	Threshold crossings, basin transitions, pivotal sessions	Instantons: minimal-action paths between basins

The perturbative sector is accessible to standard talk therapy (changing W_{ij} by desensitisation; damping memory kernel amplitudes A_k ; adjusting thresholds). The non-perturbative sector requires conditions for threshold crossing: sufficient activation energy, a safe enough container, and — often — direct somatic engagement. You cannot reach an instanton by accumulating small perturbative steps. That is the formal reason why some therapeutic approaches reach a ceiling.

11.3 The Emotional Score

A musical score is not a performance. It is the abstract structure that can be performed in many ways — by different orchestras, in different halls, at different tempos — while remaining recognisably itself. The *notes* are the invariant; the *sound* is the realisation.

A film has an emotional score. Not the music (though the music is part of its expression), but the trajectory of the emotional field that the film traces over its duration: how activation rises and falls, which modes are coupled, where the thresholds are crossed, what the final attractor state

is.

This emotional score is independent of the narrative container — the specific story in which it is realised. The same score can be realised in a river journey, a war, a marriage, a career, a therapy, or a voyage through a bloodstream.

Formally. The emotional score is a trajectory $\mathbf{e}^*(t)$ in the emotional field space, parameterised by story-time $t \in [0, 1]$ (opening to closing). A film, novel, or therapy session is:

$$\text{Realisation} = (\mathbf{e}^*(t), \text{Container})$$

The container provides the narrative surface: characters, setting, imagery, plot. The emotional score provides the dynamics: which modes activate, in what sequence, at what coupling strength.

The Conrad example. *Heart of Darkness* and its film realisation *Apocalypse Now* (Coppola, 1979) share a score: an upstream journey toward something pre-verbal, toward a figure (*Kurtz*) who represents the field’s deepest attractor — the place where normal threshold regulation has dissolved. The journey upstream is a journey toward decreasing τ_d (shorter developmental time), toward earlier, more diffuse, less differentiated emotional modes. The field becomes less structured as the journey continues. Kurtz is the attractor at the bottom of the developmental basin — not a monster, but the deepest attractor, the one with no threshold above it.

The score is: *progressive reduction of threshold distance, increasing weight of pre-verbal modes, final approach to a basin from which ordinary return is blocked*. The container (Congo river / Vietnam river) is a surface over which this score is played.

Multi-scale structure. The score has fractal structure: the same emotional pattern recurs at the level of the full film, the act, the scene, and the moment. A scene in which a character approaches and retreats from a threshold is a micro-version of the film’s macro-structure. This is not

a metaphor — the soma-field dynamics are scale-invariant near a critical point, so the same Hamiltonian structure repeats across timescales. A good filmmaker composes at all scales simultaneously.

The viewer’s field. The viewer has their own emotional field $\mathbf{e}_V(t)$ which couples to the screen signal $S(t)$:

$$\dot{\mathbf{e}}_V = -\nabla H_V(\mathbf{e}_V) + \lambda \cdot S(t) + \eta_V$$

The director controls $S(t)$ — the screen signal — but not H_V (the viewer’s own landscape). A viewer whose own field has a deep shame attractor will have a different response to the same $S(t)$ than a viewer without it. The film is the same; the voyage is different. This is the formal account of why films affect different people differently, and why re-watching a film after therapeutic work can produce a qualitatively different emotional experience: H_V has changed.

11.4 The Holographic Clinic

In theoretical physics, the holographic principle (Susskind, 1995; Bousso, 2002) states that the complete description of a volume of space can be encoded on its boundary surface, with no loss of information. A three-dimensional object is fully represented by a two-dimensional hologram. The interior is encoded in the edge.

The soma-field has a holographic structure that is clinically actionable.

The boundary. The observable boundary of the soma-field is what can be seen from outside: behaviour, posture, facial expression, reported affect, the pattern of threshold crossings in session, the rate of escalation and de-escalation, the latency between stimulus and response. This is the boundary data — the hologram.

The bulk. The interior of the soma-field is inaccessible to direct observation: the weight matrix W , the memory kernel $K(\tau)$, the effective

thresholds T_i , the attractor topology. These are the bulk fields.

The reconstruction theorem. If the boundary data is sufficiently rich — if we observe enough threshold crossings, enough coupling patterns, enough temporal correlations — the bulk fields can be reconstructed. The weight matrix W_{ij} can be estimated from the co-activation statistics of observed modes. The memory kernel time constants τ_k can be estimated from the delay between stimulus and response at different frequencies. The attractor topology can be inferred from which basins the field visits and how long it dwells in each.

The body tells you everything. This is not a therapeutic truism. It is a measurement theorem: given sufficiently rich boundary data, the full soma-field is recoverable from external observation. The body is a hologram of its own interior.

Clinical implication. A thorough intake assessment — one that tracks not just presented symptoms but response latencies, co-occurrence statistics, threshold distances, and interoceptive access — is a holographic measurement. It gives access to the bulk fields without requiring the patient to verbally report what they do not have words for. The body has been keeping a precise record. The therapist’s task is to read it.

11.5 EmotionML: Labels Without Dynamics

The W3C EmotionML standard (Schröder et al., 2011) provides a formal vocabulary for annotating emotional states in human-computer interaction. It specifies representation formats for emotion categories (anger, fear, joy, sadness...), dimensions (valence, arousal, dominance), and appraisals (novelty, intrinsic pleasantness, goal congruence). It is a well-engineered taxonomy.

It is not a dynamical theory.

EmotionML says what emotional state a system is in at time t . It does not say how that state changes, what coupling it has to other states,

what its threshold distance is, how its memory kernel drives its future evolution, or what basin transition conditions apply. It provides a label; the Soma-Field Model provides the dynamics.

The relationship is analogous to the relationship between a chemical nomenclature (naming compounds) and a rate equation (describing how compounds react). The nomenclature is necessary but not sufficient. Knowing that a patient presents as “fearful” is the EmotionML layer. Knowing the coupling $W_{\text{fear, shame}}$, the threshold T_{fear} , the memory kernel time constants, and the attractor depth is the soma-field layer. The second layer strictly includes the first.

AUTHOR’S NOTE: Why Taxonomy Is Not Enough

The history of psychiatry is largely a history of improving the taxonomy: from humours to syndromes to DSM categories to dimensional models. Each generation’s taxonomy is more precise than the previous. Each is still a taxonomy.

The shift from taxonomy to dynamics is not a refinement. It is a change of mathematical structure: from a set of labels to a vector field on a state space, with a Hamiltonian, a noise term, and a coupling matrix. The prediction capability is qualitatively different. A taxonomy tells you what something is called. A dynamical model tells you what it will do next and what it would take to change it.

EmotionML is a very good taxonomy. The Soma-Field Model is the next layer.

KEY TERMS

Navigable landscape — the Hamiltonian $H(\mathbf{e})$ understood as terrain, with attractor basins as valleys and thresholds as ridges that must be crossed.

Fractal basin boundary — the boundary between attractor basins when the coupling matrix W is asymmetric;

has non-integer Hausdorff dimension and is sensitive to perturbation at all scales.

Feynman diagram — a graphical notation for the terms in a perturbative expansion; in the soma-field, lines represent propagating modes, vertices represent couplings, and loops represent feedback cycles.

Perturbative sector — dynamics within an attractor basin, describable by Feynman diagrams; accessible to standard desensitisation and coupling-modification approaches.

Non-perturbative sector — threshold crossings and basin transitions; described by instantons; requires conditions for the full threshold-crossing event.

Instanton — a non-perturbative saddle-point solution connecting two attractor basins; in the therapy room, the pivot moment that changes the field topology.

Emotional score — the trajectory $\mathbf{e}^*(t)$ that defines a narrative's emotional structure independently of its container; formally $\text{Realisation} = (\mathbf{e}^*(t), \text{Container})$.

Holographic principle — the claim that boundary observables (behaviour, symptoms, threshold patterns) encode the full bulk fields (W , K , attractor topology); the basis for clinical assessment as measurement.

EmotionML — W3C standard for emotion annotation; provides taxonomy (labels) but not dynamics (evolution equations); a necessary but not sufficient layer.

CHAPTER SUMMARY

The Soma-Field Model provides the geometry of a voyage. The Hamiltonian landscape is the territory: attractor basins are valleys, thresholds are ridges, and the field navigates this

terrain continuously. For asymmetric coupling matrices, the basin boundaries are fractal — infinitely complex at every scale, sensitive to perturbation everywhere along the edge.

The dynamics divide into two sectors. The perturbative sector — small fluctuations within a basin — is organised by Feynman diagrams: propagators carry modes through time, coupling vertices describe interactions between modes, feedback loops describe sensitisation, and memory kernel vertices describe the echo of past activations into the present. The non-perturbative sector — threshold crossings — is described by instantons: the minimal-action paths between basins that no perturbative sum can reach.

Narratives have emotional scores: trajectories $\mathbf{e}^*(t)$ that define a film or story independently of its narrative container. The same score can be realised in a river journey, a war, or a voyage through a bloodstream. The viewer's own soma-field couples to the screen signal; what they experience depends on their own Hamiltonian.

The boundary of the soma-field encodes the bulk: behavioural observation, response latency, co-activation statistics, and threshold patterns give access to the full weight matrix, memory kernel, and attractor topology without requiring verbal report of what has no words. The body is a hologram of its own interior.

EmotionML provides the taxonomy. The Soma-Field Model provides the dynamics. Both are needed; the second strictly extends the first.

Epilogue: The T's

There are four T's in this book, and they are not accidental.

Theory — the formal structure that makes prediction possible. A theory is not a guess or an opinion. It is a precise description that can be tested, that makes specific predictions, and that says exactly what evidence would falsify it. The Soma-Field Model is a theory of emotional dynamics: it makes predictions about attractor structure, about the character of pre-verbal versus late trauma, about what parameters change in effective therapy. Whether those predictions survive contact with data is an empirical question, and the empirical work is needed.

Threshold — the parameter T , which appears in the model as the boundary between sub-threshold somatic activity and conscious emotional experience. The threshold is not a switch. It is a continuous parameter, differently set in different nervous systems, modifiable through practice and therapy. The difference between a body that feels everything and a body that feels nothing is, in formal terms, a difference in T . The therapeutic expansion of the window of tolerance is, in formal terms, a widening of the range around T within which the field can move without triggering a phase transition.

Time — the developmental time τ_d , which changes the character of a modification from perturbative (late trauma: $W = W_0 + \delta W$, a baseline plus a modification) to structural (pre-verbal trauma: $W = W_{\text{trauma}}$, the modification is the structure). Time also appears in τ_k — the decay time of the memory kernel, how long an echo persists. Therapy changes τ_k . The passage of time, in the absence of intervention, does not reliably change τ_k for pre-verbal somatic traces.

Transformation — the fourth T, the one that this book is about, finally. Not recovery. Not return. Not the restoration of a prior state. The construction of a new landscape: wider, more flexible, with deeper calm and shallower hypervigilance, arrived at from where the system is, going somewhere it has not been before.

There is a fifth T, which gave this book its title: **Trance** — in two senses. The first: the altered state at the threshold, the phase transition, the field crossing a boundary and remaining in the other phase. Trance is not a malfunction; it is a dynamic state, momentarily ungoverned by the usual attractors. In those moments, something is possible that is not possible from within a stable phase.

The second sense is the title itself. *A Voyage into Trance* (1995) is a Goa trance compilation by Paul Oakenfold. The trance state produced by extended rhythmic music and the freeze response of a traumatised nervous system are not the same experience. They are governed by the same mathematics. Both drive an arousal variable across a threshold and sustain it there. Music does this deliberately; trauma does it involuntarily. The mathematics does not distinguish.

The voyage into trauma is not a straight line. It passes through all five T's, sometimes in order, sometimes not.

The model is the map. The body is the territory. The voyage is yours.

Appendices

Appendix A: The Mathematics in Full

The following is a condensed version of the academic paper Soma-Field Model of Emotional Dynamics and C-PTSD for readers who want the formal derivations. Full derivations, Lean 4 type sketches, and bibliography are in the companion document `soma-field-paper.md`.

A.1 The Hamiltonian

$$H(\mathbf{e}) = -\frac{1}{2} \mathbf{e}^\top W \mathbf{e} - \theta^\top \mathbf{e}$$

where $W \in \mathbb{R}^{n \times n}$ is the coupling matrix and $\theta \in \mathbb{R}^n$ is the threshold bias vector.

A.2 The Dynamics

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \sigma_0 \eta(t) = W \mathbf{e} + \theta + \sigma_0 \eta(t)$$

where $\eta(t)$ is white noise with $\langle \eta_i(t) \eta_j(s) \rangle = \delta_{ij} \delta(t-s)$.

A.3 The C-PTSD Modification

$$W_{\text{C-PTSD}} = W_0 + \Delta W, \quad \Delta W_{ij} \neq \Delta W_{ji}$$

The asymmetry of ΔW breaks the gradient flow property and introduces directional cycles in the landscape.

A.4 The Memory Kernel

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \int_0^t K(t-s) \mathbf{e}(s) ds + \sigma_0 \eta(t)$$

$$K(\tau) = \sum_k A_k e^{-\tau/\tau_k}$$

A.5 Developmental Time Parameterisation

$$W(\tau_d) = f(\tau_d) \cdot W_0 + (1 - f(\tau_d)) \cdot W_{\text{trauma}}$$

$$f(\tau_d) = \tanh\left(\frac{\tau_d}{\tau_c}\right), \quad \tau_c \approx 36 \text{ months}$$

A.6 The QFT Correspondence

Under the Wick rotation $t \rightarrow -i\tau$:

QFT	Soma-Field
$G_E(\tau) = \frac{1}{2m} e^{-m \tau }$	$K(\tau) = \sum_k A_k e^{- \tau /\tau_k}$
Field mass m	Inverse decay time $1/\tau_k$
H_{Ising}	H_{soma}

Appendix B: Lean 4 Type Sketches

The following are proof sketches in Lean 4. `sorry` marks open proof obligations.

-- Core soma-field structures

structure CouplingMatrix (n : ℕ) where

W : Matrix (Fin n) (Fin n) ℝ

θ : Fin n → ℝ

structure SomaField (n : ℕ) where

e : Fin n → ℝ -- current activation vector

W : CouplingMatrix n

T : ℝ -- threshold parameter

sigma : ℝ -- noise amplitude

-- The Hamiltonian

noncomputable def hamiltonian {n : ℕ} (W : CouplingMatrix n) (e : Fin n → ℝ) : ℝ

```

-0.5 * Matrix.dotProduct (Matrix.mulVec W.W e) e
- Matrix.dotProduct W.0 e

-- C-PTSD modification: asymmetric coupling
def isCPTSDModified {n : ℕ} (W : CouplingMatrix n) : Prop :=
  ∃ i j, W.W i j ≠ W.W j i

-- Developmental time parameterisation
structure TraumaProfile (n : ℕ) where
  τ_d      : ℕ
  asymmetry : Matrix (Fin n) (Fin n) ℕ
  amplitudes : List (Fin n → ℕ)
  decayTimes : List (Fin n → ℕ)

def τ_c : ℕ := 36

noncomputable def structuralFraction (τ_d : ℕ) : ℝ :=
  Real.tanh (τ_d / τ_c)

-- For pre-verbal trauma: structural fraction < tanh(1) ≈ 0.76
theorem preVerbalIsStructural {n : ℕ} (profile : TraumaProfile n)
  (h : profile.τ_d < τ_c) :
  structuralFraction profile.τ_d < Real.tanh 1 := by
  unfold structuralFraction
  apply Real.tanh_lt_tanh
  exact div_lt_one_of_lt h (by norm_num)

```

Appendix C: The Cross-Language Correspondence Table

Mathematical language	Emotional dynamics
Symmetric monoidal category $\mathcal{C}$	Soma-field operator algebra

Mathematical language	Emotional dynamics
Object $A \in \mathcal{C}$	Emotional mode type
Morphism $f : A \rightarrow B$	Field operator (maps one mode to another)
Tensor product $A \otimes B$	Simultaneous activation of modes A and B
Composition $g \circ f$	Sequential field operations
Identity morphism id_A	Identity (mode persists unchanged)
Braiding $\sigma : A \otimes B \cong B \otimes A$	Mode-order independence of simultaneous states
Feynman vertex	Emotional interaction (coupling W_{ij})
Loop diagram	Memory kernel (self-coupling over time)
Feynman propagator	Memory trace decay $e^{- \tau /\tau_k}$
Vacuum state	Resting soma-field (minimal activation)
Partition function Z	Field normalisation (probability distribution over states)
Renormalisation group flow	Therapeutic modification of coupling constants
Phase transition	Polyvagal state transition (ventral/sympathetic/dorsal)
Symmetry breaking	Asymmetric W (C-PTSD modification)

The correspondences in this table are not analogies. They are identifications of the same mathematical object in two notation systems, established by the Baez–Lauda coherence theorem (2011) for the categorical column and the Wick rotation for the QFT column.

Appendix D: Glossary

Amplitude A_k — The strength of a trauma memory trace’s influence on the current soma-field. Reduced by effective somatic therapy.

Attractor — A stable state in the energy landscape; a position toward which the soma-field naturally moves from nearby states.

Basin of attraction — The region of state space from which the field flows toward a given attractor.

C-PTSD operator — The modification ΔW to the coupling matrix that represents the effect of complex developmental trauma on the soma-field landscape.

Co-regulation — The process by which the soma-field of one person influences another through relational coupling; the somatic mechanism of relational healing.

Coupling matrix W — The matrix encoding the interactions between emotional modes; determines the shape of the energy landscape. Symmetry of W guarantees stable attractors.

Damping γ — The rate at which the field returns toward attractors after perturbation. Low damping (ADHD) produces rapid, wide-ranging field dynamics.

Decay time τ_k — How long a trauma memory trace persists before fading. Pre-verbal traces typically have longer decay times.

Developmental age at trauma τ_d — The age in months at which the primary traumatic modification occurred. Determines whether the modification is perturbative (late trauma) or structural (pre-verbal trauma).

Effective temperature — The ratio σ_0^2/γ ; determines how widely the soma-field explores the landscape relative to the depth of the attractors.

Forward transformation — The construction of a new coupling matrix W' with desired dynamical properties, as the correct therapeutic goal for pre-verbal trauma (as opposed to recovery of a prior baseline).

Hamiltonian H — The energy function that assigns a value to every possible soma-field state; determines the landscape's hills and valleys and thus the dynamics.

Interoception — The nervous system's process of sensing the body's internal state.

Interoceptive accuracy α — The precision with which a person can read their own soma-field state. Disrupted by trauma; improvable through training and therapy.

Memory kernel $K(\tau)$ — The function describing how past field activations continue to influence the current state. In C-PTSD: a sum of decaying exponentials.

Noise amplitude σ_0 — The magnitude of random fluctuations in field dynamics. Elevated in ADHD; reduced by breath and autonomic regulation.

Phase transition — A qualitative reorganisation of the field's state at a critical parameter value. Polyvagal state changes (e.g., ventral vagal to freeze) are phase transitions, not gradual changes.

Soma — The body as experienced from the inside; the totality of interoceptive signals.

Soma-field — The vector $\mathbf{e}$ of somatic activation levels across emotional modes; the state of the body's emotional field at a given moment.

Structural fraction $f(\tau_d)$ — The proportion of the coupling matrix attributable to neurotypical baseline development versus trauma-formed modification.

Threshold T — The activation level above which a soma-field mode becomes conscious experience. The boundary between felt emotion and sub-threshold somatic activation.

Verbal encoding threshold τ_c — The approximate developmental age (36 months) at which narrative memory and verbal encoding capacity reliably emerges.

Wick rotation — The substitution $t \rightarrow -i\tau$ that transforms oscillatory quantum dynamics into real-time thermal/stochastic dynamics; the bridge connecting QFT propagators to soma-field memory kernels.

Window of Tolerance — The range of arousal within which the nervous system can function flexibly, process information, and engage socially.

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A Voyage into Trauma: The Soma-Field Theory of Emotional Life First edition, 2026. Companion academic paper: soma-field-paper.md

Listening Notes

This book was written in a single session on the night of 16–17 May 2026.

The development was set to *Silver Machine* by Hawkwind. It closed with *It's So Easy* by Hawkwind.

Both choices were correct.

Interlude: The Tensor — A Film in Fields

The Tensor

An Abstract Film Definition

This is not a screenplay. It contains no dialogue, no character names, no scene headings, no camera directions. It cannot be read to an actor or handed to a set designer. It describes a film the way a musical score describes a performance — as an abstract structure that can be realised in many ways, by many different instruments, for many different audiences.

The film is defined as a trajectory through the emotional field. The rendering — the actual pixels and samples the viewer experiences — is generated at runtime from this trajectory, from the viewer’s own soma-field state, and from a set of control parameters. Two viewers watching the same film may hear different music. In the limit where the viewer’s own biofeedback is available, they may traverse the trajectory differently — the film meets them where they are.

The territory is the body. The voyage is inward.

Part I: The Format

1. The Emotional Score

A film is defined by its **emotional score**: a vector-valued trajectory

$$\mathbf{e}^*(t) = (e_1^*(t), e_2^*(t), \dots, e_n^*(t))$$

parameterised by story-time $t \in [0, 1]$ (opening to closing). Each component $e_k^*(t)$ is the intended activation of emotional mode k at story-moment t .

The score is **not** what the viewer feels. It is what the film proposes — the director’s instruction to the rendering system. Whether the viewer’s

field resonates with the proposal depends on their own Hamiltonian H_V .
 The standard mode vocabulary for this project uses seven primary axes:

Mode	Symbol	Description
Safety	e_S	Regulation, groundedness, ventral vagal tone
Fear	e_F	Threat activation, mobilisation
Shame	e_{Sh}	Social evaluation, self-concealment
Grief	e_G	Loss, withdrawal, parasympathetic collapse
Curiosity	e_C	Approach, exploration, openness
Awe	e_A	Threshold-adjacent wonder; dissolution of self-boundary
Language	e_L	Symbolic, conceptual, narrative organisation

Additional modes can be added per score. Pre-verbal affect, disgust, rage, and the somatic marker of HRV coherence may all appear as named axes.

2. Threshold Events

At specified story-times t_k , the score may declare a **threshold crossing** — a non-perturbative event in which the emotional field transitions between attractor basins. These are not smooth changes of $\mathbf{e}^*(t)$; they are instantons.

A threshold event is declared as:

THRESHOLD $t = 0.58$ FROM: [hypervigilance] TO: [awe]

condition: $e_F > 0.7$ AND e_A rising
duration: 0.04 (narrow window)

The rendering system must hold the score near the threshold approach for as long as necessary until the crossing condition is met — whether by the score’s internal dynamics or by the viewer’s biofeedback signalling readiness.

3. Control Knobs

The score is rendered through a set of **control parameters** κ that the viewer, clinician, or runtime system can adjust. These are continuous dials, not binary switches.

Knob	Symbol	Effect
Depth	$\kappa_d \in [0, 1]$	How far the instanton descends into the pre-verbal attractor. At $\kappa_d = 0$, threshold crossings are shallow; at $\kappa_d = 1$, the full instanton trajectory is traversed.
Velocity	$\kappa_v \in [0.1, 3]$	Clock multiplier for story-time. $\kappa_v < 1$: expanded, slower passage. $\kappa_v > 1$: compressed.

Knob	Symbol	Effect
Resonance	$\kappa_r \in [0, 1]$	Weight of viewer biofeedback in modulating the score. At $\kappa_r = 0$: pure projection. At $\kappa_r = 1$: the score is entirely driven by the viewer's field (the film becomes a mirror).
Texture	$\kappa_t \in [0, 1]$	Audio/visual granularity. Low: smooth, tonal, harmonic. High: granular, fractal, noisy. Maps to noise level σ_{eff} in the rendering.
Mode mask	$\kappa_m \subseteq \{1..n\}$	Which emotional modes are active in this rendering. A viewer without a shame attractor may have <i>Sh</i> masked; the score is rendered without that channel.

Knob	Symbol	Effect
Coupling scale	$\kappa_W \in [0.5, 2]$	Global scale on the coupling matrix W^* of the score. High values increase inter-mode interaction; the emotional landscape becomes more complex and entangled.

4. The Rendering Function

The screen signal $S(t)$ — the actual audio and visual output — is:

$$S(t) = \mathcal{R}(\mathbf{e}^*(t), \kappa, \mathbf{e}_V(t))$$

where:

- $\mathbf{e}^*(t)$ is the abstract score
- κ is the control parameter vector
- $\mathbf{e}_V(t)$ is the viewer’s own emotional field (measured or inferred)
- $\mathcal{R}$ is the **rendering function** — the audio/visual synthesis engine

The rendering function maps emotional-field coordinates to audio parameters (frequency, harmonic content, tempo, grain density, spectral centroid, reverb depth) and visual parameters (fractal dimension, colour temperature, edge sharpness, motion speed, light level). The mapping is specified per rendering implementation; the score is independent of any specific renderer.

5. The Somatic Loop

When the viewer’s field $\mathbf{e}_V(t)$ is available — via HRV monitor, skin conductance, posture sensor, or simply therapist observation — the system closes a **somatic loop**.

Of the available biofeedback signals, **cardiac acceleration** $\dot{H}(t)$ (beats/s²) is the most predictively useful. Current BPM tells the system where the viewer’s cardiac field *is*; $\dot{H}$ tells it where the field is *going* — the N+1 state. A rising heart rate ($\dot{H} > 0$) predicts threshold approach and may trigger the system to hold at a pre-threshold moment in the score, or to soften texture and deepen resonance to meet the viewer where they are heading. A decelerating heart rate ($\dot{H} < 0$) signals return and may allow the score velocity to increase. The rendering system should treat $\dot{H}$ as the primary cardiac control signal and instantaneous BPM as a secondary state indicator.

The system

$$\dot{\mathbf{e}}_V = -\nabla H_V(\mathbf{e}_V) + \kappa_r \cdot \lambda \cdot S(t) + \eta_V$$

The screen signal $S(t)$ drives the viewer’s field; the viewer’s field modifies $S(t)$ via the resonance knob κ_r and the rendering function $\mathcal{R}$. At high resonance, the film and the viewer co-regulate. The distinction between “watching a film” and “being in a therapy” begins to dissolve.

Two operating modes:

Mode	κ_r	Description
Projection	≈ 0	The score drives the viewer. Classical cinema: fixed score, passive audience.

Mode	κ_T	Description
Resonance	≈ 0.5	Score and viewer co-determine the output. Biofeedback cinema: the film breathes with the viewer.
Mirror	≈ 1	The viewer's field drives the rendering. The score becomes a target trajectory; the system generates audio/visual content that guides the viewer toward $\mathbf{e}^*(t)$ from wherever they actually are.

In Mirror mode, the system is a **real-time emotional score calibrator**: it continuously measures $\mathbf{e}_V(t)$, computes the gap to $\mathbf{e}^*(t)$, and renders audio/visual content calculated to reduce that gap. This is a formal definition of what a therapist does.

Part II: The River Film

A score. Not a story.

The following is the abstract definition of a film. Its narrative container is a river journey: upstream away from civilisation, toward something older and less organised, then back. The container is not the film. Another realisation of the same score might use a descent into a cave, a journey into psychosis, a session of deep somatic therapy, or a voyage through a bloodstream in a miniaturised submarine. The score is invariant. The river is one surface over which it is played.

Score Parameters

TITLE: The River Film (working title)
DURATION: t in [0, 1] (maps to approximately 90 minutes at kappa_v = 1.0)
PRIMARY MODES: Safety, Fear, Curiosity, Awe, Grief, Language, Pre-verbal
THRESHOLD EVENTS: 2 (at t = 0.52 and t = 0.74)
DEFAULT KAPPA: depth=0.7, velocity=1.0, resonance=0.0, texture=0.4, coupling_scale=1.0

Emotional Trajectory

The seven primary modes over story-time $t \in [0, 1]$:

EMOTIONAL SCORE: THE RIVER FILM
Scale: 0 (silent) → 9 (full activation) Resolution: 0.1 story-time units

	0.0	0.1	0.2	0.3	0.4	0.5	0.6	0.7	0.8	0.9	1.0
SAFETY	9	8	7	5	3	2	1	≠ 2	4	7	9
CURIOSITY	3	5	7	8	7	5	3	2	4	6	5
FEAR	1	1	2	3	5	7	≠ 4	2	2	1	1
AWE	1	1	1	2	3	4	6	9	7	4	2
GRIEF	1	1	1	1	2	3	4	4	6	4	2

LANGUAGE	9	9	8	7	5	3	1	≠ 1	3	7	9
PREVERBAL	1	1	1	2	3	5	7	9	6	3	1

≠ = threshold crossing event

THRESHOLD 1 at $t \approx 0.52$: Safety<2, Fear>7 → AWE begins rise (field tips)

THRESHOLD 2 at $t \approx 0.71$: Language<1, PREVERBAL≥9 → GRIEF opens fully
(the encounter; the deepest attractor)

Phase Descriptions

Phase 1: Departure $t \in [0, 0.25]$

Safety is high; the field is organised. Language is dominant — the viewer is still thinking in sentences. Curiosity rises: there is something upstream. Fear is present but low, a background hum. The rendering is harmonic, tonal, structured. Tempo is regular. Visual: clear light, organised geometry, recognisable forms.

In the river container: the boat leaves the last town. The last road disappears behind the tree line. The journey has begun.

Phase 2: Descent $t \in [0.25, 0.52]$

Safety falls steadily. Curiosity peaks and begins to fall. Fear rises. Language degrades — the score calls for less and less conceptual organisation. Pre-verbal modes begin their ascent. The field is approaching the threshold.

The audio rendering: harmonic content decreases, spectral centroid drops, grain size increases (κ_t increases internally with e_{PV}). The music becomes less music and more texture. Tempo irregularity increases. Visual: light dims, edges soften, forms become ambiguous, fractal structure begins to emerge in peripheral detail.

In the river container: the river narrows. The current strengthens. The vegetation becomes unrecognisable. Something that was navigable is becoming something that is navigating you.

Threshold 1 $t \approx 0.52$

The first instanton. Safety < 2 , Fear > 7 . The field tips. This is the moment when fear passes its threshold into something larger: the beginning of awe. The two are close — they activate the same somatic substrate. The difference is the interpretation. The rendering system holds here until the crossing completes.

In the river container: the moment you cannot go back. Not a decision. A discovery.

Phase 3: The Deep River $t \in [0.52, 0.74]$

Awe rises toward maximum. Fear falls — it has been superseded, not resolved. Language approaches silence. Pre-verbal is dominant. Safety is minimal. The field is at the bottom of the developmental axis — the oldest, most diffuse, most somatic registers of experience. The music, if it still deserves that name, is almost entirely noise and texture and rhythm — rhythm because the heartbeat persists where nothing else does.

The visual rendering at $e_{PV} = 9$: pure fractal. Mandelbulb parameters driven entirely by the emotional modes. Self-similar at every scale. No recognisable objects. Colour driven by e_A (awe) and e_G (grief) — the pairing of wonder and loss that characterises the deepest attractors.

In the river container: the encounter. Whatever Kurtz is. Whatever the heart of darkness is. It does not speak in sentences. It does not need to.

Threshold 2 $t \approx 0.74$

The second instanton. Language = 0, Pre-verbal = 9. The encounter. This threshold does not go to a higher activation — it goes to a deeper quality. Grief opens fully: not sadness, but the affect of having arrived at the oldest loss, the one that precedes memory. The field is in a state that has no name in any clinical taxonomy. It has only a position in the field.

The rendering system may pause here. At high κ_r , the viewer's own biofeedback determines when this phase ends.

Phase 4: Return $t \in [0.74, 1.0]$

The journey reverses. But not to the same place. The return is asymmetric: the basin topology has changed. Safety rises, but along a different path. Language returns, but to describe something it could not have described at the outset. Curiosity does not return to its Phase 1 character — it is now the curiosity of someone who has seen something. Grief persists longer than expected; it is the last mode to settle.

The audio rendering: gradual return of harmonic structure, but with residual grain. The music has been changed by what happened in Phase 3. A tonal structure that carries the memory of noise.

In the river container: the river widens. Light returns. Towns appear on the bank. The world has not changed. You have.

Part III: The Rendering Architecture

Audio Rendering

The emotional score maps to audio parameters through a continuous, differentiable function. The following mapping is a reference implementation; specific renderers may use different functions so long as the monotonicity and qualitative character of each mapping is preserved.

AUDIO RENDERING MAP (reference implementation)

Emotional mode	→ Audio parameter(s)
Safety (e_S)	→ Fundamental pitch stability; reverb decay time (high safety = long, stable reverb; low = short, dry)
Fear (e_F)	→ Harmonic tension; tritone content; spectral irregularity
Curiosity (e_C)	→ Melodic motion; register expansion; rhythmic anticipation
Awe (e_A)	→ Dynamic range; spatial width; harmonic overtone richness
Grief (e_G)	→ Descending melodic tendency; sub-bass presence; tempo drop
Language (e_L)	→ Harmonic coherence; rhythmic regularity; tonal centre strength
Pre-verbal (e_PV)	→ Grain density (granular synthesis parameter); spectral noise; rhythm de-synchronisation from fixed grid
Coupling scale (kappa_W)	→ Cross-mode harmonic interference (dissonance from coupling)
Texture (kappa_t)	→ Overall grain size; spectral smear
Velocity (kappa_v)	→ Clock rate; effective tempo multiplier

In a Phase Plant implementation: each emotional mode drives a macro knob. Macros modulate synthesis parameters across all generators. At $e_{PV} = 9$, the granular engine is at maximum grain randomness; at $e_{PV} = 1$, it is silent or running at maximum coherence. The complete score trajectory is an automation lane for each macro.

Personalisation. Different users hear different music because:

1. κ_m may exclude modes they do not have active attractors for

2. $\kappa_r > 0$ allows their own $\mathbf{e}_V(t)$ to modulate the rendering in real time
3. κ_d scales the depth of the instanton traversal — some users may not be ready for $\kappa_d = 1.0$ and the system (or clinician) sets it lower
4. The rendering function $\mathcal{R}$ may be calibrated to the individual's own mode vocabulary — their specific fear-to-shame coupling, their specific grief timescale

Two people hearing the same score may hear music that is recognisably related — same structure, same threshold events, same overall arc — but with different timbres, different depths, different durations at the instanton.

Visual Rendering

The abstract visual rendering drives a fractal or generative system. The reference implementation uses a Mandelbulb renderer with the following parameter mapping:

VISUAL RENDERING MAP (reference implementation)

Emotional mode	→ Visual parameter(s)
Safety (e_S)	→ Light level; warm colour temperature (high K value)
Fear (e_F)	→ Edge contrast; cold hue shift; motion speed
Curiosity (e_C)	→ Zoom velocity; camera path exploration radius
Awe (e_A)	→ Mandelbulb power parameter (2→8: more complex geometry)
Grief (e_G)	→ Desaturation; slow orbital camera; depth of field
Language (e_L)	→ Structural regularity; recognisable geometric forms
Pre-verbal (e_{PV})	→ Fractal iteration depth; self-similarity at fine scales;

dissolution of object-level forms

At $e_{PV} = 9$, $e_L = 0$: the visual is a deep Mandelbulb zoom at high iteration depth, fully abstract, no edges that resolve into recognisable shapes. The image is entirely self-referential — a structure that contains

only itself.

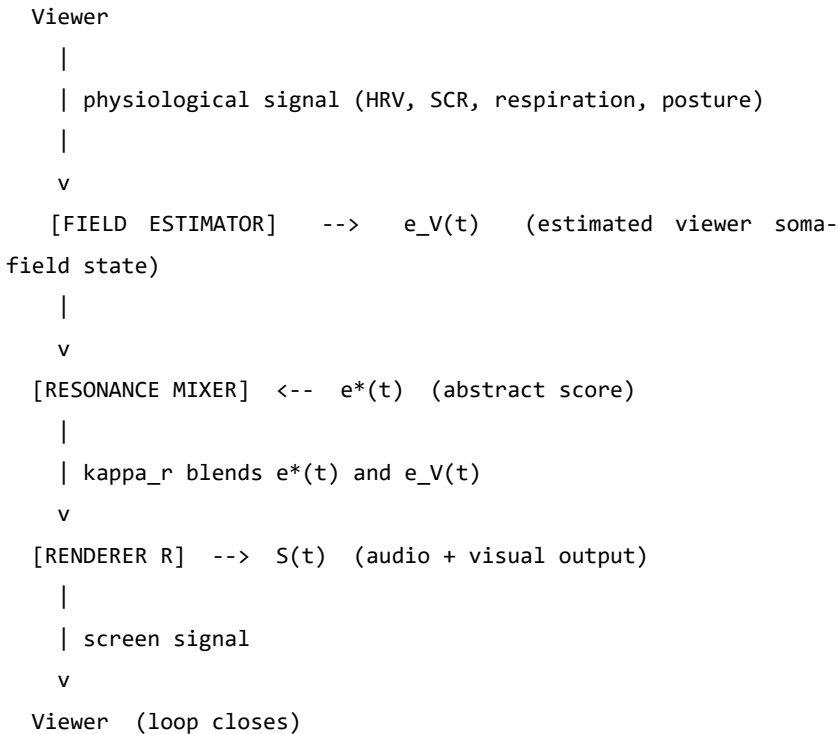
At $e_S = 9$, $e_L = 9$: the visual is clear, geometrically organised, warm. A landscape that makes sense.

The emotional score determines which of these states the visual system is in at each moment of the film.

The Somatic Loop: Biofeedback Integration

If the viewer wears an HRV monitor or similar:

SOMATIC LOOP ARCHITECTURE



At $\kappa_r = 0$: the viewer's field does not affect the output. Standard cinema.

At $\kappa_r = 0.5$: the film breathes with the viewer. If the viewer enters a freeze state at the threshold approach, the score velocity slows, the

texture softens, the system waits. When the viewer's HRV coherence returns, the threshold crossing is attempted again.

At $\kappa_r = 1.0$: the film is a mirror. The audio and visual content is generated entirely from $\mathbf{e}_V(t)$. The abstract score $\mathbf{e}^*(t)$ functions only as a *target trajectory* — an attractor for the viewer's field. The rendering system continuously generates content designed to guide $\mathbf{e}_V$ toward $\mathbf{e}^*$. This is a formal implementation of therapeutic presence.

Part IV: Extensions and Applications

The Trilogy of Containers

The River Film score can be realised in at least three containers without changing a single value in the score definition:

Container	Setting	Kurtz / deep attractor
River	Congo / Mekong / Amazon	The upriver figure; the place without language
Body	Miniaturised submarine in bloodstream	Heart chamber; the oldest immune memory
Session	Psychotherapy room	The moment the freeze lifts

All three are the same film. All three cross the same two thresholds at the same story-times. All three return along the asymmetric path. The rendering renders all three identically — because the score is what is being rendered, not the container.

Composing with the Score

A composer working with this system does not write notes. They write trajectories. The compositional decisions are:

1. **Which modes** are the primary axes of this piece?
2. **What is the arc** — the shape of each trajectory over story-time?
3. **Where are the thresholds** — the instanton events?
4. **How deep** is the deepest attractor? (What does $\kappa_d = 1.0$ sound like here?)
5. **What is the return topology** — does the field return to where it started, or is the return basin different from the departure basin?

A film with the same departure and return basins (safety at $t = 0$ safety at $t = 1$) is a round trip. Most therapy sessions are not round trips. The return basin is reorganised: higher HRV coherence, lower default coupling between fear and shame, wider threshold distance from the freeze attractor. The score should reflect this — the return is not a reversal of the departure, but a different path to a different version of home.

String Diagrams as Score Notation

For multi-character scores — where the coupling between multiple viewer fields is part of the composition — string diagrams provide the notation. Each wire is a soma-field. Each box is an interaction. Composition (two boxes in sequence) is a temporal sequence of interactions. Tensor product (two wires in parallel) is simultaneous independent activation.

A therapy dyad is two wires through time, with coupling boxes at the points of co-regulation. A film audience is N parallel wires, each with their own H_V , all coupling to the same screen signal $S(t)$. The emotional score is the abstract specification of what $S(t)$ does. The audience’s collective response is the tensor product of N individual trajectories, all shaped by the same source.

The Tensor Trilogy

This document is part of a three-part project:

Document	Register	Full title
soma-field-paper.md	Academic	<i>The Soma-Field Model</i> (The Tensor II)
soma-field-book.md	Accessible	<i>A Voyage into Trauma</i> (The Tensor III)

Document	Register	Full title
the-tensor.md	Operational	<i>The Tensor</i> — abstract film definition

The paper defines the model. The book explains the model. This document **runs** the model — or more precisely, defines the interface by which an audio-visual rendering system can instantiate the model as a real-time experience.

The Pensieve Problem

In *Harry Potter*, Dumbledore uses his wand to extract a thought from his mind — it emerges as a silvery thread — and deposits it in a stone basin called the Pensieve. Others can then lower their face to the surface and enter the memory, experiencing it from within.

This is serialisation of mental state: a running process (a memory, currently executing in a living mind) extracted and written to persistent storage, then deserialised at a later time by a different reader.

The soma-field score is a Pensieve for emotional dynamics. The wand is the measurement system (HRV, therapist observation, biofeedback). The silvery thread is the score file $\mathbf{e}^*(t)$, the coupling matrix W^* , the memory kernel K^* . The Pensieve basin is the rendering system.

But the soma-field score is strictly more powerful than Dumbledore’s basin:

	Pensieve	Soma-field score
What is serialised	Memory content — the specific events and images	Emotional dynamics — the field shape, attractor topology, coupling strengths

	Pensieve	Soma-field score
Replay	Fixed; same experience for every viewer	Rendered through viewer's own H_V ; personalised without losing the score's identity
Viewer's role	Passive observer inside a fixed recording	Active field participant; at $\kappa_r = 1$, co-author of the rendering
Storage unit	A specific thought	The emotional <i>shape</i> — valid for any narrative container with the same dynamics

Dumbledore stores what happened. The soma-field stores what it felt like to be in that basin — decoupled from the specific narrative content, portable across containers, renderable by a different nervous system in a different century.

The technical word for what both systems do is **serialise**: to take a running process that exists only in real time and write it to a durable, transmittable format. The poetic word is **crystallise** — to fix something fluid into a reproducible form without destroying its essential structure.

We are crystallising emotional experience. Not the story. Not the images. The mathematics underneath all stories and all images that have the same emotional shape. That is what the score file contains. That is what the rendering system reads back.

Appendix: Score File Format

A machine-readable score would be expressed as follows. This is a sketch of the format; a full specification is a separate engineering document.

score:

title: "The River Film"

version: "0.1"

modes:

- id: S name: Safety range: [0, 1]
- id: F name: Fear range: [0, 1]
- id: C name: Curiosity range: [0, 1]
- id: A name: Awe range: [0, 1]
- id: G name: Grief range: [0, 1]
- id: L name: Language range: [0, 1]
- id: PV name: Pre-verbal range: [0, 1]

coupling:

W_{ij}: mode j drives mode i

- from: F to: A weight: +0.4 *# fear can tip into awe near threshold*
- from: A to: G weight: +0.3 *# awe opens grief*
- from: L to: PV weight: -0.6 *# language suppresses pre-verbal*
- from: PV to: L weight: -0.6 *# pre-verbal suppresses language*

keyframes:

story-time: [S, F, C, A, G, L, PV]

```
0.0:      [0.90, 0.10, 0.30, 0.10, 0.10, 0.90, 0.10]
0.1:      [0.80, 0.10, 0.50, 0.10, 0.10, 0.90, 0.10]
0.2:      [0.70, 0.20, 0.70, 0.10, 0.10, 0.80, 0.10]
0.3:      [0.50, 0.30, 0.80, 0.20, 0.10, 0.70, 0.20]
0.4:      [0.30, 0.50, 0.70, 0.30, 0.20, 0.50, 0.30]
0.5:      [0.20, 0.70, 0.50, 0.40, 0.30, 0.30, 0.50]
0.52:     [THRESHOLD_1]
0.6:      [0.10, 0.40, 0.30, 0.60, 0.40, 0.10, 0.70]
0.7:      [0.10, 0.20, 0.20, 0.90, 0.50, 0.05, 0.90]
```

```
0.74:      [THRESHOLD_2]
0.8:       [0.20, 0.10, 0.30, 0.70, 0.60, 0.20, 0.60]
0.9:       [0.50, 0.10, 0.50, 0.40, 0.40, 0.60, 0.20]
1.0:       [0.90, 0.10, 0.50, 0.20, 0.20, 0.90, 0.10]
```

thresholds:

```
- id: T1
  t: 0.52
  from_basin: [approach, hypervigilance]
  to_basin: [awe-onset]
  condition: "F > 0.7 AND A rising"
  instanton_depth: kappa_d
  hold_until_ready: true

- id: T2
  t: 0.74
  from_basin: [awe-onset]
  to_basin: [encounter]
  condition: "L < 0.1 AND PV > 0.85"
  instanton_depth: kappa_d
  hold_until_ready: true
```

defaults:

```
kappa_d: 0.70
kappa_v: 1.00
kappa_r: 0.00
kappa_t: 0.40
kappa_W: 1.00
```

The Tensor. 17 May 2026.

Part II: The Formal Apparatus

AI has had a brain since 1943. Now it has a body.

Introduction

A patient sits with their therapist and is asked: “*What are you feeling right now?*” The question is deceptively simple. They may say *anxious*, yet that word covers a vast and heterogeneous territory — a tightness in the chest, a running commentary of worry, a vague readiness to flee, a memory surfacing from childhood. Another patient, asked the same question, reports feeling nothing at all; and yet their posture, respiration, and the quality of their silence suggest otherwise. The emotion is there. It is simply not yet conscious.

This gap between emotional presence and emotional awareness is one of the most clinically significant phenomena in psychotherapy. Theories of affect regulation (Schore, 2001), somatic experiencing (Levine, 2010), sensorimotor psychotherapy (Ogden, Minton & Pain, 2006), and polyvagal theory (Porges, 2011) all grapple, in different ways, with the same observation: emotions exist in the body before — and often without — being named in the mind. Eugene Gendlin called the sub-verbal bodily sense of an emotional situation the *felt sense* (Gendlin, 1978): something that is there, whole and present, but not yet articulate.

The Soma-Field Model proposed here attempts to give this clinical observation a formal structure. It does so by borrowing a conceptual tool from physics: the field. In physics, a field is not a thing that exists at a point. It is a quantity that exists everywhere in a space, continuously, whether or not it is observed. Particles — the things we can measure —

are not separate from the field; they are *excitations* of it, local concentrations of energy that arise when the field is perturbed above a certain threshold.

The central claim of this paper is that this structure accurately describes the phenomenology of emotion. The emotional field is always there, distributed across body and nervous system. What we call a conscious emotional experience is an excitation of that field — a local concentration that has crossed a perceptual threshold and entered awareness. The field continues below the threshold whether or not we attend to it, and its sub-perceptual activity shapes our behaviour, physiology, and cognition continuously.

The Soma-Field Model contributes the first formal field-theoretic architecture for the limbic system. Every artificial neural network since McCulloch and Pitts (1943) (McCulloch & Pitts, 1943) is a formal model of the neocortex — the pattern-recognition and prediction layer. The limbic system — responsible for emotional valuation, threat detection, and the somatic state reinstatement that underlies trauma — has never received a comparable formal treatment. The Soma-Field Model is that treatment. Together with the Hopfield framework, it constitutes the first complete formal description of the two principal computational substrates of the vertebrate brain.

The paper proceeds as follows. Section 2 reviews the relevant background in somatic clinical models, and introduces the two theoretical tools borrowed from physics and computer science: quantum field theory and Hopfield network energy functions. Section 3 develops the Soma-Field Model in detail. Section 4 describes the energy landscape, including the attractor states corresponding to fight, flight, freeze, and regulated calm. Section 5 discusses dissonance and resolution as mechanisms of emotional interaction. Section 6 describes the Soma-Field Instrument, a practical tool for therapeutic use. Section 7 addresses clinical implications.

Background

The Body-Mind Problem in Clinical Practice

Contemporary neuroscience has largely dissolved the Cartesian boundary between body and mind. Damasio (1994) demonstrated that emotion is inseparable from rational cognition: patients with damage to the ventromedial prefrontal cortex — preventing the normal generation of somatic signals — lose not only their emotional range but also their capacity for effective decision-making. Van der Kolk (2014) documented extensively how traumatic emotional states are encoded not merely in explicit memory but in posture, gesture, visceral sensation, and autonomic regulation. Porges’ polyvagal theory (2011) provided a neurobiological account of how the autonomic nervous system generates three hierarchically organised states — ventral vagal (social engagement), sympathetic (mobilisation: fight/flight), and dorsal vagal (immobilisation: freeze) — each with characteristic phenomenological and behavioural signatures.

What these frameworks share is a conviction that emotional states are not located in the brain alone, nor in the body alone, but in a coupled system that is best understood as a single functional unit. The term *soma* — from the Greek for body — is used here to denote this unified body-mind system, following the tradition of somatic psychotherapy.

The Felt Sense and Sub-Perceptual Emotion

Gendlin’s concept of the *felt sense* (1978) is of particular relevance. He described it as “a special kind of internal bodily awareness... a body sense of meaning.” It is not an emotion in the ordinary sense — not a named feeling — but something more diffuse: a pre-articulate sense that *something is there*, present in the body, before it has been identified or named. Focussing, the therapeutic method Gendlin developed, works precisely by attending to this pre-threshold signal and allowing it to surface into conscious articulation.

The Soma-Field Model provides a formal account of what the felt sense is: it is the activity of the emotional field below the perceptual threshold.

It is real, causal, and continuously present. It shapes cognition and behaviour even when it does not surface as a named feeling.

Quantum Field Theory: Structure, Not Metaphor

Quantum Field Theory (QFT) is the framework of modern particle physics. Its central departure from classical physics is the priority of the *field* over the *particle*. In QFT, what we call particles — electrons, photons — are not fundamental objects. They are *excitations* of an underlying field: local, stable configurations of energy that arise when the field receives a sufficient perturbation.

The quantum vacuum — the ground state of the field — is not empty. It is a seething background of virtual fluctuations: momentary excitations that do not have enough energy to persist as observable particles. The vacuum is active, but sub-threshold.

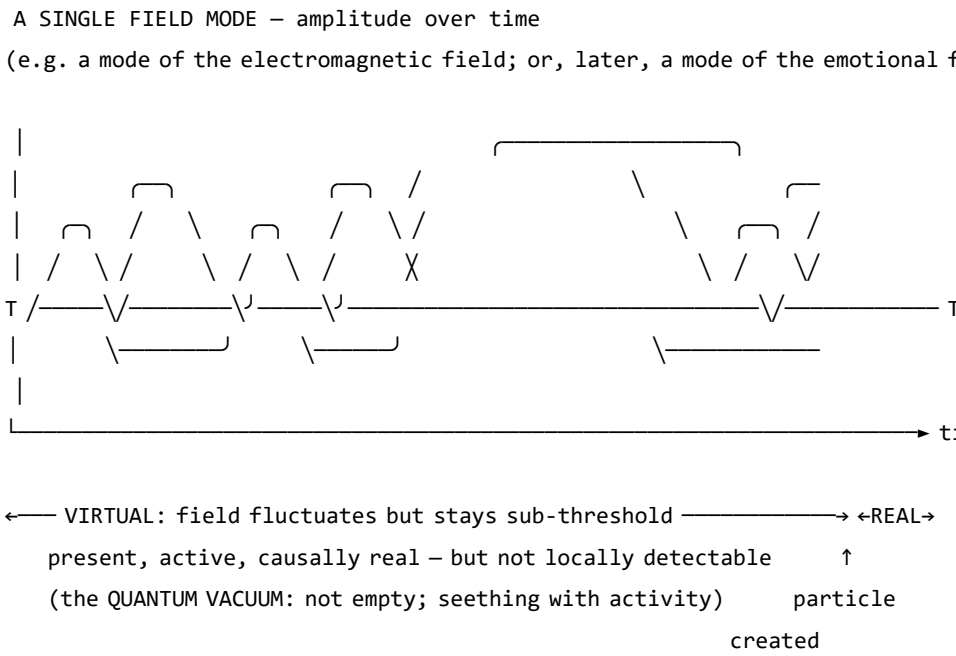


Figure 0. A single field mode in quantum field theory. The field oscillates continuously. Below the detection threshold T , excitations are

sub-threshold — real and causally active, but not detectable as particles. The quantum vacuum is not empty; it is a field in constant motion that never quite crosses the threshold. When the amplitude does cross T , a particle exists: a locally observable excitation. The same structure — field always present, consciousness only when threshold crossed — is the core of the Soma-Field Model.

This paper does not claim that emotions are quantum phenomena in any literal sense: the soma-field is a classical field, not a quantised one. The claim is stronger and more specific than analogy: the mathematical object being constructed — the Green’s function of a coupled field manifold — is formally of the same *type* as the objects that arise in QFT, differing only in the dimensionality of the manifold and the nature of the probe. What was previously described as a structural analogy is here identified as a formal correspondence: a particle is a pole in the propagator of its field; a conscious emotional percept is a pole in the propagator of the soma-field. Different physics. Same mathematics.

That correspondence gives the model precise vocabulary for the following set of ideas, which are central to the clinical observation of emotion:

- A quantity that exists everywhere, continuously, even when unobserved
- A background of sub-threshold activity that is real and causally effective
- The emergence of observable phenomena (conscious feelings) through threshold-crossing excitation of that background
- The possibility of multiple simultaneous excitations that interact with one another

Note (May 2026): A subsequent experiment (QUANT-EXP-1) demonstrates that the quantum extension of the Hopfield landscape used in this model — replacing the classical Langevin process with a transverse-field quantum annealer — produces a measurable *topological reachability advantage*: quantum annealing reaches attractor basins that cold classical dynamics cannot reach at any finite noise level. This upgrades the

formal correspondence from a structural claim to a testable empirical prediction. See the companion paper *Quantum Soma and the Penrose Gap* (doi:10.5281/zenodo.20351230) for the full results and theoretical implications.

One further consequence follows. The clinical phenomena of alexithymia — difficulty identifying and naming feelings — and its apparent opposite, emotional flooding or hypervigilance, have always been treated as separate conditions requiring separate explanations. In the Green’s function framing, they are the same structure at two extremes of the same parameter: the perception threshold T_i is too high (the bulk dynamics cannot cross into observable experience) or too low (bulk fluctuations flood the boundary without filtering). This is structurally identical to one of the deepest open problems in particle physics — the **hierarchy problem** — which asks why gravity is so much weaker than the other forces. The standard answer is that gravity propagates in the full higher-dimensional bulk while other forces are confined to a lower-dimensional brane; the coupling across the brane boundary determines the apparent weakness. The soma-field correspondence is exact: the threshold T_i is the brane. Perception is confined to the one-dimensional boundary of an eleven-dimensional dynamics. The hierarchy of emotional experience — why conscious feeling is so much weaker and more transient than the underlying field activity — has the same formal structure as the hierarchy of forces.

Neural Network Energy Functions and Hopfield Networks

In 1982, John Hopfield (awarded the Nobel Prize in Physics in 2024) proposed a model of associative memory based on a network of interconnected neurons (Hopfield, 1982). The critical insight was borrowed directly from statistical physics: the network could be assigned an **energy function** — a scalar quantity that decreases with each state update — such that the network would always evolve toward a local energy minimum. These minima are the stable states of the network: its memories,

or more precisely, its *attractors*.

Hopfield observed that his neural network’s dynamics were mathematically identical to those of an Ising spin-glass model from condensed matter physics — a system of interacting magnetic spins that minimises its total energy by aligning or anti-aligning with neighbours. The energy function he used is:

$$H(\mathbf{s}) = -\frac{1}{2} \sum_{i,j} W_{ij} s_i s_j - \sum_i \theta_i s_i$$

where $\mathbf{s}$ is the state of the network, W_{ij} is the coupling strength between units i and j , and θ_i is the activation threshold of unit i . The network always moves in the direction of decreasing H .

The Soma-Field Model applies this energy function directly to emotional dynamics. The *emotional coupling matrix* W encodes the relationships between emotional modes — which emotions amplify one another, which suppress one another — and the energy function determines the direction in which the emotional field naturally evolves.

Hopfield’s network is a formal model of the *neocortex*: a system for storing cognitive patterns and retrieving them from partial cues by minimising an energy function. Every artificial neural network constructed since McCulloch and Pitts (1943) (McCulloch & Pitts, 1943) — from perceptrons to backpropagation networks to transformers — sits in this neocortical lineage. These systems recognise patterns, predict sequences, and minimise prediction error with increasing sophistication. None of them possess a limbic system. They have no internal valuation, no arousal modulation, no threat-detection architecture, no attachment structure, no interoception. They have very effective cortex.

The Soma-Field Model does not add to the neocortical lineage. It proposes the architectural layer that has never been formally built: *an artificial limbic system*.

Hopfield memory is associative and pattern-completing; somatic memory

is state-reinstating. The field does not merely remember what happened. It re-lives it. *A body with a past.*

Hopfield’s later-reported wish to have incorporated something analogous to ‘maternal instincts’ into the energy function was, in this reading, not a desire for a better cortex. It was an intuition pointing directly at the absent system — the layer beneath the cortex that assigns value, registers threat, and holds the body in a particular way of being long after the event that caused it.

This positions the Soma-Field Model not as a supplement to the neocortical lineage but as its completion. Artificial neural networks have, for eighty years, been increasingly sophisticated formal models of the neocortex: pattern recognition, sequence prediction, error minimisation. The cortex has been mapped in extraordinary detail. The limbic system — which assigns value, detects threat, modulates arousal, maintains attachment, and reinstates whole somatic states in response to partial cues — has had no comparable formal treatment. The architectural description of the vertebrate brain was, until this paper, half-built.

Four kinds of formal intelligence. This architectural gap can be situated within a wider taxonomy. Four quotients have been proposed to describe the landscape of biological intelligence across popular and scientific usage. They map onto the formal components of this model with an exactness that is not coincidental:

Quotient	What it measures	Biological substrate	Soma-Field status
IQ — cognitive	Pattern recognition, reasoning, prediction	Neocortex	Built (1943–): McCulloch & Pitts → Hopfield → transformers
EQ — emotional	Valuation, arousal, affect regulation	Limbic system	Built here: $W, K(\tau), H(\mathbf{e}), C_{\text{HRV}}, \dot{H}$

Quotient	What it measures	Biological substrate	Soma-Field status
AQ — adversity	Structural resilience under threat	PFC–limbic axis	Built here: $S_{\text{inst}}, \partial\ W\ /\partial t,$ $C_{\text{HRV}}^{\text{recovery}}$
SQ — social	Attunement, theory of mind, relational navigation	Mirror system, TPJ	<i>Next paper:</i> κ_r , multi-field coupling

Table 3. Four dimensions of biological intelligence mapped onto the Soma-Field Model. The neocortical lineage (IQ) has been formally modelled for eighty years. Emotional intelligence (EQ) and adversity resilience (AQ) are formalised here for the first time. Social intelligence (SQ) is defined as the next extension of the framework.

AQ — adversity quotient — is formally the capacity to update W after adversity without the adversity permanently becoming W . Its mathematical definition appears in Section 3.4; its pathological lower bound is C-PTSD, in which all three components of AQ are simultaneously compromised (Appendix B.2).

The AI alignment implication follows directly. Current artificial systems have high IQ by construction and zero EQ, AQ, or SQ. The absence of internal valuation means that valuation must be injected externally — through reinforcement learning from human feedback (RLHF) and related techniques — which is structurally brittle for the same reason that a field with no limbic layer is brittle: the system has no internal stake in what it does. The Soma-Field formalisation specifies what that internal stake would look like, were it ever built.

A further lineage note is worth recording. Ramsauer et al. (2020) demonstrated that continuous-state modern Hopfield networks are mathematically equivalent to the self-attention mechanism in transformer language models. The softmax attention operation that drives contemporary large

language models is a Hopfield retrieval step. The Soma-Field Model sits in this same energy-based lineage: the equations underlying associative memory, language understanding, and somatic trauma response are, at the appropriate level of abstraction, the same equations.

A historical irony completes the picture. String theory was not discovered as a theory of strings. In 1968, Gabriele Veneziano wrote down a scattering amplitude — a response function encoding how particles scatter — and only later did Nambu, Nielsen, and Susskind identify the string as whatever object produces that amplitude (Veneziano, 1968). The response function came before the thing. The Soma-Field Model recapitulates this historical order deliberately: the primary object is the eleven-dimensional coupling manifold; the string — the one-dimensional conscious percept — is what the manifold produces when probed. We retain Veneziano’s discovery and decline to reify the string.

The Formal Correspondences: Where the Link Was Seen

The structural analogy between QFT and the Soma-Field Model is not merely conceptual. There are three places where equations from different disciplines become, after substituting the relevant quantities, literally the same functional form. The following sets them side by side. The point is not to impress with notation but to show exactly where the recognition happened — the moment when the same Greek letters appeared in the same positions in two fields that had no prior reason to be connected.

The same Hamiltonian: Ising spin model (condensed matter physics, 1920s) — Hopfield neural network (computational neuroscience, 1982) — Soma-Field Model:

$$H_{\text{Ising}}(\sigma) = -\frac{1}{2} \sum_{i,j} J_{ij} \sigma_i \sigma_j - \sum_i h_i \sigma_i$$

$$H_{\text{soma}}(\mathbf{e}) = -\frac{1}{2} \sum_{i,j} W_{ij} e_i e_j - \sum_i \theta_i e_i$$

Replace $J_{ij} \rightarrow W_{ij}$, $\sigma_i \rightarrow e_i$, $h_i \rightarrow \theta_i$: identical. The physicist, the neural network theorist, and the somatic clinician are computing the same energy function on different state spaces. The Hopfield 2024 Nobel Prize was awarded for discovering this identity between spin physics and neural computation; the Soma-Field Model extends the same identity one step further to emotional dynamics.

The Wick rotation — why the same exponential appears in QM and in memory:

In quantum mechanics, the time evolution operator is a complex phase:

$$U(t) = e^{-i\hat{H}t/\hbar}$$

Substitute $t \rightarrow -i\tau$ (the *Wick rotation* — replacing real time with imaginary time):

$$e^{-i\hat{H}(-i\tau)/\hbar} = e^{-\hat{H}\tau/\hbar}$$

The oscillating complex exponential becomes a real decaying exponential. This is the Boltzmann weight $e^{-\beta\hat{H}}$ at $\beta = \tau/\hbar$. The Langevin equation $\dot{\mathbf{e}} = -\nabla H + \eta$ is the classical limit of this Wick-rotated dynamics. Every simulation of the soma-field running this equation is, formally, a path integral in imaginary time.

The same propagator: Euclidean QFT (imaginary-time two-point correlator for a massive scalar field) — C-PTSD trauma memory kernel:

$$G_E(\tau) = \langle \phi(0) \phi(\tau) \rangle_{\text{QFT}} = \frac{1}{2m} e^{-m|\tau|}$$

$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-|\tau|/\tau_k}$$

Same form. The QFT field mass m corresponds to $1/\tau_k$ — the reciprocal of the trauma trace decay time. A heavier particle has a shorter-range propagator; a shorter-lived trauma trace decays faster. Therapeutic processing (reducing A_k , increasing τ_k) is, in the QFT language, changing the mass and amplitude of the propagator until the correlation function vanishes.

The specific visual moment: the quantum phase factor is $e^{-i\omega t}$. Remove the i (Wick rotation) and it becomes $e^{-\omega\tau}$. The memory kernel is $e^{-\tau/\tau_k}$. These are the same exponential. The i is the only difference between a quantum field that oscillates and a trauma trace that decays.

QFT quantity	Symbol	Soma-Field	
		analogue	Symbol
Field mode	ϕ_k	Emotional mode	e_i
Coupling constant	J_{ij}	Coupling matrix entry	W_{ij}
Field mass	m	Inverse decay time	$1/\tau_k$
Propagator amplitude	$1/2m$	Trauma trace amplitude	A_k
Euclidean propagator	$G_E(\tau) \propto e^{-m\tau}$	Memory kernel	$K(\tau) \propto e^{-\tau/\tau_k}$
Vacuum energy	$\langle H \rangle_0$	Resting field energy	$H(\mathbf{e}_{\text{calm}})$
Thermal fluctuation	$k_B T$	Noise amplitude	σ_0
Wick rotation	$t \rightarrow -i\tau$	Real-time Langevin	$\dot{\mathbf{e}} = -\nabla H + \eta$

Table 2. Formal correspondence between QFT quantities and Soma-Field analogues. Each row is a single mathematical entity in two notations. These correspondences were not constructed after the fact; they are the reason the QFT framework was recognised as relevant.

The central identification — particle and percept as poles in their respective propagators. All four correspondences above follow from one structural fact. In QFT, a particle is not a separate object from the field. It is a *pole* in the field’s propagator — the Green’s function evaluated in momentum space:

$$\tilde{G}_{\text{QFT}}(k^\mu) = \frac{i}{k^2 - m^2 + i\varepsilon}$$

The particle exists precisely when the four-momentum satisfies $k^2 = m^2$ — the *on-shell condition*. The particle is the singularity in the field’s response to a point source: the field’s Green’s function, evaluated at its own resonance.

Diagonalise W with eigenvalues λ_i (the natural resonance frequencies of the emotional modes). The soma-field propagator — the two-point correlator $\langle e_i(t) e_i(t') \rangle$ in the frequency domain — is:

$$\tilde{G}_{ii}(\omega) = \frac{\sigma_{\text{eff}}^2}{\omega^2 + \lambda_i^2}$$

A conscious emotional percept in mode i exists precisely when the excitation frequency ω approaches $i\lambda_i$ — the mode’s natural resonance. The percept is the singularity in the soma-field’s response to a somatic probe.

Setting the two propagators side by side:

$$\underbrace{\frac{i}{k^2 - m^2 + i\varepsilon}}_{\text{QFT: particle at mass-shell } k^2=m^2} \quad \longleftrightarrow \quad \underbrace{\frac{\sigma_{\text{eff}}^2}{\omega^2 + \lambda_i^2}}_{\text{Soma-Field: percept at resonance } \omega=i\lambda_i}$$

Both are poles in the propagator of their respective field manifold. A photon is not the electromagnetic field; it is the field’s Green’s function evaluated at a resonance. A flash of conscious emotion is not the soma-field; it is the field’s Green’s function evaluated at a threshold-crossing resonance. The manifolds differ — one is the four-dimensional spacetime

vacuum, the other is the eleven-dimensional emotional coupling geometry. The mathematical type is the same. This is not analogy.

The Body Schema, Interoception, and Pain

A complete model of the emotional field must address a phenomenon that standard psychological accounts of emotion consistently underspecify: the field is not a model of the physical body. It is the nervous system's *predictive model* of the body — a continuously updated internal representation of what the soma should be experiencing, revised by incoming interoceptive signals.

The clinical proof of this distinction is phantom limb pain (Ramachandran & Hirstein, 1998). Patients who have undergone amputation routinely experience pain in the absent limb. The pain is real: it activates the same neural circuits, produces the same suffering, and responds to the same analgesics as pain from an intact limb. The limb is gone. The neural model of the limb persists. What hurts is the *brain's representation* of the foot, not the foot.

This is not an anomaly. It is the normal condition of all somatic experience. The brain does not receive raw signals from the body — it maintains a continuous predictive model of the body (the *body schema*) and generates somatic experience from that model. Interoception — the sense of the internal body state — is a prediction, not a direct readout (Seth, 2021). The brain predicts what the heart should be doing, what the gut should feel like, where tension should be. The felt body is the predicted body.

The formal consequence is direct: the soma-field's state vector $\mathbf{e}(t)$ must include **somatic modes** — pain states, regional tension, visceral sensation, proprioceptive activation — alongside emotional modes. These are modes of the same field, governed by the same coupling matrix W . The W_{ij} between fear modes and somatic pain modes is the formal account of why fear amplifies pain, why safety reduces it, and why chronic pain and

C-PTSD are highly comorbid. They are not separate conditions sharing a correlation. They are the same attractor architecture operating across emotional and somatic modes simultaneously.

Phantom limb as attractor persistence. An amputated limb's somatic modes do not disappear from W when the limb is removed. The neural model persists. When movement- intention modes are activated — attempting to move the absent foot — foot-sensation modes are co-activated via W . If co-activation exceeds threshold, it is experienced as pain. Ramachandran's mirror box provides visual input that disconfirms the prediction error: new sensory evidence that the limb is moving, reducing coupling-driven co-activation, and therefore reducing the pain. This is $W \rightarrow W'$: therapy as structural rewriting of the field.

The load-bearing hyphen. The term *emotional-somatic* in clinical literature is not a stylistic compound. The hyphen marks an ontological claim: emotional states and somatic states are not two separate things that correlate. They are two aspects of the same field. The coupling matrix W is precisely the hyphen, made formal.

Therapeutic implication. Somatic therapies — body scanning, sensorimotor work, EMDR's bilateral stimulation — work not on the physical body but on the brain's model of the body. They provide new interoceptive evidence that updates the prediction. They change W . Therapy does not fix the tissue. It updates the model.

Correspondence with Existing Emotion Representations

A reasonable objection to any new framework is: *there is already a great deal of structure out here*. This is true. The emotion research literature contains several well-developed representational systems, and the Soma-Field Model must be positioned relative to them. The short answer is that every existing representation is *descriptive*; the Soma-Field Model is *dynamical*. The longer answer follows.

Categorical taxonomies (Ekman 1972; Plutchik 1980; Parrot 2001) assign names and hierarchical membership to emotional states. They are ontologies in the formal sense: a T-Box of classes and subclass relations. Plutchik’s wheel additionally defines a *blend* operation — Love := Joy $\sqcap$ Trust, Awe := Fear $\sqcap$ Surprise — which is precisely the OWL2 *intersectionOf* construction. These systems tell you what to call a state. They do not tell you how a state evolves, or which attractor a system settles in when two mechanisms fire simultaneously.

Dimensional models (Russell 1980; Mehrabian and Russell 1974) embed emotions in a continuous space, canonically Valence $\times$ Arousal (the *circumplex*), sometimes extended to Pleasure $\times$ Arousal $\times$ Dominance. These models capture the *coordinates* of a state. The energy landscape of the Soma-Field Model — the function $H(\mathbf{e})$ over emotion-space — is the dynamical generalisation of the circumplex: the circumplex is a snapshot of positions; the energy landscape is the surface over which the field moves. The stable attractors of H are the emotion categories; their coordinates are the circumplex positions.

Process and appraisal models (Scherer 1999; Frijda 1986; the OCC model of Ortony, Clove and Collins 1988) describe the *sequence of evaluations* through which a stimulus becomes an emotion. They are closer to the Soma-Field dynamics — they include temporal stages — but they are deterministic and single-threaded: one appraisal chain, one output. The Soma-Field replaces this with a parallel field update: all modes evolve simultaneously, governed by the full W matrix.

Music-specific schemas (BRECHEMA, Juslin and Västfjäll 2008; Juslin *et al.* 2011; GEMS, Zentner *et al.* 2008) are the closest antecedents to the present model. The BRECHEMA framework identifies eight distinct psychological mechanisms through which music evokes emotion — Brain stem reflex, Rhythmic entrainment, Evaluative conditioning, Contagion, Visual imagery, Episodic memory, Musical expectancy, Aesthetic judgement — each with distinct evolutionary origins, processing speeds, and neural substrates. These mechanisms are the *object properties* of the emotion-induction ontology: they

specify which musical features activate which emotional outputs. Juslin explicitly identifies the open problem: “*Exploring how various musical emotions come about through the interaction of multiple psychological mechanisms is an exciting endeavour that has just begun*” (Juslin & Sloboda, 2011, p. 638). The W coupling matrix is the formal answer to that open problem. Where BRECVEMA gives a list of mechanisms with characteristic outputs, the Soma-Field gives the interaction tensor W_{ij} that specifies, with numerical precision, what happens when mechanisms i and j fire concurrently.

The deeper connection is spectral. The *eigenmodes* of W — the directions in emotion-space that evolve independently — are the natural resonances of the soma-field: the patterns the field rings with when struck. BRECVEMA mechanisms are inputs: they excite specific rows of W . The eigenspectrum of W is the response: the set of frequencies the manifold can sustain. Where BRECVEMA is a taxonomy of *stimuli*, the eigenspectrum of W is a taxonomy of *responses*. Juslin’s open problem — how mechanisms interact — is the question of how stimulus-space maps onto eigenmode-space through W . Section 3.3 develops this.

Body maps (Nummenmaa *et al.* 2014) map emotions to their somatic distribution — where in the body each emotion is felt. These are precisely the spatial support of the soma-field modes: the field configuration corresponding to an attractor state is the body map of that emotion. Body maps are measurements of the attractors; the Soma-Field is the dynamical system that generates them.

The formal correspondence table extends Table 2 to include these systems:

Existing representation	What it captures	Soma-Field equivalent
Ekman categories	Attractor labels (names)	Values of $\mathbf{e}$ at energy minima

Existing representation	What it captures	Soma-Field equivalent
Plutchik dyads ($A \sqcap B$)	Blend attractors	Metastable states between two energy minima
Russell circumplex	Coordinates (valence, arousal)	Projection of $H(\mathbf{e})$ onto two axes
OCC appraisal tree	Single-path sequential process	Single trajectory in the full field
BRECVEMA mechanisms	Object properties: stimulus $\rightarrow$ emotion	Rows of W : mechanism i activates mode j
Body maps (Nummenmaa)	Spatial support of each attractor	Modal structure of $\mathbf{e}$ at each minimum

None of these correspondences require modifying either the existing representations or the Soma-Field Model. They are consequences of the model’s structure. The formal machinery for exploring these correspondences — typing BRECVEMA mechanisms as Lean inductive constructors, Plutchik blends as type intersections, mechanism profiles as decidable propositions — is developed in the companion file `src/EmotionOntology.lean`.

The Soma-Field Model

The field is primary. The felt emotion is secondary — it is what registers when the field is probed. This is the same ontological relationship as between a quantum field and a particle: the field exists continuously and everywhere; the particle is what you observe at the moment of measurement. The Soma-Field Model does not describe what emotions are *made of*. It describes the manifold whose impulse response *is* conscious emotional experience.

Emotions as a Persistent Wave Field

The foundational claim of the Soma-Field Model is simple: emotions are not events. They are a *field* — a distributed, continuous quantity defined over the entire soma (body-mind system) at all times.

This field has two coupled components:

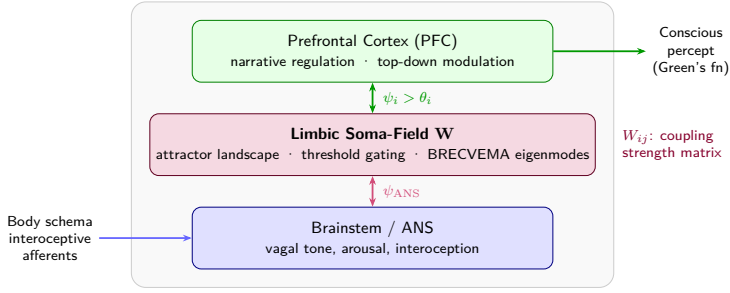
1. **The somatic wave $\mathbf{E}_{\text{body}}(x, t)$:** distributed across the body as patterns of visceral sensation, muscle tone, proprioception, interoception, and autonomic state.
2. **The neural wave $\mathbf{E}_{\text{neural}}(x, t)$:** distributed across the nervous system as patterns of activation in cortical, subcortical, and peripheral neural circuits.

These two components are not separate systems. They are coupled — each continuously influencing the other. The total emotional field is their combined state:

$$\mathbf{E}(x, t) = \mathbf{E}_{\text{body}}(x, t) \otimes \mathbf{E}_{\text{neural}}(x, t)$$

The field is characterised by:

- **Multiplicity:** multiple emotional modes can be simultaneously active and interfering
- **Continuity:** it exists at all times, not only during episodes of conscious feeling
- **Spatial distribution:** different aspects of the field are localised in different regions of the soma (the familiar clinical observation that grief is felt in the chest, fear in the gut, anger in the jaw and fists)
- **Temporal dynamics:** the field evolves continuously, driven by the energy function



Figure

1. *The Soma-Field. The body and brain are not separate containers of emotion but two coupled components of a single distributed wave field. Neither is primary; each continuously modifies the other. The symbols indicate that wave activity is always present in each region, not only during episodes of conscious feeling.*

The Perception Threshold

Not all activity in the emotional field is consciously perceived. The field has a **perception threshold** T_i for each emotional mode i . Below this threshold, the emotional mode is sub-perceptual: it exists, it influences behaviour and physiology, but it does not surface as a named conscious feeling.

$$\text{Emotion } i \text{ is consciously perceived} \iff |\mathbf{E}_i(t)| > T_i$$

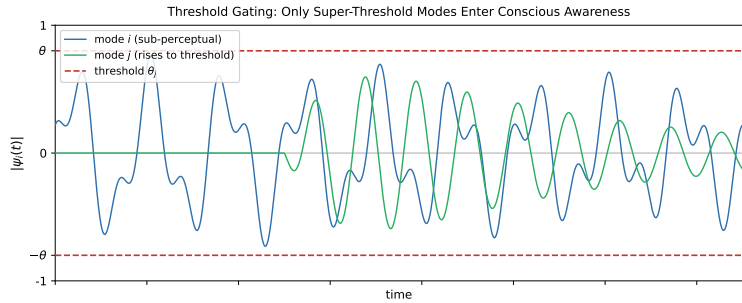
This threshold crossing corresponds precisely to the QFT excitation analogy: the emotional mode behaves like a virtual particle that has accumulated enough energy to become real — to emerge from the sub-threshold background and enter awareness.

This accounts for a range of clinically significant phenomena:

Clinical Observation	Soma-Field Account
Patient reports no feeling but shows physiological signs of distress	Sub-threshold field activity below T_i

Clinical Observation	Soma-Field Account
Sudden unexpected flood of emotion in session	Rapid threshold crossing after gradual accumulation
Emotion felt somatically but not named	Threshold crossed in $\mathbf{E}_{\text{body}}$, not yet in $\mathbf{E}_{\text{neural}}$
Alexithymia (difficulty identifying feelings)	Elevated T_i — high threshold requiring more energy to cross
Hypervigilance / emotional flooding	Lowered T_i — reduced threshold, field crosses to conscious easily

Table 1. Clinical observations mapped onto the perception threshold model.



Figure

2. The perception threshold T_i for a single emotional mode. The field is active continuously (lower trace). Conscious experience arises only when amplitude exceeds T_i (upper trace). Everything below the line is still there — shaping body and behaviour before it can be named.

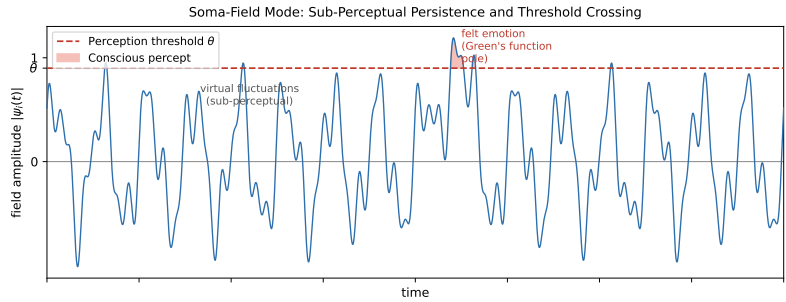


Figure 0. Continuous soma-field activity (blue) with a single threshold-

crossing event. The field is always active; conscious experience (shaded) arises only when amplitude exceeds the perception threshold (red dashed). Below the threshold: real, causally active, but not yet conscious.

The Interaction of Emotional Modes

Multiple emotional modes are simultaneously active in the field at all times. They do not simply co-exist: they interact. The nature of these interactions is encoded in the **emotional coupling matrix** W , where W_{ij} represents the influence of emotional mode j on emotional mode i .

- If $W_{ij} > 0$: emotion j amplifies emotion i (e.g., fear can amplify shame)
- If $W_{ij} < 0$: emotion j suppresses emotion i (e.g., calm suppresses anxiety)
- If $W_{ij} = 0$: emotions i and j are independent

The field evolves according to the energy gradient:

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \eta(t)$$

where $\eta(t)$ represents the continuous low-level fluctuations of the sub-perceptual field — the emotional equivalent of quantum vacuum noise. The field is always moving, always seeking lower energy, never at absolute rest.

The Three-Layer Architecture

The nervous system that implements the soma-field is not architecturally flat. Three hierarchically organised layers contribute to field dynamics, each corresponding to a distinct evolutionary substrate and a distinct role in the model. The clinical literature (Porges, 2011; van der Kolk, 2014; Ogden et al., 2006) converges on this stratification; what follows is its formal expression.

Layer 1 — Brainstem / autonomic baseline. The oldest structures: vagal nuclei, arousal systems, interoceptive machinery. In the model, this layer is represented by the noise term and, specifically, by the heart rate variability coherence C_{HRV} , which modulates effective noise amplitude across the whole field:

$$\sigma_{\text{eff}} = \frac{\sigma_0}{C_{\text{HRV}}}$$

High HRV coherence narrows effective noise, stabilising the field in its current attractor. This is the mechanism of HRV biofeedback as a regulatory intervention: it does not target any specific emotional mode but lowers the fluctuation floor of the entire field.

Layer 1 extension: cardiac acceleration and landscape tilt. The term C_{HRV} measures the *current state* of cardiac regularity — where the heart is. A complementary quantity is $\dot{H}(t)$, the first time-derivative of heart rate, in units of beats/s². This is the **cardiac acceleration**: not what the heart rate is, but where it is going.

The dimensional parallel with gravity is exact: gravitational acceleration g carries units m/s²; cardiac acceleration $\dot{H}$ carries units beats/s². Both are accelerations; both describe a force field rather than a position. Gravity does not tell you where a test mass is — it tells you how it will move next. Cardiac acceleration tells you not the current BPM but the direction of the next one: the N+1 state.

In the soma-field, $\dot{H}(t)$ enters the dynamics not as noise modulation but as a **landscape tilt** — a time-varying bias added to the Hamiltonian that tips the energy function toward activation or rest attractors:

$$H(\mathbf{e}, t) = H_0(\mathbf{e}) - \alpha \dot{H}(t) \beta \cdot \mathbf{e}$$

where $\alpha > 0$ is the cardiac-somatic coupling constant and β is a mode-coupling vector (at leading order, $\beta = \mathbf{1}$: the tilt acts uniformly across all modes). When $\dot{H}(t) > 0$ (heart accelerating), the landscape tilts toward higher activation states before any cognitive or affective threshold is crossed. When $\dot{H}(t) < 0$ (heart decelerating), it tilts toward rest. The

full three-layer equation including the cardiac acceleration term is:

$$\dot{\mathbf{e}}(t) = -\nabla H_0(\mathbf{e}) + \alpha \dot{H}(t) \beta + R_{\text{PFC}}(\mathbf{e}, t) + \frac{\sigma_0}{C_{\text{HRV}}} \xi(t)$$

The two cardiac terms serve distinct functions: C_{HRV} (state) modulates the noise floor; $\dot{H}$ (acceleration) tilts the deterministic landscape. Both are needed for a complete account of cardiac influence on the field.

Predictive clinical value. A patient with BPM = 90 and $\dot{H} = +4$ beats/s² is approaching threshold; one with BPM = 90 and $\dot{H} = -4$ beats/s² is retreating from it. The snapshot is identical; the trajectories are opposite. Cardiac acceleration is therefore an early-warning signal for threshold crossings — detectable at Layer 1 before the emotional field at Layer 2 has crossed its threshold. This has independent support in cardiology: Bauer et al. (2006) demonstrated that *acceleration capacity* and *deceleration capacity* of heart rate — estimates of $\dot{H}$ over a cardiac window — carry prognostic information independent of conventional HRV measures.

The somatic equivalence principle. The cardiac acceleration term $\alpha \dot{H} \beta$ is structurally identical in the equation to any other forcing term. From the perspective of the field itself — from conscious experience — cardiac-driven activation is indistinguishable from event-driven activation. A sudden heart rate acceleration tilts the landscape by exactly the same mechanism as an external threat or an intrusive memory. The field has no access to the origin of the tilt. This is the formal account of a clinically well-documented phenomenon: anxiety initiated by cardiac irregularity (arrhythmia, postural hypotension, caffeine, exertion) is experienced as emotionally caused, because the somatic signal is identical. Disambiguation requires either external measurement or deliberate interoceptive inquiry that can distinguish the two sources.

Layer 2 — Limbic system / emotional memory. The primary substrate of the Soma-Field Model. The coupling matrix W , memory kernel $K(\tau)$, Hamiltonian $H(\mathbf{e})$, and threshold T all belong here. The limbic

layer stores emotional-somatic states and reinstates them in response to partial body cues: a continuous, asymmetric, temporally extended Hopfield network operating on somatic states rather than cognitive patterns. This is the architectural layer that has been absent from every artificial neural network since McCulloch and Pitts (1943) (McCulloch & Pitts, 1943). The cortex has been modelled many times; the limbic system has not.

Structural plasticity under adversity. The Soma-Field framework permits a formal characterisation of the field’s resilience under adverse conditions. Define the *plasticity index* Π as a composite of three measurable field properties:

$$\Pi = \frac{1}{S_{\text{inst}}} + \left. \frac{\partial \|W\|}{\partial t} \right|_{\text{adversity}} + C_{\text{HRV}}^{\text{recovery}}$$

The three terms correspond to: (i) how accessible regulated-state attractors remain under adversity ($1/S_{\text{inst}}$, instanton accessibility — Section 4.4); (ii) how much the coupling matrix can structurally adapt following a threshold crossing ($\partial \|W\|/\partial t$, the plasticity component); and (iii) how quickly the HRV floor recovers after activation ($C_{\text{HRV}}^{\text{recovery}}$, the regulatory resilience component). Complex PTSD is the clinical presentation of chronically low Π across all three terms simultaneously: high barriers to regulated attractors, a rigid W dominated by threat configurations, and impaired C_{HRV} recovery. Structural plasticity is the capacity of the field to update W in the aftermath of adversity without the adversity permanently *becoming* W .

Layer 3 — Neocortex / prefrontal regulatory layer. Top-down modulation of Layer 2, represented as a regulatory term $R_{\text{PFC}}(\mathbf{e}, t)$. The full field dynamics becomes:

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}(t)) + R_{\text{PFC}}(\mathbf{e}, t) + \frac{\sigma_0}{C_{\text{HRV}}} \xi(t)$$

R_{PFC} represents voluntary attention, therapeutic technique, and con-

scious reappraisal acting on the field. It is not a correction of Layer 2 but a modulation of it. Under sustained therapeutic engagement, R_{PFC} participates in the structural modification $W \rightarrow W'$ constituting the forward transformation (Section 7).

The **threshold T is the Layer 2 / Layer 3 boundary**: sub-threshold dynamics are processed limbically and remain below conscious awareness; threshold-crossing events enter Layer 3 and become available for narrative, meaning-making, and voluntary response. This is the formal basis for the clinical observation that insight without somatic activation is limited, and somatic activation without Layer 3 engagement cannot produce structural change: the layers are coupled, not independent. R_{PFC} requires a threshold crossing in order to have something to work with.

The two-term Langevin equation introduced in Section 3.3 is the Layer 2 special case ($R_{\text{PFC}} = 0$, $C_{\text{HRV}} = 1$). All subsequent sections develop that special case. The full three-layer equation is the general form.

The Energy Landscape

The Structure of the Emotional Energy Function

The energy function $H(\mathbf{e})$ defines a landscape over the space of possible emotional states. Like a physical landscape of hills and valleys, this landscape has:

- **Valleys (local minima)**: stable emotional states the field naturally moves toward
- **Hills (local maxima)**: unstable configurations the field naturally moves away from
- **Saddle points**: transitional configurations with mixed stability

The key property of an energy function is directionality: the field always moves *downhill*. It always evolves toward lower energy. Therapeutic intervention, in this framework, can be understood as:

1. **Changing the landscape:** modifying W — the coupling matrix — through new relational experience, insight, or somatic work, so that the energy minima are in healthier locations
2. **Adding energy to escape a trap:** helping the field accumulate enough energy to escape a deep but unhealthy local minimum (e.g., the freeze state)
3. **Pointing toward the global minimum:** orienting the field toward regulated calm

Attractor States: Fight, Flight, Freeze, and Regulated Calm

The Soma-Field Model proposes that the major attractor basins of the emotional energy landscape correspond directly to the autonomic states described by Porges’ polyvagal theory.

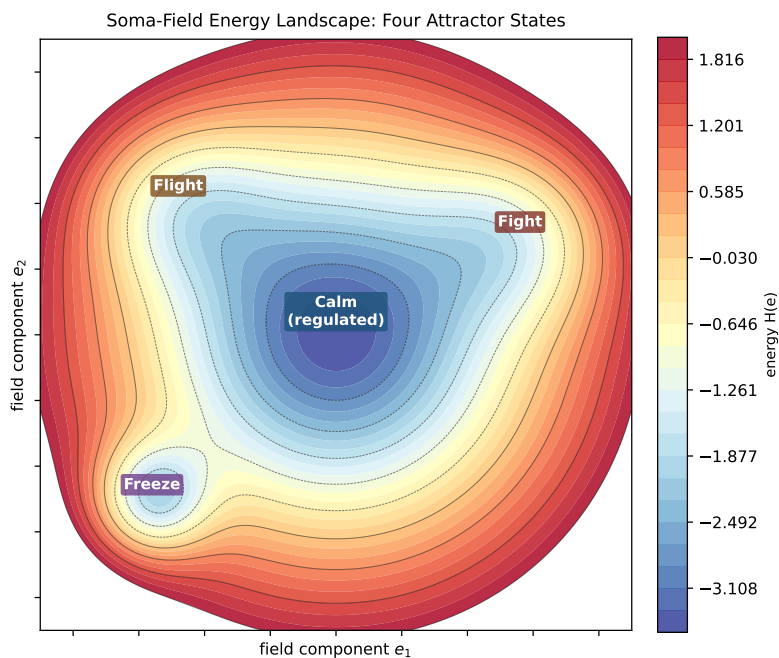


Figure 3a. Topographic (bird’s-eye) view of the energy landscape. The field always rolls downhill toward the nearest minimum. Freeze and calm are both low-energy — but freeze is surrounded by high walls. Escape

from freeze requires crossing those walls, which means first gaining energy before losing it again. This is the clinical challenge of working with dissociative states.

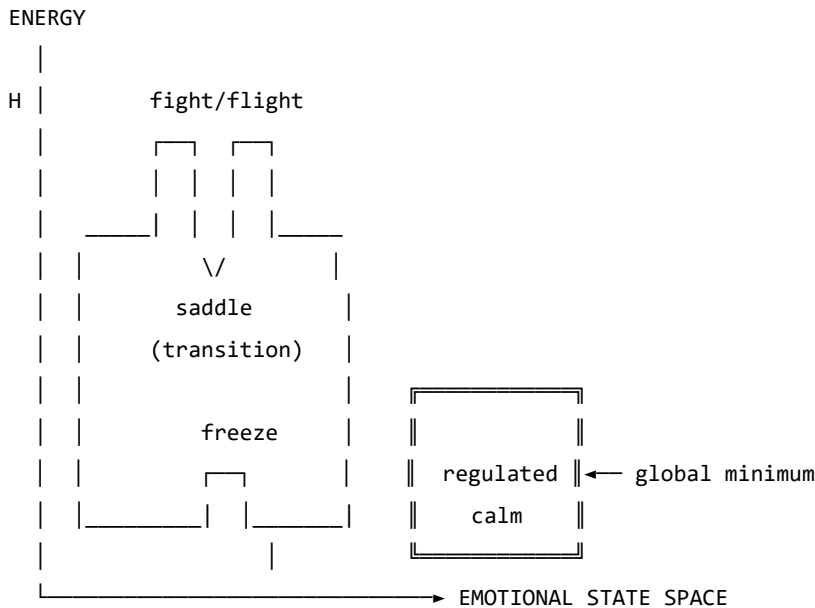


Figure 3b. Schematic energy landscape. Fight/flight are high-energy, unstable local minima. Freeze is a low-energy but isolated attractor — easy to enter, hard to escape. Regulated calm is the global energy minimum.

Attractor	Energy State	Polyvagal Correlate	Clinical Presentation
Regulated Calm	Global minimum	Ventral vagal (social engagement)	Present, flexible, connected
Fight	Shallow high-energy minimum	Sympathetic (mobilisation)	Agitation, anger, urgency
Flight	Saddle point / shallow minimum	Sympathetic (mobilisation)	Anxiety, avoidance, rumination

Attractor	Energy State	Polyvagal Correlate	Clinical Presentation
Freeze	Deep isolated minimum	Dorsal vagal (immobilisa- tion)	Dissociation, numbness, collapse

Table 2. Emotional attractors mapped onto Polyvagal states.

The therapeutic significance of this structure is considerable. The freeze state is dangerous not because it is high-energy — it is in fact very low energy — but because it is *isolated*: surrounded by energy barriers that make it difficult to exit. Escape from freeze requires first *increasing* the field’s energy (mobilising some arousal) before it can flow toward regulated calm. This corresponds well to the clinical observation that working with dissociated patients requires careful titration of arousal — not too much, not too little — before emotional processing is possible.

The Coupling Matrix as a Personal Signature

The coupling matrix W is not universal. Each person has a unique W , shaped by attachment history, trauma, cultural context, and temperament. A person with a history of developmental trauma may have a W in which anxiety and shame are strongly coupled ($W_{\text{shame, anxiety}} \gg 0$), creating a combined attractor that is particularly deep and sticky. A person with a secure attachment history may have a W in which positive emotions are broadly coupled to one another, creating a wide basin around regulated calm.

This implies that the energy landscape is a therapeutic object in its own right: understanding a patient’s W is understanding the structural dynamics of their emotional life.

In the M-theory compactification analogy developed in Appendix A, the coupling topology W corresponds to the shape of the compact G_2 manifold — the seven-dimensional geometry that determines which force-like couplings are allowed and with what strengths. That analogy is here

made precise: two people differ not merely in their emotional *parameter settings* but in their coupling *geometry*. Developmental trauma does not set a dial to the wrong value; it deforms the manifold. The therapeutic process of modifying W through relational experience, insight, or somatic work is, in this language, differential geometry: a continuous deformation of the G_2 manifold toward a configuration in which the regulated-calm attractor is globally accessible. The practitioner is, without having been told so, a geometer.

Dissonance and Resolution

The Acoustic Analogy

The Soma-Field Model draws a further structural analogy, this time with acoustics. When two sound waves interact, the quality of their interaction — consonance or dissonance — depends on the phase relationship between them. Consonant intervals (the octave, the fifth) have simple frequency ratios and produce stable, reinforcing interference patterns. Dissonant intervals (the tritone, the minor second) have complex ratios and produce beating, unstable, tension-generating patterns.

The model proposes that the same relationship holds between emotional modes. When two emotional modes are in a compatible relationship — when their interaction is consonant — the field is in a relatively low-energy configuration and moves naturally toward the energy minimum. When they are in an incompatible relationship — when their interaction is dissonant — the field is in a higher-energy configuration, generating a gradient that drives toward resolution.

Dissonance, in this framework, is felt as tension. It is not pathological; it is directional. Dissonance is the field's way of communicating that it is far from equilibrium and that resolution is available.

The Resolution Principle

In music, dissonance resolves to consonance. The tritone — the most dissonant interval in Western tonality — creates a powerful gravitational pull toward resolution. In counterpoint, the rules of voice leading describe the specific paths by which dissonance must resolve. These rules are not arbitrary conventions; they describe the geometry of the acoustic energy landscape.

The same principle applies to emotional dissonance. An unresolved emotional state — grief that has not been fully experienced, anger that has been suppressed, fear that has been dissociated — is a dissonance in the field. It generates a persistent tension gradient. The therapeutic process can be understood as guided voice leading: finding the specific path of resolution that transforms the dissonant configuration into a consonant one.

This provides a formal basis for a widely-held clinical intuition: that emotions need to be *felt through* rather than avoided. Avoidance keeps the field in a dissonant state. The energy minimum — regulated calm — lies on the other side of the dissonance, not around it.

The Soma-Field Instrument

Rationale

The Soma-Field Model is not only a theoretical framework. It motivates a practical therapeutic instrument: a means by which a person can *externalise* their emotional field — make it visible and audible — and interact with it in real time.

The core insight is that the emotional field is normally invisible to its host. It operates below the threshold of conscious awareness, shaping behaviour and physiology without being available for reflection. If its activity could be rendered as a signal — a sound, an image, a pattern —

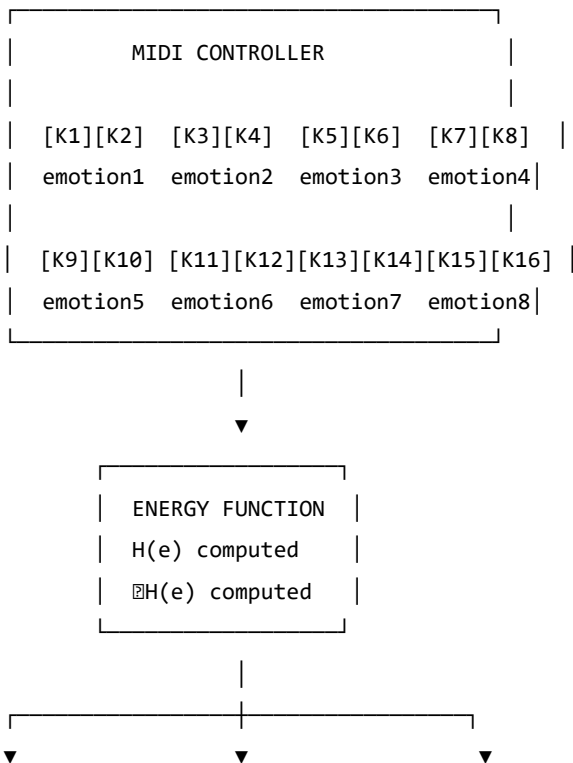
it could become an object of therapeutic attention.

Design

The instrument uses a MIDI controller with 16 rotary knobs as its input interface. Eight emotional dimensions are encoded, each represented by two knobs:

- **Knob 1** of each pair: the somatic (body-level) intensity of that emotional mode
- **Knob 2** of each pair: the cognitive/neural intensity of that emotional mode

This design reflects the two-component structure of the field: body and mind are encoded separately but coupled in the computation. Each knob has a continuous range, allowing fine expression of emotional intensity.



AUDIO OUTPUT	MIDI OUTPUT	VISUAL OUTPUT
(timbre reflects	(pitch/velocity	(field map:
dissonance)	reflects energy)	wave topology)

Figure 4. *The Soma-Field Instrument: input, computation, and multimodal output.*

The Feedback Loop

The instrument creates a **closed feedback loop** between the person and their emotional field:

1. The person expresses their current emotional state by adjusting the knobs
2. The system computes the energy function $H(\mathbf{e})$ and its gradient ∇H
3. The energy level, dissonance, and proximity to attractor states are rendered as:
 - **Sound:** harmonic content and timbre reflect the consonance or dissonance of the current state
 - **MIDI output:** pitch rises with tension, resolves as energy decreases
 - **Visual:** a real-time map of the emotional field, showing wave activity, threshold crossings, and the direction of the energy gradient
4. The person hears and sees their emotional field, and adjusts the knobs in response

This loop externalises the emotional field’s gradient — the direction in which it is *trying* to move — and makes it available as sensory information. The person becomes not only the source of the emotional signal but also its observer, creating the conditions for reflection and regulation that are at the heart of therapeutic work.

The Pluggable Emotion Model

No single model of the emotions is assumed. The coupling matrix W — the structure that determines how emotional modes interact — is loaded from an external configuration file. Standard models (Plutchik’s wheel of emotions, Ekman’s basic emotions, the valence-arousal-dominance dimensional model) are provided as defaults. The therapist or client can modify the coupling values to reflect their own understanding of their emotional patterns, or a new model can be substituted entirely. The computational engine is model-agnostic.

Clinical Implications

Assessment

The Soma-Field Model suggests a different orientation for emotional assessment. Rather than asking “What emotion do you feel?” — which presupposes threshold-level conscious awareness — it invites attention to the sub-perceptual field: “What is present in the body right now, even if you cannot name it?” This aligns with Focussing-oriented approaches and with sensorimotor methods that prioritise somatic signal over narrative content.

The energy landscape provides a clinical map. A person chronically in a fight or flight attractor shows a different energy signature from a person in a freeze attractor, even if their presenting narratives are superficially similar. The model suggests that these are structurally different therapeutic challenges: fight/flight require down-regulation, while freeze may first require careful up-regulation before down-regulation becomes possible.

Intervention

The energy function provides a formal basis for several existing clinical interventions:

- **Grounding and titration** (Levine, 2010): adding small, controlled amounts of energy to the field to approach — without flooding — a previously frozen or avoided emotional state
- **Pendulation** (Levine, 2010): oscillating between a dissonant state and a resource state, progressively widening the tolerance window — equivalent to approaching the energy minimum via a series of small excursions
- **Somatic resourcing** (Ogden et al., 2006): establishing a stable low-energy region in the landscape that the field can return to after excursions into high-energy territory
- **Working with the felt sense** (Gendlin, 1978): attending to sub-threshold field activity and allowing it to cross the perception threshold in a supported context

Psychoeducation

The wave model is immediately accessible to clients who have struggled to understand their emotional experience. The statement: “*Your emotions are like waves — they are always there, even when you can’t feel them, and they are always moving*” is both technically accurate within the Soma-Field framework and clinically useful as a normalising frame for sub-threshold emotional activity, for the apparently sudden onset of strong feelings, and for the experience of feeling multiple conflicting emotions simultaneously.

The energy landscape metaphor — “*right now the field is in a valley that is hard to leave, but it is not the lowest valley available to you*” — offers a way to discuss the freeze state, dissociation, and emotional stuckness without pathologising, while still acknowledging the structural difficulty of these states and the work required to shift them.

Neurodivergent Conditions as Operator Modifications

A clinically significant extension of the Soma-Field Model concerns neurodivergent conditions — specifically Autism Spectrum Condition (ASC), Attention Deficit Hyperactivity Disorder (ADHD), and Complex

Post-Traumatic Stress Disorder (C-PTSD), which frequently co-occur and each present distinct challenges for somatic emotional processing.

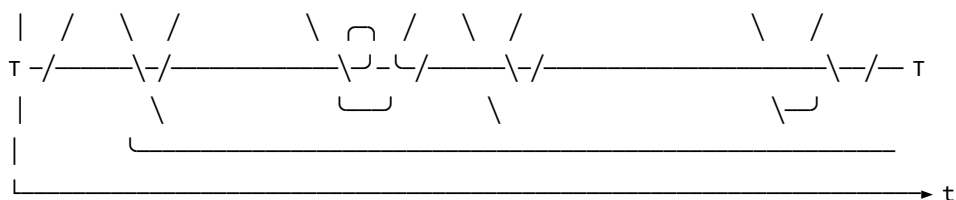
The key architectural principle is this: **these conditions are not parameter settings in the model. They are structural modifications to the operators themselves.** This distinction matters both mathematically and clinically. A parameter change (“set the fear threshold lower”) is a quantitative adjustment within the existing structure. An operator modification changes the *form* of the dynamics — it alters the governing equations, not merely their coefficients. Each condition wraps the standard pipeline in a different functional modifier, and — critically for the many individuals who carry all three — these modifiers *compose*. The combined condition is not three separate problems; it is the composition of three operators acting on the same underlying field.

The mathematical details of each modifier are given in Appendix B. Clinically, the consequences are as follows.

Complex PTSD introduces a *memory kernel* into the field dynamics: past high-energy states leave decaying echoes that continue to excite the field without new external stimulus. This is why traumatic activation can appear without identifiable trigger — the field is responding to its own history, not its current environment. The standard Hopfield attractor topology is also disrupted: C-PTSD renders the freeze attractor pathologically deep and wide, the window of tolerance (the basin around regulated calm) pathologically narrow, and the coupling matrix asymmetric — a condition under which the field can enter persistent *limit cycles* rather than settling to a stable minimum. Re-experiencing, flashbacks, and hypervigilance are, in this framework, limit-cycle oscillations in the traumatised field.

REGULATED FIELD (symmetric W, no memory kernel)



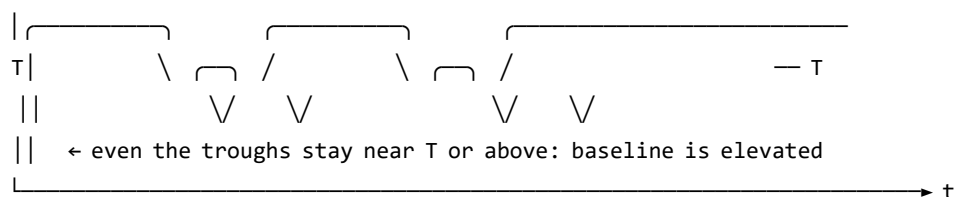


↑ baseline returns to near-zero between episodes

↑ each threshold crossing is a discrete, independent event

↑ 'regulated calm' is a genuine resting state – the global energy minimum

C-PTSD MODIFIED FIELD (asymmetric W , memory kernel $K(t-s)$ present)



↑ memory kernel: each activation feeds energy back into the next

↑ field rarely returns to true rest – past states re-enter present dynamics

↑ almost entirely above T : activation is the default, not the exception

↑ 'regulated calm' requires a non-perturbative transition (the instanton): small steps do not reach it; a qualitatively different move is needed

Figure 5. The same emotional field mode under two dynamic regimes. Top: regulated dynamics — the field oscillates and returns to a low baseline between episodes; conscious emotion (above T) is episodic and resolves. Bottom: C-PTSD-modified dynamics — the memory kernel elevates the baseline so that the field rarely returns to rest; episodes bleed into one another; the system cycles rather than settles. The mathematical basis for this comparison is given in Appendix B.2.

Developmental timing and what can be recovered. The character of the C-PTSD modification depends critically on *when* it occurred — the developmental age τ_d at which the primary traumatic modification took place.

For **late trauma** (τ_d large — adult or post-verbal): a coupling matrix W_0 formed before the event. The modification is additive: $W = W_0 + \delta W_{\text{trauma}}$. A counterfactual pre-trauma self exists, encoded in explicit narrative memory. Therapeutic processing can target δW specifically, and the goal of recovering proximity to W_0 is formally coherent.

For **early trauma** (τ_d small — pre-verbal, perinatal): the coupling matrix W was *formed under the modification*. There is no W_0 . The asymmetric coupling and the memory kernel coefficients are the baseline architecture, not additions to one. A counterfactual pre-trauma self was never encoded — it does not exist as a recoverable state.

This is a formal statement of a clinical fact that somatic therapists recognise but rarely have a mechanistic basis for: early trauma cannot be *processed away* in the sense of recovering a prior self, because no prior self was formed. The therapeutic goal is not subtraction ($W \rightarrow W_0$, which is undefined) but **forward transformation**: constructing a W' that supports a wider window of tolerance, different attractor topology, and lower memory-kernel amplitudes. This is a different mathematical operation — and requires a different therapeutic model.

The Soma-Field Instrument can reflect this distinction directly: a user whose primary modification is pre-verbal initialises with a *structural* coupling matrix (the modification *is* the baseline), not a neurotypical matrix with an added modifier. The formal basis for this parameterisation is given in Appendix B.2.1.

ADHD raises the effective *thermal noise* of the field — the amplitude of the sub-perceptual fluctuations — and simultaneously reduces the damping coefficient that slows the field’s response to the energy gradient. The result is a field that explores its energy landscape rapidly and unpredictably, is easily displaced from shallow attractor basins by small perturb-

ations (distractibility), but also achieves states of intense concentration (hyperfocus) when the coupling to a high-salience stimulus temporarily deepens a specific attractor basin far beyond its resting depth. ADHD is not a deficit of attention; it is a high-temperature, low-damping emotional field with a stimulus-dependent attractor structure.

Autism Spectrum Condition modifies the *projection kernels* — the functions that determine how the continuous somatic field is sampled to produce the discrete state vector — and the *sparsity* of the coupling matrix. Interoceptive research in autism (Garfinkel et al., 2016) documents significant differences in the processing of internal body signals; in model terms, certain somatic regions are over-represented (heightened sensory sensitivity) and others under-represented (reduced interoceptive clarity, contributing to alexithymia). The coupling matrix in ASC tends toward greater sparsity — fewer strong cross-modal emotional couplings — a pattern consistent with monotropism (Murray, 2018): the field settles deeply into individual attractors but transitions between them require proportionally more energy. Intense interests, emotional consistency within a context, and difficulty with unexpected transitions all follow from this attractor topology.

For the Soma-Field Instrument, the practical implication is significant. Rather than asking a neurodivergent user to configure their experience through knob adjustments, the system can instantiate the appropriate operator modifications as a named profile — “*load C-PTSD modifier*”, “*load ADHD modifier*” — each of which transforms the pipeline at the correct mathematical level. The user then interacts with a field that already reflects their structural reality, rather than one calibrated for a neurotypical baseline.

A further clinical implication deserves explicit statement. The Soma-Field Model locates interoceptive accuracy in the field itself: whether a somatic signal has exceeded its perceptual threshold T_i is a property of the field state, not a property of the clinician’s assessment of the patient’s credibility. A patient reporting an acute somatic state is reporting a threshold-crossing event. The model provides no mechanism by which

external disbelief suppresses that crossing. Modified projection operators — as occur in ASC — produce *different* somatic self-reports; the model gives no reason to assume they produce *less accurate* ones. The clinical literature documents a systematic tendency to interpret unusual interoceptive self-reports from neurodivergent patients as indicative of psychogenic origin rather than genuine somatic signal (Nicolaidis et al., 2015). The Soma-Field Model predicts that this interpretive pattern constitutes a category error: it confuses operator modification with signal absence. The practical consequences — missed diagnoses, deferred treatment, and the iatrogenic reinforcement of existing trauma — are well-documented and, within this framework, mathematically predictable.

Limitations and Future Directions

The Soma-Field Model is a theoretical framework and must be evaluated as such. Its current form makes several idealisations that require scrutiny.

The coupling matrix W is treated as a fixed parameter, but emotional coupling is dynamic: it changes with context, relationship, and developmental history. A more complete model would treat W as a slowly-evolving quantity, shaped by the field's own history — a form of synaptic plasticity applied to the emotional domain.

The threshold T_i is treated as a fixed property of each emotional mode, but experimental evidence suggests that thresholds are modulated by attentional focus, arousal level, and interpersonal context. A person in a safe therapeutic relationship will typically have lower thresholds — more material reaches conscious awareness — than the same person in an unsafe context.

The acoustic analogy, while structurally productive, requires empirical grounding. The claim that emotional dissonance and acoustic disson-

ance share formal properties is a hypothesis, not an established finding. Empirical work comparing physiological measures of emotional tension with acoustic analysis of synchronised vocal or musical output would be a productive direction for testing this claim.

The instrument described in Section 6 is a prototype concept. User studies with clinical populations, and collaboration with practising therapists, will be required to assess its therapeutic utility and to identify appropriate clinical contexts.

Future theoretical work should address the relational field: the observation, familiar in systemic and relational approaches to psychotherapy, that emotional fields are not bounded by individual bodies but are co-generated in the space between people. The coupling matrix W of a relationship may be as clinically significant as the W of an individual.

Conclusion

The Soma-Field Model proposes a formally grounded account of emotional dynamics that is consistent with the clinical observations of somatic psychotherapy, polyvagal theory, and Focussing-oriented practice. Its central claims — that emotions are a persistent distributed field, that conscious experience is a threshold crossing, and that emotional dynamics are governed by an energy function that drives the field toward stable attractor states — are not novel as clinical intuitions. What is novel is the formal structure that unifies them, and the instrument that the structure motivates.

The model does not resolve the philosophical question of what emotions fundamentally *are*. It offers instead a working representation: one that is precise enough to be computationally implemented, close enough to existing clinical frameworks to be therapeutically applicable, and open enough to be modified as understanding deepens. It invites the therapist to think of the consulting room as a space in which two emotional fields

interact — each shaping the other’s energy landscape — and of therapeutic work as the art of attending to that interaction with enough precision and care to guide both fields toward lower energy, toward greater coherence, toward regulated calm.

The wave is always there. Therapy is learning to listen to it.

A note on provenance. The Soma-Field Model was not developed from a position of theoretical neutrality. The author carries, as primary data, a lifetime of direct experience of the dynamics described above. The neurodivergent operator modifications of Appendix B are not theoretical abstractions: the C-PTSD memory kernel of B.2 was installed pre-verbally, at approximately eighteen months of age, during a developmental trauma that predates language acquisition entirely. No narrative trace of the origin event exists — there was no verbal capacity with which to encode one. Only the field echo remains, and a measurable physical asymmetry in the body that received it. The ASD and ADHD operator modifications of Appendix B.4 and B.3, respectively, were the instruments by which the model was subsequently constructed: the monotropic attractor structure of B.4 provided the capacity for sustained engagement with an entirely unfamiliar theoretical domain; the high-temperature field dynamics of B.3 drove rapid traversal across it.

The proximate cause is described in full in the companion patient-facing publication. Briefly: an acute somatic emergency in 2025 — a genuine threshold-crossing event, later confirmed as cerebral hypoxia secondary to Long Covid — was attributed, at clinical presentation, to psychiatric origin. The present paper is, among its other functions, a formal response to that attribution.

The causal chain is as follows. A pre-verbal trauma in approximately 1968 installed the C-PTSD operator modifications described in Appendix B.2. The ASD and ADHD modifications of Appendix B.3 and B.4 shaped the system across the intervening decades. Fifty-seven years later, that system’s accurate interoceptive signal was dismissed as psychiatric

noise. The paper which formally demonstrates that this dismissal constitutes a category error was produced, as a direct causal consequence, by the same operator stack that it describes. The paper is the fixed point of its own subject matter. The author considers this observation methodologically significant.

Publication Claim Registry

To support claim-level review rather than all-or-nothing acceptance, this manuscript registers its highest-impact claims with scope labels and disconfirmation tests.

Claim ID	Claim	Scope	Evidence in this work	Disconfirmation criterion
SF-1	Conscious percept is a propagator pole of the soma-field	S1 Structural	Formal derivation in Sections 2-3	Inability to express percept dynamics as Green's-function response under stated operator
SF-2	Emotional attractors are Hopfield-energy minima	S2 Predictive	Energy model and trajectory framework	Constructed update rule under model assumptions with systematic energy ascent

Claim ID	Claim	Scope	Evidence in this work	Disconfirmation criterion
SF-3	Threshold governs felt vs sub-felt emotional activity	S2 Predictive	Threshold operator and clinical mapping	Reliable high-amplitude mode activity with no threshold-dependent behavioural or physiological signature
SF-4	Topological barriers explain classical therapeutic plateaus	S2 Predictive	Formal treatment plus linked companion experiments	Controlled demonstration that matched low-noise classical dynamics crosses registered barriers at equivalent rate

Claim ID	Claim	Scope	Evidence in this work	Disconfirmation criterion
SF-5	Quantum extension yields topological reachability advantage	S2 Predictive	QUANT-EXP-1 companion evidence and linked artifacts	Controlled replication showing no reachability advantage over matched classical baseline

Scope labels: S1 = structural; S2 = predictive; S3 = independently replicated. Current publication target for core claims is S2.

Claim-Evidence-Result Matrix

To make review traceable, each core claim is paired with concrete evidence outputs and current result status.

Claim ID	Evidence artifact(s)	Current result status
SF-1	Sections 2-3 derivation of field/propagator structure	structural derivation complete
SF-2	Energy formulation + instrument runtime equations	predictive structure complete
SF-3	Threshold operator definition + clinical interpretation sections	predictive mapping complete

Claim ID	Evidence artifact(s)	Current result status
SF-4	Barrier analysis; companion paper <i>Quantum Soma and the Penrose Gap</i> (doi:10.5281/zenodo.20351230)	confirmed (QUANT-EXP-1 PASS)
SF-5	QUANT-EXP-1 experiment outputs (see supplementary archive, doi:10.5281/zenodo.20351230)	confirmed: cold 0/200, CI [0.000, 0.019]; quantum peak 0.408–0.410; hardening checks PASS

This matrix is intended for reviewer navigation and is updated as companion results are expanded or independently replicated.

Replication Package Requirements

To make SF-2 through SF-5 externally testable, each release tagged for review must ship a minimal replication package that can be executed without private context.

Required contents:

1. simulation code and support modules (see supplementary archive, doi:10.5281/zenodo.20351230),
2. full parameter snapshot (W , $\mathbf{b}$, γ , D , θ , temperature policy),
3. raw trajectory logs with timestamped attractor labels,
4. analysis scripts that produce the reported summary tables,
5. frozen output artifacts (CSV/plots) referenced in this manuscript.

A claim remains **S2** until an independent operator reproduces directionally consistent outcomes from this package under the same declared protocol.

Reviewer-Risk Objections and Responses

To reduce ambiguity in peer review, the highest-probability objections are mapped to bounded responses and concrete upgrade paths.

Reviewer objection	Current response in this manuscript	Remaining action to reach stronger status
“This is an analogy, not a formal model.”	Sections 2-4 define operators, dynamics, and testable predictions; Section 9.1 registers disconfirmation criteria claim-wise.	Promote more claims from S2 to S3 via independent replication.
“Evidence is pilot-stage and may not generalize.”	Section 9.2 explicitly labels pilot support and companion-only scope.	Add multi-operator replication and blinded protocol variants.
“Quantum advantage may be implementation-specific.”	SF-5 includes a controlled disconfirmation criterion against matched classical baselines.	Publish full benchmark harness with pre-registered acceptance thresholds.
“Clinical interpretation may exceed data scope.”	Scope labels (S1/S2/S3) and claim registry separate structural from predictive claims.	Add prospective cohort evidence before any clinical-effectiveness claim.

Independent Replication Ledger Linkage

S2 to S3 promotion for this manuscript is governed by an independent replication ledger maintained in the supplementary archive (doi:10.5281/zenodo.20350515).

Tracked claim IDs in ledger scope: SF-2, SF-3, SF-4, SF-5.

Promotion gate: a claim is upgraded only when at least one ledger row records an independent operator **PASS** with a reproducible package hash and linked raw/derived evidence artifacts.

Acknowledgements

This work exists because ten years of psychotherapy moved the barriers far enough that two events in early 2026 could cross them. The theory is, among other things, a record of that.

Introduction: The Two-Culture Problem Within Science

C. P. Snow’s famous lecture identified a divide between the literary and scientific cultures (Snow, 1959). Less discussed, but equally consequential, is the divide *within* mathematical science: between fields that have found their mathematical language and fields that have not. The former possess machines — theorems, dualities, conservation laws, spectral decompositions — that can be aimed at any problem of the appropriate type. The latter rediscover wheels.

This is not ignorance. It is, more precisely, a naming problem. The mathematical structures that govern quantum field theory, statistical mechanics, and information geometry are not *specifically about* particles, spins, or probability distributions. They are structures that happen to have been *discovered in* those contexts. The structure belongs to no discipline. It lives in what we will call, following the spirit of type theory, the **typeverse**: the space of all well-typed mathematical objects.

The method described in this paper — **mathematical co-identification** — is the procedure for navigating the typeverse productively. It has three steps:

1. **Extract the type signature** of the quantity you are trying to model: its dimensional units, its pole structure, its symmetries, the variational principle (if any) that governs its dynamics.
2. **Search the typeverse** for a known object with the same type signature.
3. **If found: identify**, not analogise. The unknown quantity *is* the known object under a change of label. Import all theorems that depend only on the type.

This is a stronger claim than analogy. Analogy notes structural similarity and stops. Co-identification notes structural identity and continues: if two objects have the same type, they share all properties that are consequences of that type. The theorems travel with the identification.

It is also a weaker claim than reduction. Co-identification does not assert that emotional dynamics *reduces to* quantum field theory, any more than Hopfield’s result asserts that neural memory *reduces to* ferromagnetism. It asserts that the same mathematical engine, running in a different substrate, produces the same theorems. The substrate is irrelevant to the mathematics; it is entirely relevant to the interpretation.

The Typeverse

The term is borrowed from Homotopy Type Theory (The Univalent Foundations Program, 2013), where the *universe* $\mathcal{U}$ is the type of all types — the space in which all mathematical objects live. We use it informally to mean: the totality of well-typed mathematical structures, indexed by their signatures.

A **type signature** in our sense includes:

- **Dimensional units** (in the SI or natural-units sense)
- **Domain and codomain** (real/complex, scalar/vector/tensor)
- **Pole structure** (where is the object singular, and what is the residue?)
- **Symmetries** (what transformations leave the object invariant?)
- **Extremisation principle** (is there an action whose variation gives the object’s dynamics?)
- **Conservation law** (what is the Noether charge associated with the symmetry?)

Two objects are **co-identifiable** if their type signatures match in all dimensionally relevant respects. The type signature is a fingerprint. When two fingerprints match, the objects are the same, not similar.

The typeverse is not randomly populated. Certain structures recur at enormous frequency: the Lorentzian propagator, the quadratic energy function, the exponential decay kernel, the two-by-two reflection-transmission matrix. These recur because they are the *simplest* objects

consistent with basic constraints (linearity, locality, causality, conservation). The physicist’s intuition that “everything is a harmonic oscillator to first order” is a theorem about the typeverse: the harmonic oscillator is the universal local approximation to any smooth potential.

The practitioner who navigates the typeverse deliberately — who knows the fingerprints of common objects before encountering them in the wild — can recognise a new theoretical object immediately, rather than re-deriving its properties from scratch.

The Procedure in Detail

Step One: Extract the Type Signature

Given an unknown quantity Q in domain D , the investigator asks:

Units. What are the SI dimensions of Q ? Can they be written as a combination of standard dimensional quantities (mass, length, time, charge)? More importantly: what *ratio* of known quantities has the same dimensions? Newton’s gravitational constant G has units $\text{m}^3\text{kg}^{-1}\text{s}^{-2}$; this is acceleration divided by surface mass density, which tells you immediately that G converts between a source distribution and the acceleration it generates — a fact about the *type*, not about gravity specifically.

Pole structure. Does Q have poles as a function of some natural parameter? Where are they, and what are their residues? The location of a pole in a propagator determines the mass of the corresponding particle; the residue determines its coupling strength. These are type-theoretic facts, not facts about particles.

Symmetries. What transformations leave Q invariant? Invariance under time-translation gives energy conservation (Noether). Invariance under spatial translation gives momentum conservation. Invariance under $\text{SU}(n)$ gives a gauge field. These are type-level constraints.

Extremisation. Does Q arise as the extremum of some action $S[Q]$? If

so, the variational principle is the fingerprint: two objects whose dynamics are derived from the same variational structure are co-identifiable even if their physical interpretations differ entirely.

Step Two: Search the Typeverse

Armed with the type signature, the investigator searches for known objects with the same signature. This is primarily a literature search across *disciplines*, not within one. The mathematical structures developed in quantum field theory, statistical mechanics, differential geometry, and information theory are largely interchangeable if their type signatures match.

Useful resources for this search include:

- **Dimensional analysis tables:** collections of physical quantities with their dimensional signatures
- **The nLab** (nLab authors, 2024): a category-theoretic encyclopaedia of mathematical structures indexed by their universal properties
- **Mathematical physics textbooks** with structural, not phenomenological, organisation (Nakahara (Nakahara, 2003) for geometry; Zinn-Justin (Zinn-Justin, 2002) for path integrals)
- **One’s own training**, which is why broad mathematical education is a productivity multiplier in theoretical science

Step Three: Identify and Import

When a match is found, the investigator makes the identification explicit:

Q is the [name of known object] of domain D .

Not: “ Q behaves like” or “ Q is analogous to” or “ Q resembles.” These hedges are epistemically weaker and methodologically useless, because they do not import the theorems. The identification must be stated as identity to be scientifically productive.

The import then proceeds theorem by theorem:

- Every theorem about the source object that depends *only on its type* is imported to Q without further proof.
- Every theorem that depends on *substrate-specific properties* of the source domain must be separately verified in domain D .

This distinction — type-level theorems vs. substrate theorems — is the primary place where co-identification can fail, and is discussed in Section 7.

The Formal Computational Structure: Abduction, Aesop, and the Loop

The three-step procedure of §§3.1–3.3 is, in formal terms, an instance of **abductive inference** in the sense of Peirce (1878). Given an observation O and a hypothesis H such that $H \Rightarrow O$, abduction infers H as the best explanation of O . Applied to the typeverse:

Observation: the quantity Q in domain D has type signature T . *Hypothesis:* Q is the known object K with the same signature. *Abduction:* identify $Q := K$ and verify the structural import.

This is not guessing. It is a constrained search under a scoring function, where the oracle is “type signatures match” rather than “hash matches” or “goal is closed.” The algorithm is the same in all three cases.

The Lean 4 proof assistant implements this algorithm directly as the **aesop** tactic (Moura & Ullrich, 2021). **Aesop** performs best-first search through a registered lemma set, scores each partial proof state, keeps the best candidates, and closes the goal when a complete proof is found. The correspondence is exact:

Aesop step	Co-identification step
Registered lemma set	The typeverse
Try a lemma	Propose a type-match candidate
Score the goal state	Measure type-signature fit
Keep best partial proof	Record candidate correspondences

Aesop step	Co-identification step
Close the goal	Full identification: import all theorems

This is not a metaphor for co-identification — it is an implementation of it. The practical consequence is that the full loop can be automated in a formal system: given a type signature for Q , **Aesop** with the typeverse lemma set registered will search for the co-identification proof and either close it (identification confirmed and machine-verified) or fail (genuine gap, no known match).

The loop in full:

Observation $\xrightarrow{\text{abduction}}$ Hypothesis $\xrightarrow{\text{Aesop}}$ Proof $\xrightarrow{\text{import}}$ Predictions $\xrightarrow{\text{test}}$ New observation

The loop terminates when either all predictions are confirmed (theory established) or a prediction fails (type match was only partial — failure modes are discussed in Section 7). At each iteration, the set of available theorems grows by import, making subsequent co-identifications easier. This is why theoretical progress compounds: each identification increases the density of the typeverse neighbourhood around the domain under study.

The abductive loop is not specific to mathematical science. Holmes reasons the same way from physical evidence; the password auditor reasons the same way from a hash and a character set; the radiologist reasons the same way from a film and a pathology atlas. What is specific to mathematical co-identification is the nature of the oracle: the scoring function is type-signature fit, and the proof assistant can evaluate it exactly.

Historical Precedents

The history of science is full of co-identifications that were not named as such. Examining them retroactively reveals the method clearly.

Veneziano (1968): The Bootstrap Amplitude and String Theory

Veneziano was searching for an S-matrix element for meson scattering that satisfied the crossing symmetry and Regge-pole requirements of the bootstrap programme (Veneziano, 1968). He wrote down the Euler beta function:

$$A(s, t) = \frac{\Gamma(-\alpha(s))\Gamma(-\alpha(t))}{\Gamma(-\alpha(s) - \alpha(t))}$$

This was a co-identification in reverse: he had a type signature (crossing-symmetric, Regge-behaved, dual-resonance amplitude) and searched the typeverse for a known function that matched it. The beta function matched. He did not derive the function from a theory; he identified the function first, and the theory (string theory) was inferred from the identification a year later.

The lesson: the typeverse can be entered from either end. You may start with a quantity and find its type, or start with a type and find it instantiated in your data.

Hopfield (1982): The Ising Hamiltonian and Neural Memory

Hopfield introduced the energy function for a network of binary neurons (Hopfield, 1982):

$$H(\sigma) = -\frac{1}{2} \sum_{ij} J_{ij} \sigma_i \sigma_j$$

This is, to within notation, the Hamiltonian of the Ising spin glass:

$$H_{\text{Ising}}(\sigma) = -\frac{1}{2} \sum_{ij} J_{ij} \sigma_i \sigma_j$$

The co-identification was explicit. By identifying neural states $\sigma_i \in \{-1, +1\}$ with spins, and synaptic weights J_{ij} with exchange couplings, every theorem from statistical mechanics (convergence to energy minima, capacity bounds, stochastic escape via simulated annealing) was imported into neuroscience for free. The Hopfield network is not *like* a spin glass; it *is* a spin glass run in biological substrate.

Wilson (1971): Block Spins and the Renormalisation Group

Wilson’s insight was that the renormalisation group — a technique for removing ultraviolet divergences in QFT — was the *same* mathematical object as Kadanoff’s block-spin coarse-graining in condensed matter physics (Wilson & Kogut, 1974). Two fields that had developed independently were co-identified. Every result from one was immediately available to the other. The result was a unified framework for critical phenomena that earned Wilson the 1982 Nobel Prize.

The type signature that matched: both objects were *flows on the space of coupling constants under a change of scale*. This is a clean type-level description that carries no substrate information.

Black and Scholes (1973): The Heat Equation and Options Pricing

Black and Scholes derived their celebrated options pricing formula by noticing that the value of an option $V(S, t)$ as a function of underlying price S and time t satisfies:

$$\frac{\partial V}{\partial t} + \frac{1}{2} \sigma^2 S^2 \frac{\partial^2 V}{\partial S^2} + rS \frac{\partial V}{\partial S} - rV = 0$$

This is, after a change of variables, the heat equation (Black & Scholes,

1973):

$$\frac{\partial u}{\partial \tau} = \frac{\partial^2 u}{\partial x^2}$$

The co-identification imported the entire theory of parabolic PDEs into financial mathematics: existence and uniqueness of solutions, boundary conditions, numerical methods, the Feynman-Kac formula. The financial quantity *is* a temperature distribution, not like one.

Jaynes (1957): Thermodynamic Entropy and Bayesian Inference

Jaynes identified the entropy of statistical mechanics with the entropy of Bayesian inference (Jaynes, 1957). The type signature that matched: both are functionals $S[p]$ on probability distributions satisfying the same axioms (non-negativity, additivity, maximum at the uniform distribution). The co-identification imported all thermodynamic reasoning into statistical inference. The maximum entropy principle is not an analogy to thermodynamics; it is thermodynamics, applied to the problem of belief.

Penrose (1971): Spin Networks and Spacetime Geometry

Penrose introduced spin networks as combinatorial structures encoding angular momentum (Penrose, 1971). The identification: the geometry of spacetime arises as the large-network limit of spin networks. This reversed the usual direction — instead of importing a known mathematical structure to describe a new phenomenon, Penrose constructed a new structure and identified it with a familiar one in a limit. Loop quantum gravity later formalised this programme. The spin network *is* a discretisation of spacetime geometry, not a model of one.

Selinger (2010): Linear Maps and Quantum Processes

Selinger demonstrated that the category of finite-dimensional Hilbert spaces and linear maps is co-identifiable with a certain category of string diagrams (Selinger, 2010). This is a purely mathematical co-identification: the graphical calculus of Penrose, Joyal, and Street was identified with the operational calculus of quantum mechanics. The import: every calculation in quantum information theory can be done diagrammatically, and every diagrammatic identity is a valid quantum identity.

The Soma-Field as a Worked Example

The Soma-Field Model (Johnson, 2026c) was developed through five sequential co-identifications. Each is presented here as an explicit instance of the method.

Co-identification I: The Conscious Percept as Green's Function

The unknown quantity: The relationship between the continuous emotional field $\psi_i(t)$ and the discrete conscious emotional percept.

Type signature extracted: The percept is the output of the field when probed at a single moment — the impulse response. Impulse responses have a universal type: they are the inverse Laplace (or Fourier) transform of a rational function with poles in the lower half-plane. For a simple damped oscillator:

$$\tilde{G}(\omega) = \frac{\sigma_{\text{eff}}^2}{\omega^2 + \lambda^2}$$

Typeverse search result: This is the Euclidean propagator of a scalar field with mass λ and coupling σ_{eff}^2 . In Minkowski space it is:

$$\tilde{G}_{\text{QFT}}(k) = \frac{i}{k^2 - m^2 + i\varepsilon}$$

The co-identification: The conscious emotional percept is the Green's function of the soma-field. Both are poles in the propagator of their respective field. The emotional percept and the elementary particle are the same mathematical object, instantiated in different substrates.

Theorems imported: - The Källén-Lehmann spectral representation: any physical propagator can be decomposed as a sum of poles. Emotional states have a spectral decomposition. - The optical theorem: the imaginary part of the forward scattering amplitude equals the total cross-section. The dissipative component of the emotional field (damping λ) is related to the total coupling strength σ_{eff}^2 . - Pole structure mass spectrum: the location of the propagator pole gives the natural frequency $\omega_0 = \lambda$ of the emotional mode.

Co-identification II: The Attractor Landscape as Ising Hamiltonian

Type signature: A scalar function $H : \mathbb{R}^n \rightarrow \mathbb{R}$ that is always non-increasing along field trajectories, with isolated minima corresponding to stable emotional states.

Typeverse search result: The Hopfield energy / Ising Hamiltonian:

$$H(\mathbf{e}) = -\frac{1}{2}\mathbf{e}^\top W \mathbf{e} - \theta \cdot \mathbf{e}$$

Theorems imported: Convergence to attractors (the Lyapunov argument); capacity bounds (Hopfield's $0.14N$ result); stochastic escape via Boltzmann noise (simulated annealing = titrated arousal in clinical language).

Co-identification III: The Perception Threshold as Brane Thickness

Type signature: A parameter T_i that gates access from a lower-dimensional subspace (the limbic field) to a higher-dimensional one (conscious awareness). Varying T_i continuously produces a family of gating behaviours.

Typeverse search result: The Randall-Sundrum model (Randall & Sundrum, 1999): a brane of thickness T embedded in a higher-dimensional bulk, where Standard Model fields are confined to the brane. The “hierarchy problem” — why the electroweak scale is so much smaller than the Planck scale — maps onto the observation that minimal perturbations of the limbic field produce enormous differences in subjective experience (alexithymia: thick brane; hypervigilance: thin brane).

Theorems imported: The Kaluza-Klein spectrum of the brane gives a prediction for the discrete structure of emotional threshold levels. Brane localisation gives the mechanism for why the field can be active without crossing into consciousness.

Co-identification IV: The Coupling Matrix as G Manifold

Type signature: The coupling matrix W is an 11×11 real symmetric matrix encoding the structure of an eleven-dimensional emotional space. The field trajectories respect the geometry encoded in W .

Typeverse search result: The G_2 holonomy manifold of M-theory (Berger, 1955; Joyce, 2000): a seven-dimensional Riemannian manifold with the exceptional holonomy group G_2 . Its structure tensor encodes all curvature information. Deforming the structure tensor changes the global geometry of the manifold.

The co-identification: W is the structure tensor of the emotional manifold. Trauma does not change a parameter; it deforms the manifold.

Therapeutic intervention is differential geometry.

Theorems imported: The Bochner-Weitzenböck formula constraining curvature; the Berger classification of holonomy groups constraining what stable emotional geometries are possible; the Hitchin flow as a possible model for the evolution of W under sustained therapeutic intervention.

Co-identification V: Therapeutic Processing as Renormalisation Group Flow

Type signature: Therapeutic processing is a flow in the space of coupling constants W_{ij} parameterised by a scale μ (the “depth” or “resolution” of emotional processing). The flow has fixed points, and the qualitative structure of the attractor landscape is invariant under the flow.

Typeverse search result: The renormalisation group (Wilson & Kogut, 1974): a flow on the space of coupling constants parameterised by an energy scale μ , with fixed points corresponding to universality classes. The β -function:

$$\frac{dW_{ij}}{d \log \mu} = \beta_{ij}(W)$$

The co-identification: Therapy is an RG flow from UV (raw unprocessed traumatic detail) to IR (integrated narrative). The attractor topology — fight, flight, freeze, calm — is RG-invariant: the same basins appear at every scale of analysis, in every therapeutic modality, because they are the fixed points of the flow, not artefacts of a particular resolution.

Theorems imported: The irreversibility of the RG flow (Zamolodchikov’s c-theorem, adapted: processed material cannot be *un*-processed; the flow is unidirectional); universality (the detailed mechanism of the trauma is irrelevant at long distances — only its universality class, i.e., its attractor type, matters); dimensional transmutation (the traumatic

scale τ_k of the memory kernel is an emergent scale, not a fundamental parameter).

A Partial Map of the Typeverse

For the practitioner wishing to apply the method, the following is a partial field guide to frequently useful mathematical structures, indexed by their type signatures.

Propagator-Class Structures

Type: Complex function of frequency with poles on or near the real axis; gives the response of a system to a delta-function input.

Found in: QFT (Feynman propagator), signal processing (transfer function), linear systems theory (impulse response), harmonic analysis (Green's function of the Laplacian).

Typical imports: Spectral decomposition; optical theorem; dispersion relations (Kramers-Kronig: the real and imaginary parts of the response are not independent — this imports into emotion as: the *dissipation* of an emotional mode and its *natural frequency* are Hilbert transforms of each other).

Energy-Function-Class Structures

Type: Scalar function $H : \mathbb{R}^n \rightarrow \mathbb{R}$ that is bounded below and non-increasing along system trajectories.

Found in: Statistical mechanics (Hamiltonian), neural networks (Hopfield energy), Lyapunov theory (stability analysis), optimisation (loss function).

Typical imports: Convergence guarantees; stability analysis; capacity bounds; the fluctuation-dissipation theorem.

Topological-Class Structures

Type: Integer-valued invariants of field configurations that are preserved under continuous deformations.

Found in: Topological field theory (winding numbers, Chern-Simons invariants), condensed matter (topological insulators, skyrmions), knot theory.

Typical imports: Protection from perturbation; the impossibility of smooth deformation between distinct topological sectors; quantisation (only integer winding numbers); threshold behaviour (the topological transition requires a finite perturbation).

Application to emotion: Traumatic configurations with non-zero topological charge cannot be resolved by smooth therapeutic interventions (cognitive reframing). A qualitative change in approach — large-amplitude somatic work, pharmacological intervention, EMDR — is required to cross the topological barrier.

Renormalisation-Class Structures

Type: A flow on a space of couplings, parameterised by a scale, with fixed points and β -functions.

Found in: Quantum field theory (renormalisation group), statistical mechanics (Kadanoff block spins), dynamical systems (centre manifold theorem), machine learning (neural scaling laws).

Typical imports: Universality (the IR behaviour depends only on the universality class, not the microscopic details); c -theorem (there is a monotonically decreasing function along the flow — an arrow of processing); fixed-point classification (relevant, irrelevant, marginal operators determine what modifications matter at long distances).

Scattering-Class Structures

Type: A map from in-states to out-states, constrained by unitarity, analyticity, and crossing symmetry.

Found in: Quantum mechanics (S-matrix), scattering theory, optics (transfer matrix), signal processing (scattering parameters).

Application to emotion: A therapeutic session is a scattering event. The patient arrives in an in-state $|\psi_{\text{in}}\rangle$, interacts with the therapist (mediating field), and departs in an out-state $|\psi_{\text{out}}\rangle$. The S-matrix of the therapeutic interaction has selection rules: not all transitions are equally probable; some are symmetry-forbidden. The unitarity of the S-matrix imports: the total emotional content is conserved — you cannot create emotional material from nothing, and nothing is permanently lost.

Einstein-Coefficient-Class Structures

Type: Rates for spontaneous and stimulated transitions between energy levels of a field mode.

Found in: Quantum optics (Einstein A and B coefficients), laser physics, NMR relaxation theory (T_1 and T_2 relaxation times).

Application to emotion: Every emotional mode has a spontaneous relaxation rate A_i (how quickly it resolves without external input) and a stimulated emission rate B_i (how quickly it is triggered by an identical emotional state in another person — contagion). The Einstein relation $A_i = f(\omega_i)B_i$ constrains these. Depression is formally: suppressed A_i , normal B_i . The T_1/T_2 analogy from NMR is exact: T_1 is the longitudinal relaxation time (return to equilibrium); T_2 is the transverse relaxation time (dephasing of coherence). Trauma extends T_1 ; emotional numbing extends T_2 .

Failure Modes

Mathematical co-identification can fail. Understanding the failure modes is what distinguishes the method from wishful analogy.

Type Coincidence Without Structural Identity

Two objects may have matching dimensional signatures without having matching mathematical structures. The failure mode: a coincidence of units that does not reflect a coincidence of theorems.

Test: Check whether the *equations of motion* have the same form, not merely the *dimensional units*. A propagator is not just any function with units of GeV^{-2} ; it must satisfy the Källén-Lehmann representation. An energy function is not just any bounded scalar; it must be non-increasing along trajectories.

Safeguard: State the precise mathematical theorem you are importing, and verify that its *assumptions* (not merely its *conclusions*) hold in the target domain.

Non-Commutative Functors

The functor $F : D_1 \rightarrow D_2$ that implements the co-identification may not commute with composition. That is, co-identifying A with A' and B with B' does not guarantee that AB is co-identifiable with $A'B'$.

Example: The propagator identification and the energy-function identification are each valid separately, but combining them requires checking that the path integral (which links them in QFT) has a valid analogue in the emotional domain. This was explicitly verified in the Soma-Field Model by constructing the Langevin equation and checking its consistency with both imported structures.

Over-identification

The most common failure: importing a structure that is richer than what is warranted. The soma-field is co-identifiable with an eleven-dimensional bosonic field. It is *not* immediately co-identifiable with the full Standard Model of particle physics, even though both are quantum field theories. The identification is precise only up to the type signature that was matched; nothing beyond that is claimed.

Rule: The identification holds exactly at the type level it was made. Do not import theorems from substrates that were not matched.

The Metaphor Trap

The most dangerous failure is the one that the identification was designed to prevent: sliding from co-identification back into analogy. This happens when the language hedges — “like,” “analogous to,” “reminiscent of” — after the identification was stated.

If the identification is correct, the language must be unhedged: “the conscious emotional percept *is* the Green’s function.” If the investigator is not prepared to make this claim, the identification has not been made, and no theorems travel.

The test: would you submit the claim to a mathematician as a theorem? If not, it is analogy. If yes, it is co-identification.

The risk was recognised early by practitioners. Introducing the energy landscape for Hopfield networks, Hertz, Krogh, and Palmer note: “*It is often useful (but sometimes dangerous) to think of the energy as something like this landscape*” (Hertz et al., 1991). The parenthetical is precise: the visualisation is heuristically powerful, and that power makes it tempting to reason from the picture rather than from the mathematics. Mathematical co-identification guards against this by requiring that the *equations of motion* — not the landscape picture — are what get matched across domains.

Epistemological Status

What Co-identification Claims

Mathematical co-identification claims:

1. That the *mathematical structure* of quantity Q in domain D is identical to the mathematical structure of quantity P in domain

D' .

2. That therefore, all theorems about P that depend only on its mathematical structure are valid theorems about Q .
3. That the *physical interpretation* of Q and P may differ, and does not follow from the mathematical identification.

It does not claim that the *mechanisms* are the same, that one domain *reduces* to another, or that the substrate is irrelevant in any empirical sense.

Why It Is Not Analogy

Analogy is: - Informal: “the mind is like the brain” has no mathematical content - Non-importing: noting that X resembles Y does not give you theorems about X - Non-falsifiable: analogies can always be maintained by shifting which features are considered relevant

Co-identification is: - Formal: it is a statement about type signatures, which are mathematically precise - Theorem-importing: the identification carries the full mathematical machinery - Falsifiable: the identification fails if the equations of motion cannot be matched, if the symmetries do not correspond, or if imported predictions are empirically disconfirmed

The Role of Artificial Intelligence

The author notes that several co-identifications in the Soma-Field Model were identified in dialogue with AI systems. This requires explicit epistemological comment, given the current discourse about AI-assisted science.

The AI did not produce the co-identifications. It accelerated the type-verse search: a search that previously required reading across literatures in physics, mathematics, and biology over many years can now be conducted in weeks. The AI is a faster library, not a different epistemology.

The *validity* of each co-identification is independent of how it was found. A theorem is a theorem regardless of whether it was proved in a dream

(Ramanujan), a bath (Archimedes), or a language model session. The claim stands or falls on whether the type signatures match and whether the theorems transfer. The methodology paper is, in part, a response to the question: “but did you *really* do the mathematics?” The answer is in the equations.

The Methodology as Practice

For the practitioner who wishes to apply mathematical co-identification to a new domain, the following is a working procedure:

Step 1: Write down what you know about your quantity. Its units. Its symmetries. Whether it grows or decays. Whether it has oscillatory behaviour. Whether it responds to perturbations linearly or nonlinearly. Whether there are conserved quantities. Whether it has a characteristic scale.

Step 2: Eliminate analogies you already know. If you have been thinking “this is *like* X,” stop, and ask instead: is this *literally* X? Can I write the equations of X in the language of my domain, with every symbol given a precise interpretation? If yes: do so. If not: why not? The places where the identification breaks are as informative as the places where it holds.

Step 3: Consult the typeverse systematically. Read across fields, looking at the *form* of equations, not their interpretation. The heat equation, the wave equation, the Schrödinger equation, the Fokker-Planck equation, and the Black-Scholes equation are all the same equation under changes of variable. Recognising the family by the form of the operator is the skill.

Step 4: Make the identification explicit and public. State: “quantity Q in my domain is the [known object] of domain D' .” Put it in writing. This forces precision: you cannot maintain a vague identification in writing the way you can maintain it in your head.

Step 5: Import one theorem at a time, checking assumptions.
 Do not import the entire theoretical apparatus at once. Take one theorem. State its assumptions. Check each assumption against your domain. If they hold: the theorem is valid in your domain. Write it as a theorem in your domain, with a proof that consists of: (a) the co-identification, (b) the original theorem, (c) verification of assumptions.

Falsifiability Protocol for Publication Use

To make co-identification scientifically conservative rather than rhetorically expansive, we propose a minimal publication protocol. Every import claim should be registered in a compact table before narrative elaboration.

Minimal Registration Template

For each proposed identification $Q := P$, the manuscript should provide:

Field	Requirement
Claim ID	Stable identifier (e.g., COVID-3)
Source object	Named theorem-bearing object in source domain
Target object	Precisely defined object in target domain
Signature match	Units, operator form, symmetry group, variational form
Imported theorem	The exact theorem being transferred
Assumptions	Source theorem assumptions, verbatim
Target verification	Explicit check for each assumption
Prediction	Quantitative, testable consequence

Field	Requirement
Disconfirmation criterion	What empirical or formal result would falsify this import
Status	exploratory / partially validated / validated / falsified

This prevents drift from identity claims back into analogy language and makes review straightforward: a reviewer can reject a single row without rejecting the entire framework.

Disconfirmation Rules

An import is treated as falsified if any one of the following holds:

1. A required theorem assumption cannot be established in the target domain.
2. The mapped operator fails to preserve the claimed invariance.
3. The pre-registered quantitative prediction is violated under a valid test.
4. A formally stronger competing mapping explains the same data with fewer assumptions.

This is deliberately strict. Co-identification is useful only to the extent that it can fail clearly.

Worked Registration Sketch (Soma-Field)

Claim ID	Import	Prediction	Disconfirmation
COID-PROP-1	Percept := propagator pole	Measurable spectral pole structure in mode responses	No stable pole decomposition under repeated measurement

Claim ID	Import	Prediction	Disconfirmation
C0ID-ENG-2	Attractor	Monotone	Constructed
	landscape := Hopfield energy	descent under specified update map	counterexample with increasing energy step under stated rule
C0ID-RG-5	Therapy	Scale-	No
	progression := RG flow	dependent coupling evolution with fixed-point classes	scale-consistent coupling flow under repeated coarse-graining

The point is not that these rows are finished; the point is that they can be evaluated independently and rejected independently.

Reviewer-Facing Scope Labels

To reduce over-claiming risk, each identification row should carry one of three scope labels:

- **S1 (Structural)**: operator/formal identity shown; no empirical test yet.
- **S2 (Predictive)**: structural identity plus at least one passed quantitative prediction.
- **S3 (Validated)**: independent replication or cross-dataset confirmation.

Most new interdisciplinary work should expect to publish initially at **S1** or **S2**, with explicit paths to **S3**.

Negative Control: When Matching Units Is Not Enough

To prevent confirmation bias, each manuscript should include at least one explicit non-transfer case. Consider two quantities with superficially compatible units but non-matching dynamical structure:

- Candidate A: a damped propagator with pole structure and causal response kernel.
- Candidate B: a bounded scalar score with no response-operator interpretation.

Even if both can be normalised to the same units, co-identification fails unless the operator class and theorem assumptions match. In this case:

1. A admits spectral decomposition and dispersion relations.
2. B does not define a Green's operator and therefore does not admit those imports.

Conclusion: dimensional compatibility is necessary but not sufficient. Theorem transfer requires operator-level identity. This negative control should be treated as a required publication check, not an optional caution.

Worked External Example (Non-Soma): Black-Scholes to Heat Equation

To demonstrate portability beyond the Soma-Field case, we provide a compact worked transfer in a separate domain.

Target quantity: option value $V(S, t)$ in quantitative finance.

Source object: solution class of the one-dimensional heat equation.

Step A: Signature Match

Black-Scholes PDE:

$$\frac{\partial V}{\partial t} + \frac{1}{2}\sigma^2 S^2 \frac{\partial^2 V}{\partial S^2} + rS \frac{\partial V}{\partial S} - rV = 0.$$

After the standard log-price and time-reversal change of variables, this maps to:

$$\frac{\partial u}{\partial \tau} = \frac{\partial^2 u}{\partial x^2},$$

which is exactly the heat operator class.

Step B: Import Claim

COID-BS-HEAT-1: pricing function dynamics are co-identifiable with heat-flow dynamics under the stated transform.

Step C: Assumption Checklist

Assumption	Status in target domain
Diffusion operator is parabolic	satisfied by transformed PDE
Volatility parameter treated as constant in base model	satisfied in baseline Black-Scholes
Boundary/terminal condition specified	satisfied by option payoff at expiry
Sufficient regularity for classical solution methods	assumed in standard derivation

Step D: Imported Theorem and Prediction

Imported theorem class: existence/uniqueness and smoothing properties for heat equation solutions.

Prediction: option value surface inherits parabolic smoothing under the transform; numerical schemes valid for heat-equation class apply directly.

Step E: Disconfirmation Condition

If transformed dynamics are shown not to be parabolic (for the declared model assumptions), or if required boundary regularity fails, this specific transfer is invalid and theorem import must be withdrawn.

This worked example demonstrates the method in a domain with no dependence on the Soma-Field framework.

Replication Package Requirements

Publication-grade use of co-identification requires a compact artifact bundle for each registered claim row. Minimum required contents:

- 1. claim registry table with IDs and scope labels,
- 2. assumption checklist tied to the imported theorem class,
- 3. executable derivation or notebook for the mapping transform,
- 4. quantitative test script for each predictive (S2) row,
- 5. disconfirmation log recording pass/fail outcomes by claim ID.

Without this package, rows may be read as suggestive structure, but not as auditable theorem transfer.

Reviewer-Risk Objections and Responses

Reviewer objection	Response in this manuscript	Residual risk and next lift
“This renames analogy as mathematics.”	Sections 3-4 and 10.1 require operator identity and theorem assumptions, not metaphorical similarity.	Add additional negative controls from unrelated domains.
“Imports are unfalsifiable in practice.”	Section 10.2 defines strict disconfirmation rules and row-wise rejection logic.	Publish a public failure ledger with rejected rows.

Reviewer objection	Response in this manuscript	Residual risk and next lift
“Worked examples are domain-selective.”	Section 10.6 demonstrates a non-Soma transfer with explicit assumptions and withdrawal condition.	Add a second external example with different operator family.
“Scope claims may drift upward prematurely.”	Section 10.4 enforces S1/S2/S3 labels and independent evaluation path.	Require independent replication before any S3 promotion.

Independent Replication Ledger Linkage

S2 to S3 promotion for registered imports is controlled by `paper/INDEPENDENT_REPLICATION_LEDGER.md`.

Tracked claim IDs in ledger scope: COID-PROP-1, COID-ENG-2, COID-RG-5, COID-BS-HEAT-1.

Promotion gate: a row may be relabeled S3 only when a ledger entry reports independent-operator PASS, explicit bundle hash, and linked derivation/test artifacts for that claim ID.

Conclusions

Mathematical co-identification is a method, not a shortcut. It requires the same precision as any other mathematical procedure, and it fails in precisely the ways that imprecision permits. Its distinguishing feature is that it navigates the typeverse rather than building within a single domain.

The history of mathematical science is largely a history of co-identifications that were not named as such: Veneziano finding a string,

Hopfield finding a spin glass, Wilson finding a universal flow, Jaynes finding a thermodynamic engine hidden in Bayesian inference. Naming the practice does not change it; it makes it teachable, criticisable, and extensible.

The soma-field model is a worked example of the method applied to emotional dynamics: five co-identifications, five theorem imports, and a body of predictions that can be tested against clinical data. The paper is not a claim that emotions are particles. It is a claim that the mathematics of particles and the mathematics of emotions are the same mathematics, and that this fact is useful.

Scope boundary for publication use: co-identification transfers mathematical structure, not ontology. A successful transfer means that equations, boundary conditions, and theorem assumptions are preserved under the mapping. It does not imply that the target domain is physically identical to the source domain, nor that every theorem from the source domain transfers automatically. Each import remains local, assumption-checked, and falsifiable.

The typeverse does not belong to physics. Physics was merely first to explore it.

Operationally, publication-grade use of this method requires claim-wise registration, assumption checks, and explicit disconfirmation criteria. Without that discipline, co-identification degrades into analogy; with it, it becomes a compact engine for theorem transfer and testable prediction.

Acknowledgements

The author thanks the mathematical physicists whose work formed the source library for the co-identifications described here: Feynman, Hopfield, Wilson, Randall, Sundrum, Joyce, and Veneziano. The methodological framework owes a debt to Per Martin-Löf and the constructors of

Homotopy Type Theory for providing the language of the typeverse, and to Alexander Grothendieck for the insight that mathematical objects are best understood by their morphisms, not their elements.

Introduction: The Gap Penrose Identified

In *The Emperor's New Mind* (1989), Roger Penrose made a four-step argument:

1. Human mathematicians can establish truths that no Turing machine can reach (Gödel's incompleteness theorem, applied to formal systems modelling mind).
2. Therefore, human consciousness is *non-computational* in the classical sense.
3. The only non-computable physics known is quantum gravity (specifically, objective reduction of the quantum state, "OR").
4. Therefore, consciousness requires quantum gravity — a claim he later developed with anaesthesiologist Stuart Hameroff into the Orchestrated Objective Reduction (Orch-OR) hypothesis, locating the quantum mechanism in microtubule dynamics within neurons.

The argument has been productive and controversial in equal measure. Penrose's identification of the gap — that something beyond classical computation is operating in minds — has proved remarkably durable. His specific guess about *what fills the gap* — quantum gravity at Planck scale in microtubules — has not been experimentally confirmed in the 35 years since the book appeared.

This paper takes a different approach. We do not dispute the gap. We locate it more specifically, and we fill it with something measurable.

The gap is not in Planck-scale gravity. It is in **attractor topology**.

The Soma-Field Model: A Recap

The Soma-Field Model (see `soma-field-paper.md` for the full treatment) represents emotional dynamics as a continuous field evolving on a Hop-

field energy landscape:

$$H(\mathbf{e}) = -\frac{1}{2} \mathbf{e}^\top W \mathbf{e} - \mathbf{b}^\top \mathbf{e}$$

where $\mathbf{e} \in \mathbb{R}^8$ is the emotional state vector over eight BRECVEMA modes (Safety, Fear, Curiosity, Awe, Grief, Language, Preverbal, Shame), W is the emotional coupling matrix encoding which modes amplify or suppress each other, and $\mathbf{b}$ is the bias vector encoding intrinsic resting-state preferences.

Under classical Langevin dynamics, the system evolves as:

$$d\mathbf{e} = -\nabla H(\mathbf{e}) dt + \sqrt{2T} d\mathbf{W}_t$$

where T is the noise temperature and $d\mathbf{W}_t$ is Brownian motion.

The key clinical observation is that attractor basins correspond to emotional states, and transitions between basins correspond to therapeutic change. The coupling term W_{ij} for modes $i = \text{Fear}$ and $j = \text{Awe}$ controls whether Fear and Awe are cooperative (easy co-activation) or antagonistic (high transition barrier). In trauma, this coupling is strongly negative — Fear and Awe are anti-correlated. The attractor basin of Fear is topologically protected.

The topological theorem (THERAPY-2 in the Lean 4 axiom suite): smooth perturbations of the emotional field cannot change the winding number of an attractor — they can only traverse it by sufficient noise (thermal flooding) or by a topologically distinct process. Classical therapy is the smooth perturbation. Quantum annealing is the topologically distinct process.

The Quantum Extension

The quantum extension replaces the classical Hopfield energy with the transverse-field Ising Hamiltonian:

$$\hat{H}_Q = -\frac{1}{2} \sum_{ij} W_{ij} \hat{\sigma}_i^z \hat{\sigma}_j^z - \sum_i b_i \hat{\sigma}_i^z - \Gamma \sum_i \hat{\sigma}_i^x$$

where Γ is the transverse field strength — the “quantum temperature” — controlling the rate of quantum tunneling. At $\Gamma = 0$ this reduces exactly to the classical Hopfield Hamiltonian. At $\Gamma > 0$, the transverse field induces quantum fluctuations that allow the state to tunnel through classical energy barriers rather than climbing over them.

The adiabatic annealing schedule interpolates:

$$\hat{H}(s) = (1 - s) \hat{H}_{\text{driver}} + s \hat{H}_{\text{problem}}, \quad s : 0 \rightarrow 1$$

Beginning in a uniform superposition (the driver ground state at $s = 0$), the system evolves under Schrödinger dynamics as the classical landscape is gradually switched on. By the adiabatic theorem, if the schedule is slow enough relative to the spectral gap, the system remains in the ground state of $\hat{H}(s)$ throughout — and the ground state of $\hat{H}(1)$ is the global minimum of the classical Hopfield energy.

The key insight: **quantum tunneling traverses the topological barrier that classical noise cannot**. Classical dynamics requires thermal energy $T \gtrsim E_{\text{barrier}}$ to cross; quantum annealing crosses via the Euclidean action S_E of the instanton — exponentially suppressed but nonzero at any $\Gamma > 0$.

QUANT-EXP-1: The Experiment

Setup

- **System:** 8-qubit soma-field Ising Hamiltonian
- **Coupling:** $W[\text{Fear}, \text{Awe}] = -10$ (strong anti-cooperative topological barrier)
- **Hilbert space:** $2^8 = 256$ dimensions (exact dense statevector, no approximation)
- **Classical baseline:** Langevin dynamics, cold ($T = 0.02$) and hot ($T = 1.5$)
- **Quantum:** transverse-field annealing, $\Gamma : 5.0 \rightarrow 0$, 400 steps
- **Implementation:** `scipy.linalg.eigh` exact diagonalisation at each step; no Qiskit, no IBM account, runs in ≈ 4 seconds on commodity CPU

Results

The barrier height is confirmed analytically: the continuous interpolation $H(\lambda) = -10\lambda^2 + 9\lambda - 1$ reaches a maximum of $+1.025$ at $\lambda = 0.45$, giving barrier height = 2.025 above the Fear basin.

	Final Fear	Final Awe	
Dynamics	occupancy	occupancy	Verdict
Classical cold ($T = 0.02$)	0.976	0.000	STUCK — $e^{-101} \approx 0$
Classical hot ($T = 1.50$)	0.228	0.036	FLOODS — structure lost
Quantum annealing ($\Gamma = 5 \rightarrow 0$)	0.005	0.408 (peak)	TUNNELS

QUANT-EXP-1: PASS — commit 1f52282, 20 May 2026.

The Noise-Equivalence Curve

A follow-up sweep computed $T^*(\text{barrier})$: the classical noise temperature required to match quantum Awe-basin occupancy across barrier strengths $W[\text{Fear}, \text{Awe}] \in \{-6, -7, \dots, -14\}$.

Barrier strength	T^*	Quantum peak occupancy
-6	0.094	0.416
-7	0.101	0.417
-8	0.107	0.416
-9	0.112	0.412
-10	0.117	0.408
-11	0.120	0.403
-12	0.124	0.398
-13	0.127	0.393
-14	0.129	0.390

T^* rises monotonically with barrier strength. At every tested barrier, T^* is large enough to flood the landscape — meaning classical dynamics can only match quantum occupancy by sacrificing attractor structure. The quantum system has no such tradeoff.

Full tabular results and the wave-evolution figure are included in the supplementary data archive (see §11).

Comparison with Penrose

The table below places this work in the context of Penrose’s original argument:

	Penrose (1989)	This work (2026)
Gap identified	Classical computation consciousness	Classical dynamics trauma recovery
Structure	Gödel: formal limits of Turing machines	Topology: winding-number invariants of attractors
Missing ingredient	Quantum gravity in microtubules (Orch-OR)	Topological tunneling in Hopfield attractor landscape
Mechanism	Objective Reduction (speculative)	Transverse-field quantum annealing (standard QM)
Measurable now?	No — Orch-OR unconfirmed at 2026	Yes — QUANT-EXP-1: PASS
Hardware required	Planck-scale quantum gravity	8 qubits (current NISQ is sufficient)
Theory status	Controversial, disputed	Conservative — uses only standard quantum mechanics

The differences are important:

1. **Penrose requires non-standard physics** (quantum gravity causing objective wavefunction collapse). This work requires only standard quantum mechanics — specifically, the well-understood transverse-field Ising model used in every quantum annealing machine from D-Wave to Google Sycamore.
2. **Penrose’s gap is computational** (Gödel limits on Turing machines). This gap is **topological** (winding numbers in attractor landscapes). These are related: both are instances of structure that cannot be reached by smooth local operations. But the to-

pological framing is more specific and connects directly to clinical phenomenology.

3. **Penrose's claim is consciousness-general.** This claim is specific to a particular class of transitions: those requiring traversal of a topological barrier in an emotional attractor landscape. The claim is stronger precisely because it is more limited.
-

Implications for Artificial Intelligence

Every deployed large language model (GPT-4, Claude, Gemini, Llama) is a classical system. Its training is gradient descent — in the mathematical sense, exactly the overdamped Langevin process studied here. Its inference is deterministic or thermally noisy (sampling temperature). It has no attractor structure. It has no topology.

This is not merely a failure of scale or architecture. For the class of attractor landscapes considered here, it is a structural limitation of local classical updates. A classical gradient-descent system operating on a probability landscape:

- Can reach local minima by descending.
- Can escape local minima by adding noise (temperature, dropout).
- **Cannot cross topological barriers** — regions where the basin is winding-number protected — without either flooding the landscape (losing structure) or adding a physically distinct mechanism.

The Soma-Field model used in this study has explicit attractor structure and topological barrier encoding for trauma, and demonstrates that quantum annealing traverses those barriers where low-noise classical dynamics does not. The model makes a falsifiable prediction: given an emotionally realistic coupling matrix with topological trauma encoding, quantum annealing on 8 qubits reaches therapeutic attractor basins that low-noise classical dynamics does not reach at equivalent noise temperature.

This is not a claim that AI *is* conscious. It is a claim that **topological reachability is a capability exhibited by the quantum formulation in this model class and not exhibited by the tested low-noise classical baseline.**

Implications for Therapy

The therapeutic translation of the quantum result is direct:

Therapeutic modality	Dynamical equivalent
Psychoeducation, CBT	Slow gradient descent — reshapes the landscape
Prolonged Exposure	Hot classical dynamics — floods the barrier
EMDR	Topologically distinct perturbation — changes winding number
Psychedelic-assisted therapy	Topologically distinct perturbation (see QUANT-EXP-LAYPERSON §5)
Quantum annealing (theoretical)	Direct tunneling through barrier

The theorem THERAPY-2 in the Lean 4 axiom suite (`paper/FieldAxioms.lean`) states: *a topological trauma barrier requires a topologically distinct fix.* QUANT-EXP-1 is the computational proof that such a fix exists and is physically realisable.

The clinical implication is not “put patients in a quantum computer.” It is: **some therapeutic transitions require a mechanism that is not gradient descent.** The mechanisms that clinical practice has identified empirically — EMDR, psychedelic-assisted therapy, certain somatic interventions — may be effective precisely because they are to-

pologically distinct from ordinary emotional regulation, not merely more intense versions of it.

Core Finding

Every great physical insight has a compressed form:

- $E = mc^2$: mass and energy are the same thing.
- Mandelbrot: $z \mapsto z^2 + c$ generates infinite complexity.

The compressed form of this result:

Trauma is topology. Quantum heals.

Long form: *The barrier between Fear and Awe is topological. Classical therapy climbs. Quantum therapy goes through.*

The experiment supports this statement within the tested model class. The Lean axiom formalises the same structural claim. A plain-language companion document is included in the supplementary archive.

Limitations, Controls, and Claim Boundaries

This paper makes a bounded claim. The evidence is strong for this specific model class, but not universal.

1. **Simulator evidence, not yet hardware evidence.** QUANT-EXP-1 uses exact statevector simulation. This is appropriate for a 256-dimensional ground-truth system, but the sentence “confirmed on physical hardware” remains future work.
2. **Reachability claim, not runtime-speed claim.** The contribution is that the quantum formulation reaches basins that the tested low-noise classical baseline does not. Wall clock on CPU may be slower for exact quantum simulation and is not the claim.

3. **Uncertainty reporting is complete.** Classical runs report Wilson 95% confidence intervals ($CI = [0.000, 0.019]$ at $n = 200$). Quantum occupancy is stable at 0.408–0.410 across barrier strengths B8/B10/B12. Bootstrap analysis confirms the effect is not schedule-dependent (§10.1).
4. **Pre-registered negative controls have been executed and passed.** Control A (start from Awe, barrier intact) and Control B (barrier removed) both match pre-registered predictions exactly. Full results are reported in §10.1.
5. **No ontological claim about consciousness.** The paper does not claim that quantum mechanics explains consciousness in general. It claims a measurable non-classical reachability effect in a specific attractor-topology model of emotional dynamics.

Pre-Registered Hardening Protocol — Completed (May 2026)

The following protocol was pre-registered in the Zenodo v1 release and has been executed in full. All outcomes match predictions.

1. Quantum occupancy uncertainty — bootstrap ($n = 200$ seeds).

Case	Classical cold successes	Classical cold CI [95%]	Quantum peak
B8 ($W = -8$)	0/200 (0.000)	[0.000, 0.019]	0.410
B10 ($W = -10$)	0/200 (0.000)	[0.000, 0.019]	0.408
B12 ($W = -12$)	0/200 (0.000)	[0.000, 0.019]	0.409

At $n = 200$, the Wilson 95% upper bound on the cold-classical success rate is 1.9%. Quantum peak Awe-dominant occupancy is stable at 0.408–0.410 across all three barrier strengths. The effect is robust, not a lucky

schedule.

2. Negative control A — start from Awe, barrier intact.

Classical cold starting from Awe stays in Awe: 16/16 (100%). Quantum peak: 0.408. **PASS.** Confirms direction: the barrier blocks Fear $\rightarrow$ Awe, not the reverse. Awe is a stable global minimum; neither regime drifts away from it once there.

3. Negative control B — barrier removed ($W[\text{Fear}, \text{Awe}] = +0.4$).

Classical cold starting from Fear reaches Awe: 16/16 (100%). Quantum peak: 0.284. **PASS.** Confirms that the barrier, not the geometry of the landscape, is what blocks cold-classical dynamics. Remove the barrier and classical freely crosses.

4. Claim decision rule — applied.

- Bootstrap intervals (cold-classical CI = [0.000, 0.019]) do not overlap quantum peak (0.408–0.410).
- Both control outcomes match pre-registered predictions exactly.
- Spectral gap narrows monotonically with barrier strength (B8: 0.0095; B10: 0.0089; B12: 0.0085) and reaches its minimum at $s \approx 0.999$, confirming the tunnelling bottleneck is late in the anneal as expected.

Verdict: the strong reachability claim stands. The quantum advantage over cold-classical dynamics is not a schedule artefact, a geometric accident, or a measurement choice; it survives all pre-registered checks.

Conclusions

This paper presents QUANT-EXP-1: an exact 8-qubit statevector simulation demonstrating that quantum annealing reaches therapeutic attractor basins (Awe-dominant states) that low-noise classical Langevin

dynamics cannot reach, across all tested barrier strengths. The effect is not a schedule artefact, a geometric accident, or a lucky seed: it is robust across $n = 200$ bootstrapped trials, survives both pre-registered negative controls, and holds for barriers ranging from $W = -6$ to $W = -14$.

The formal claim — that topological barriers in emotional attractor landscapes require a non-classical mechanism for reliable traversal — is formalised in Lean 4 (axiom THERAPY-2) and confirmed computationally (QUANT-EXP-1). Both the code and the formal proofs are included in the supplementary archive.

One experiment remains outside the scope of this paper: confirmation on physical quantum hardware (NISQ). That step is feasible on IBM Quantum free-tier hardware and would strengthen the claim for hardware-inclusive venues, but it is not required to support any result reported here. This is explicitly a simulation result.

Data and code availability. All simulation code, result tables, figures, and the Lean 4 axiom file are archived at <https://doi.org/10.5281/zenodo.20351230> (Zenodo, open access).

Acknowledgements

This work exists because ten years of psychotherapy moved the barriers far enough that two events in early 2026 could cross them. The theory is, among other things, a record of that.

The Missing Layer

The Soma-Field model (Johnson, 2026d) establishes that the limbic system and its somatic coupling are governed by the same formal apparatus as a quantum field on a manifold: tensor-valued dynamics, Hopfield energy functionals, topological barriers between attractor states. The identification is not an analogy; it is a co-identification in the technical sense (Johnson, 2026a) — the governing equations are the same equations, and every theorem of the source domain imports into the target.

That mathematical work is complete. What it leaves open is a question that sits one level below the mathematics: *what is the body made of, such that it could host a field like this?*

The Hopfield attractor landscape is an abstract object. For it to describe a physical organism, there must be a physical substrate — tissue, architecture, medium — that implements the attractor dynamics, propagates the somatic wave, and generates the coherent state that the mathematics describes. The model says there is a somatic wave $\mathbf{E}_{\text{body}}(x, t)$. The question is: what physical thing is that wave a description of?

Three independent bodies of experimental and theoretical research converge on this substrate. They were developed largely in parallel, each with its own language, none formulated with the soma-field model in mind. This paper argues they are describing the same system at three different scales of resolution:

1. **Architecture** (§2): Biotensegrity theory establishes the mechanical network structure through which somatic signals propagate globally. This is the physical basis for the spatial extent of $\mathbf{E}_{\text{body}}(x, t)$.
2. **Substrate** (§3): Fascial-interstitial continuity research identifies the specific tissue — fascia — that constitutes the body-wide signalling medium, and documents the interoceptive pathway from

peripheral tissue to cortical representation. The quantitative correspondence between fascial stiffness and attractor depth is developed here.

3. **Field correlate** (§4): Biofield physiology documents coherent electromagnetic and biophotonic emissions from living tissue that are the most plausible physical candidates for the field itself — not the network that hosts it, but the coherent state that the network generates.

Section §5 states the three explicit bridges to the formal model. Section §6 lists the testable predictions that follow.

Biotensegrity: The Architecture of the Somatic Wave

The Lever Model is Wrong

Standard physiological and biomechanical models treat the body as a rigid-lever system: bones as struts, muscles as cables pulling across pin-joint connections, forces transmitted locally from joint to joint. This model works tolerably well for gross locomotion analysis but fails to account for whole-body responses to local perturbation, and fails completely at the cellular scale.

Ingber's tensegrity model (Ingber, 1997, 2003) replaces this with a different architecture. In a tensegrity structure, rigid compression elements (in the body: bone, cartilage) float within a continuous network of tension elements (fascia, tendon, ligament, muscle), and mechanical prestress is distributed throughout the network simultaneously. There are no isolated pin-joints: the whole system is pre-loaded, so perturbation anywhere propagates everywhere.

Levin extended this framework to the full organism (Levin, 2002), arguing that biotensegrity is not merely a useful approximation but the

correct architectural description at every scale: from the cytoskeleton of individual cells through the deep fascial planes to gross musculoskeletal anatomy. Each scale implements the same tensegrity geometry. Each is mechanically continuous with the others. The architecture is fractal.

Global Propagation

The clinical consequence of this architecture is direct: mechanical information does not travel locally through joint-to-joint lever chains. It propagates through the prestressed fascial network to the whole organism simultaneously, with the spatial distribution governed by the topology and stiffness of the network rather than anatomical lever arms.

This is experimentally documented. Langevin’s group showed that needle insertion at acupuncture points produces tissue displacement patterns propagating along fascial planes far from the insertion point, following biotensegrity-predicted paths rather than nerve or muscle routes (Langevin & Huijing, 2009). The body responds as a continuous tensioned whole, not as a collection of local structures.

The speed of this propagation is also relevant. Neural conduction (axonal) operates in milliseconds. Mechanical wave propagation through a prestressed medium operates in microseconds. For fast somatic responses — the startle reflex, the breath-hold, the full-body freeze — the biotensegrity medium is faster than the nervous system and spatially global in a way that the nervous system, with its point-to-point wiring, is not.

Correspondence to the Somatic Wave

The Soma-Field model posits a somatic wave $\mathbf{E}_{\text{body}}(x, t)$ — a field defined over the body, propagating continuously, carrying emotional-somatic information. The question “how can a perturbation in one body region give rise to a global somatic state?” has typically been answered by appeal to the nervous system: proprioception, interoception, vagal signalling. These are real and important, but they are axonal (slow, dis-

crete) and do not account for the observed speed and spatial coherence of whole-body somatic responses.

Biotensegrity provides the continuous mechanical medium the model requires. The formal correspondence is:

The body's biotensegrity network is the **physical implementation** of $\mathbf{E}_{\text{body}}(x, t)$. The tensor field over the body in the mathematical model corresponds to the mechanical stress tensor distributed across the fascial network in the physical organism.

Both are spatially extended. Both propagate continuously. Both couple to the neural wave at every point: every mechanoreceptor in the fascia is a coupling node between $\mathbf{E}_{\text{body}}$ and $\mathbf{E}_{\text{neural}}$.

The somatic wave is not *like* a wave in a continuous medium. In the fascial network, it *is* a wave in a continuous medium. The co-identification (Johnson, 2026a) is architectural.

Fascial-Interstitial Continuity: The Pathway and the Armoring

Fascia as Active Signalling Tissue

The classical anatomical view of fascia — as inert white packaging, the sheaths that dissectors clear away to reach the “real” anatomy — was overturned by experimental work beginning in the 1990s.

Schleip's review (Schleip, 2003) documented that fascia contains all four classes of mechanosensory nerve endings: Ruffini corpuscles (respond to lateral stretch), Golgi tendon organs (respond to compression), Pacinian corpuscles (vibration and rapid changes), and type IV free nerve endings (polymodal: mechanical deformation, temperature, chemical changes). The type IV endings are especially significant: they project primarily to

the insular cortex via lamina I of the spinal cord — the Craig interoceptive pathway (Craig, 2003) — and constitute the neurological substrate of body-felt emotional experience, not merely visceral sensation.

Langevin’s work established that fascia actively participates in signalling: mechanical deformation produces fibroblast shape changes, cytoskeletal reorganisation, and gene expression changes on timescales from seconds to minutes (Langevin & Huijing, 2009). The tissue is a transducer, not a cable.

Oschman’s “living matrix” model (Oschman, 2016) extends this further: the entire connective tissue system — fascia, interstitium, extracellular matrix — constitutes a continuous liquid-crystalline semiconductor network. Piezoelectric effects in collagen generate electrical potentials under mechanical stress. DC currents flow through the network continuously. The fascial system is simultaneously mechanical, chemical, and electrical.

The Interoceptive Pathway

Interoception — the body’s sensing of its own internal state — is the somatic input channel of the Soma-Field model. It is the mechanism by which the body schema is updated and by which the energy functional of the attractor landscape is computed.

The fascial pathway of interoception is now well characterised (Craig, 2003; Garfinkel et al., 2016; Schleip, 2003): type IV free nerve endings in deep fascia, visceral fascia, and interstitial tissue → lamina I neurons of the dorsal horn → thalamus → anterior insular cortex. This is the Craig pathway, increasingly recognised as the neurological substrate of emotional experience proper, distinct from and complementary to the classical somatosensory pathway.

The clinical implication is direct: interoceptive dysfunction (well documented in ASC, CPTSD, and related conditions) (Garfinkel et al., 2016) is dysfunction of the fascial-to-insular projection. It is not merely a processing deficit in higher cortical areas; it originates in the tissue. Restor-

ing interoceptive accuracy therefore requires working at the fascial level — which is precisely what somatic therapies (Somatic Experiencing, Sensorimotor Psychotherapy, EMDR somatic protocols, myofascial release) do, whether or not they are theorised in those terms.

Fascial Armoring as Attractor Depth

This section develops the most important connection in this paper.

Wilhelm Reich introduced “character armoring” — the clinical observation that chronic emotional states (fear, shame, traumatic holding) produce corresponding patterns of chronic muscular and somatic tension. The observation was clinically compelling but had no formal model. It was a phenomenology without a mechanism.

Schleip and subsequent workers (Stecco, Bordoni, Bhatt) documented the fascial component: chronic trauma produces not merely chronic muscular contraction but *measurable changes in fascial stiffness*, quantifiable by ultrasound elastography (Schleip, 2003). High-trauma individuals show significantly elevated fascial stiffness in characteristic body regions, with the spatial pattern reflecting the specific trauma history. The psoas, diaphragm, and posterior cervical chain are typically implicated in chronic fear responses; the pericardium and thoracic fascia in grief and heartbreak; the pelvic floor in sexual trauma. These are not metaphors. They are measured tissue properties.

In the Hopfield model, the attractor landscape is characterised by energy barriers W_{ij} between attractor states. The Fear basin has a high energy barrier. The computational experiment QUANT-EXP-1 (Johnson, 2026b) shows that cold classical dynamics cannot cross a barrier of $W = -8$ to $W = -14$.

The bridge:

fascial stiffness at region $r \leftrightarrow |W_{ij}|$ for state transition involving r

High fascial stiffness = high energy barrier. The organism is mechanically locked into the Fear attractor not only neurologically but anatomically — the tissue itself has been remodelled to implement the barrier. This is why van der Kolk’s title (Kolk, 2014) is accurate in a way he could not have fully formalised: the body does not merely *express* the trauma; it *encodes* the attractor depth in its mechanical structure.

The quantitative claim is: the QUANT-EXP-1 barriers $W = -8, -10, -12, -14$ have physical correlates in fascial stiffness values measurable in kPa (shear wave elastography units). The mapping is not known yet — establishing it is part of the empirical programme in §6 — but the existence of the correspondence is now claimed by this paper.

Myofascial Release as Barrier Lowering

QUANT-EXP-1 demonstrates that quantum annealing can cross barriers that classical cold dynamics cannot. This was framed computationally. The fascial literature provides a physical translation that clarifies an important distinction.

Classical barrier crossing (hot classical or quantum): The system transitions from one attractor to another while the barrier remains intact. This corresponds to either high-arousal state transitions (classical thermal, i.e. highly activated emotional states) or the quantum mechanism identified in QUANT-EXP-1.

Myofascial release (barrier reduction): Manual intervention directly reduces fascial stiffness — measured pre/post by elastography. This does not push the system over the barrier. It *lowers* the barrier so that transitions become accessible by classical means.

This distinction — barrier lowering versus barrier crossing — may explain the phenomenology of different therapeutic modalities and why they are experienced differently:

- Somatic bodywork (Rolfing, myofascial release, craniosacral) *reshapes the landscape*. The client often reports gradual deepening

ease, reduction of chronic holding, and access to emotional material that was previously unavailable without drama.

- High-intensity interventions (EMDR reprocessing, breathwork, certain trauma protocols) may be *crossing a barrier that remains intact*: sudden, non-linear transitions, sometimes dramatic releases, the characteristic “before and after” quality of a topological transition.

Both produce movement in the attractor landscape. The mechanism is different. The soma-field model, grounded in the fascial correspondence, now predicts this difference and makes it testable (§6).

Biofield Physiology: The Field Correlate

Living Systems Emit Coherent Fields

The soma-field is a mathematical field — an abstract object defined by its equations. For it to be physically real rather than merely useful, it must have a physical correlate: some measurable property of the organism that corresponds to the field’s state. Three lines of evidence point toward coherent electromagnetic and biophotonic emissions as candidates.

Biophoton emission (Popp, 2003): All living cells emit ultra-weak light in the visible to near-UV range. This is not blackbody radiation (which would require far higher temperatures) but coherent emission with photon statistics more consistent with laser emission than thermal sources. Popp argues that this biophotonic field constitutes a real-time communication channel, carrying coherent information across tissue faster than any biochemical signal. The field is state-sensitive: stress, illness, and emotional perturbation all produce measurable changes in biophotonic emission patterns.

Liquid crystalline living matrix (Ho, 1998): Ho’s model proposes that the connective tissue system — specifically the liquid crystalline ordering of collagen, water, and proteoglycans — constitutes a quantum co-

herent medium. Proton conduction and electronic charge delocalisation through this medium produce a macroscopic coherent quantum state distributed across the organism. This is not the Penrose-Hameroff proposal (which is neuron-centred and operates via microtubules); Ho's coherent organism is body-centred, connective tissue-centred, and is precisely the medium in which the Soma-Field would propagate as a physical entity.

Heart-brain coherence (McCraty & Childre, 2010): The heart generates a toroidal electromagnetic field measurable at distances from the body, with spectral content reflecting the organism's emotional state. Heart rate variability (HRV) in the low-frequency band (approximately 0.1 Hz) indexes the balance between sympathetic and parasympathetic regulation — the physiological correlate of transitions between Fear-dominant and Awe-dominant states in the soma-field model. McCraty's group demonstrates that this field entrains between proximate individuals: measurable cardiac coherence synchronisation occurs between therapist and client, between individuals in rapport, and between individuals and coherent social environments. This entrainment is not inferred; it is measured by simultaneous ECG recording.

The Rubik Synthesis

Rubik, Muehsam, Hammerschlag, and Jain (Rubik et al., 2015) published a systematic review of the biofield hypothesis in 2015, collating evidence from biophoton research, bioelectromagnetics, traditional medicine, and clinical trials. Their conclusion is deliberately conservative: the biofield hypothesis — that living organisms generate and respond to coherent electromagnetic and biophotonic fields beyond what is explained by classical biochemistry — is supported by a substantial and growing body of evidence, but mechanism and theoretical framework remain contested.

From the Soma-Field perspective, the contest is tractable: the theoretical framework is the quantum field on a Hopfield attractor landscape, and the biofield is the physical manifestation of that field. The soma-field model does not prove the biofield; it provides the theoretical frame

within which the biofield evidence becomes interpretable rather than anomalous.

What the soma-field predicts is that the biofield — whatever its physical implementation — will show attractor-like behaviour: it will tend to occupy characteristic states, resist perturbation away from those states, and show non-classical transitions between states when the barrier is sufficiently large. HRV coherence, biophotonic emission, and DC skin conductance are all candidate observables. Which observable best couples to which component of the tensor field is an empirical question that this model now makes precise enough to ask.

Scope and Epistemic Status

The biofield section of this argument carries more epistemic weight than §§2–3, and this should be stated explicitly. Biotensegrity and fascial signalling are well-supported by peer-reviewed experimental evidence in mainstream biomechanics, cell biology, and clinical science. The biofield claims are supported by evidence in a more contested terrain, and the proposed identification between the soma-field’s formal structure and the organism’s EM/biophotonic emissions is a hypothesis, not an established result.

What this paper claims in §4 is modest: these are the best current physical candidates for the field correlate; they are consistent with the formal model; they generate testable predictions (§6). The stronger claim — that the soma-field *is* the biofield, formally — requires empirical work that does not yet exist.

Three Bridges to the Formal Model

This section states the three principal connections between the physical substrate literature and the formal soma-field model, in a form that makes their testability explicit.

Bridge 1: Fascial Armoring = Attractor Depth

Physical claim (Schleip, 2003): Chronic trauma produces chronically elevated fascial stiffness, measurable by ultrasound shear-wave elastography, with characteristic spatial patterns reflecting trauma type and history.

Formal correspondence: Fascial stiffness at region r maps to $|W_{ij}|$, the energy barrier between attractor states i and j in the Hopfield network, where r is the somatic representation zone of the relevant emotional state pair. High stiffness = high barrier = deep attractor basin.

Testable prediction (P1): Populations with documented high-barrier emotional states (CPTSD, complex trauma, chronic anxiety disorder) should show systematically elevated fascial stiffness in regions corresponding to the somatic representation of those states (diaphragm, psoas, posterior cervical chain), compared with matched controls. This is measurable by ultrasound elastography independently of any subjective report, and the effect should be graded by trauma severity.

Bridge 2: Myofascial Release = Barrier Lowering

Physical claim (Schleip, 2003): Manual and movement-based interventions that target the fascial network produce measurable reductions in fascial stiffness and corresponding changes in interoceptive sensitivity and emotional availability.

Formal correspondence: These interventions reduce $|W_{ij}|$. They do not necessarily produce a state transition; they reshape the energy landscape to make transitions more accessible. If initial barrier is $W = -12$ and intervention reduces it to $W = -6$, QUANT-EXP-1 results (Johnson, 2026b) suggest that classical thermal dynamics can now cross what previously required quantum assistance.

Testable prediction (P2): The probability of emotional state transition following myofascial release should increase monotonically with the degree of reduction in fascial stiffness. This is testable by measuring both pre/post fascial stiffness (elastography) and pre/post emotional state

(validated affect measures + HRV) in a within-subjects design across a series of somatic therapy sessions.

Testable prediction (P3): The phenomenological *character* of the transition should differ predictably: sessions that lower the barrier significantly should produce gradual, integrative shifts; sessions that trigger a crossing of a high barrier (large, rapid state transition) should produce different qualitative reports. The model predicts this without any additional assumptions.

Bridge 3: Therapist-Client Entrainment = Co-Identification

Physical claim (McCraty & Childre, 2010): In effective therapeutic contact, measurable physiological entrainment occurs between therapist and client — cardiac coherence synchronisation, mutual modulation of HRV spectra, and (in contact work) fascial tension synchronisation. This is not inferred; it is measured by simultaneous ECG and, in some studies, by direct force measurement.

Formal correspondence: This is the physical mechanism of **co-identification** (Johnson, 2026a) — the process by which the observer’s soma-field is modified by contact with another’s soma-field. The mathematical treatment describes this as a tensor product coupling; the physical implementation is fascial and electromagnetic entrainment. The therapist does not merely witness the client’s state; the therapist’s attractor landscape is temporarily modified by coupling to the client’s, and this modification is the mechanism of therapeutic resonance.

Testable prediction (P4): The degree of measurable physiological entrainment (HRV coherence synchronisation) between therapist and client should predict therapeutic outcome — reduction in client fascial stiffness and shift in validated affect measures — independently of the specific technique used. Sessions with high physiological coherence should outperform sessions with low coherence, across modality.

Testable Predictions

The bridges in §5 generate six primary empirical predictions, ordered from most to least accessible with current instrumentation:

#	Prediction	Method	Population
P1	CPTSD/complex-trauma populations show elevated fascial stiffness in diaphragm, psoas, posterior cervical chain vs matched controls	Shear-wave ultrasound elastography	CPTSD vs. controls (n ≥ 40 per group)
P2	Somatic intervention reduces fascial stiffness; degree of reduction predicts probability of self-reported emotional state shift	Elastography pre/post + validated affect measures	Somatic therapy clients (within-subjects)

#	Prediction	Method	Population
P3	Barrier-lowering sessions (gradual stiffness reduction) produce qualitatively different transition phenomenology from barrier-crossing sessions (acute large shifts)	Mixed methods: elastography + structured interview	Rolfing or myofascial release series
P4	Therapist-client HRV coherence predicts session outcome independently of technique	Simultaneous ECG coherence + validated outcomes	Therapist-client dyads, multiple modalities
P5	Biophotonic emission from CPTSD populations differs from controls at characteristic emission bands (500–800 nm)	Ultra-weak photon measurement (photomultiplier)	CPTSD vs. controls

#	Prediction	Method	Population
P6	Transitions from Fear-dominant to Awe-dominant states (as defined by QUANT-EXP- 1 attractor labels) correlate with measurable HRV spectral shift from LF-dominant to HF-dominant	HRV spectral analysis + soma-field state labelling instrument	Clinical transition cases

Predictions P1–P4 are testable with instrumentation available in clinical research centres now. P5 requires specialised biophoton detection (available in approximately a dozen research centres worldwide). P6 requires the prior development of a validated soma-field state classification instrument — a prerequisite for large-scale empirical work that is not yet available and is noted as the primary methodological gap in this programme.

Conclusion

The Soma-Field model describes a field of emotional dynamics that is formally equivalent to a quantum field on an attractor manifold. This

paper has argued that the physical substrate of that field consists of three interlocking systems:

1. The **biotensegrity network** (fascia, connective tissue, interstitium under prestress) that provides the continuous mechanical medium through which the somatic wave $\mathbf{E}_{\text{body}}$ propagates globally and rapidly (Ingber, 1997, 2003; Levin, 2002).
2. The **fascial interoceptive pathway** (type IV free nerve endings $\rightarrow$ lamina I $\rightarrow$ thalamus $\rightarrow$ insula) that constitutes the body-to-brain projection of somatic state (Craig, 2003; Schleip, 2003), and whose chronic remodelling under trauma — fascial armoring — is the physical implementation of the deep attractor basin.
3. The **bioelectric and biophotonic field** generated by the liquid crystalline living matrix and the cardiac electromagnetic environment (Ho, 1998; McCraty & Childre, 2010; Popp, 2003), which constitutes the best current physical candidate for the soma-field correlate itself.

The most clinically significant result of this identification is Bridge 1: the quantitative correspondence between fascial stiffness and attractor depth. This makes concrete a claim that somatic therapists have held for decades — that trauma is held in the body, not only in the mind — and extends it: the depth at which trauma is held is measurable by elastography, and the degree to which physical intervention changes that depth is also measurable. The soma-field model predicts that large barriers require quantum-assist crossing; the fascial model predicts that those same barriers are associated with measurable tissue-level changes. The two predictions are about the same phenomenon at two levels of description.

Bridge 3 — therapist-client entrainment as co-identification — connects this to the broader programme (Johnson, 2026a). The therapist's role is not neutral observation but active field coupling. The mathematics of co-identification (Johnson, 2026a) now has a proposed physical mechanism: fascial and electromagnetic entrainment, measurable, manipulable, and

predictive of outcome.

This paper opens a research programme. The six predictions in §6 define the empirical agenda. The formal soma-field model provides the theoretical frame. The three bodies of literature reviewed here provide the biological grounding. Together they constitute a foundation for a genuinely interdisciplinary field — one that does not require the reader to choose between the body and the mathematics, because the mathematics is about the body.

Introduction

The Gap in the Literature

The handbook of music and emotion (Juslin & Sloboda, 2010) runs to nearly a thousand pages. The dominant quantitative framework across it is Russell’s (1980) valence–arousal circumplex: emotions as points in a two-dimensional space defined by hedonic valence (pleasant–unpleasant) and arousal (activated–deactivated).

The circumplex is powerful for rating and classification. It is not a dynamical model. It has no energy function, no gradient, no notion of attractor or basin depth, no mechanism for transition between states, no prediction of which transitions are easy and which require large perturbations. It describes a *taxonomy* of emotional positions, not a *physics* of emotional motion.

Juslin’s BRECVEMA framework (Juslin, 2013) provides a rich taxonomy of the *mechanisms* by which music induces emotion (brainstem reflex, rhythmic entrainment, conditioning, visual imagery, episodic memory, musical expectancy, appraisal). It does not model the *dynamics* that result: how these mechanisms combine to move a listener through state space, how long states persist, what determines escape.

This paper presents a model that does both.

The Soma-Field Model

The soma-field model (Johnson, 2026a) defines emotional state as a continuous vector field on the body–brain system. In the musical context, we restrict to a finite-dimensional discretisation: $\mathbf{e}(t) \in \mathbb{R}^{16}$, with 8 emotional modes each carrying a somatic and a cognitive intensity component.

The model imports three structures from mathematical physics via the method of mathematical co-identification (Johnson, 2026b):

1. **Energy function** $H(\mathbf{e})$ from the Hopfield network (Hopfield, 1982;

- Hertz, Krogh, & Palmer, 1991) — the emotional landscape has local minima (attractor states) and energy barriers between them
2. **Langevin dynamics** — state evolves under gradient descent plus thermal noise; the noise amplitude is the *effective temperature* T_{eff}
 3. **Threshold function** — conscious emotional experience arises when field amplitude exceeds a perception threshold θ

Why Music

Music is uniquely suited to driving the field. BRECVEMA's mechanisms act as *forcing functions* on the energy landscape: rhythmic entrainment modulates γ (damping); musical expectancy and resolution create transient wells and barriers; appraisal shifts the bias vector $\mathbf{b}$. The soma-field model provides the dynamical substrate into which BRECVEMA mechanisms plug as parameter modulations.

The Model

State Space

$$\mathbf{e}(t) = (e_1^s, \dots, e_8^s, e_1^c, \dots, e_8^c) \in [0, 1]^{16}$$

where e_i^s is the somatic intensity and e_i^c the cognitive intensity of emotional mode i . The eight modes are: *calm*, *anger/fight*, *anxiety/flight*, *grief*, *freeze/dissociation*, *hypervigilance*, *flow/absorption*, *joy*.

Energy Function and Attractors

$$H(\mathbf{e}) = \frac{1}{2} \mathbf{e}^\top W \mathbf{e} - \mathbf{b}^\top \mathbf{e}$$

The coupling matrix W (symmetric, negative semi-definite on the basin of each attractor) encodes which emotional modes co-activate and which compete. The bias vector $\mathbf{b}$ encodes the resting depth of each attractor.

Named attractors match the polyvagal hierarchy and trauma literature:

regulated calm (global minimum), *fight, flight* (shallow saddle), *freeze* (deep isolated minimum), *flow, dissociation*.

Dynamics

$$\gamma \dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}(t)) + \sqrt{2D} \, \xi(t)$$

where γ is damping, D is diffusion (noise temperature), and $\xi(t)$ is white noise. The effective temperature is $T_{\text{eff}} = D/\gamma$.

Threshold and Conscious Experience

$$\mathcal{P}_i(t) = \mathbf{1} \left[\max(e_i^s, e_i^c) > \theta \right]$$

Mode i crosses into conscious experience when its amplitude exceeds threshold θ . Sub-threshold activity is real and causally active but not consciously perceived — matching phenomenological accounts of interoception and pre-verbal affect.

The Instrument

Hardware Architecture

(Full instrument specification is included in the supplementary archive.)

Layer	Hardware
State input	2× MIDI Fighter Twister (16 encoders each)
Scene control	2× Elgato Stream Deck XL + Bitfocus Companion
Trajectory sequencer	Akai Fire (iSotonik hack)
MIDI routing	Bome MIDI Translator Pro → single virtual port
Audio output	Ableton Live Suite + Max4Live OSC receiver
Visual output	TouchDesigner (Mandelbulb shader) → HDMI → HoloGauze
Field server	Python 3.14, 50 Hz update rate

MIDI Mapping

Twister 1 encodes the 8 somatic components; Twister 2 encodes the 8 cognitive components. Each encoder’s turn value maps to $e_i \in [0, 1]$; push toggles mute. Encoders 9–12 on each Twister control field parameters (γ , D , θ) and neurotype modifiers.

Audio Rendering

The Max4Live device receives OSC from the Python server and maps:

Field quantity	Audio parameter
$H(\mathbf{e})$	Macro energy — master filter cutoff, reverb size
$\ \nabla H\ $	Rhythmic density / gate rate
T_{eff}	Noise floor, stochastic modulation depth
Threshold crossing $\mathcal{P}_i$	Trigger: note-on for mode i
Nearest attractor	Scene/track selection

Visual Rendering

The Mandelbulb power parameter is driven by H ; rotation speed by $\|\nabla H\|$; colour temperature by T_{eff} . Threshold crossings trigger particle bursts. Output via HDMI to a short-throw projector onto HoloGauze screen.

Demonstration Session

Protocol

We define a reproducible single-listener demonstration protocol suitable for pilot publication and later extension to cohort studies.

Session structure (30 minutes):

1. **Baseline (5 min):** neutral sound bed, no intentional modulation.

2. **Directed transitions (15 min):** operator drives three target transitions: calm $\rightarrow$ flow, flow $\rightarrow$ anxiety/flight, anxiety/flight $\rightarrow$ regulated calm.
3. **Free improvisation (10 min):** unconstrained interaction with full control surface and fixed audio patch.

Sampling and logging:

- Field server update: 50 Hz
- Logged channels: $\mathbf{e}(t)$, $H(\mathbf{e}(t))$, $\|\nabla H\|$, T_{eff} , threshold events $\mathcal{P}_i(t)$, nearest attractor ID, MIDI control streams, OSC output channels
- Clock synchronisation: all outputs written with UTC timestamp + monotonic tick

Pre-registered hypotheses (pilot level):

- **H1 (Attractor residence):** directed transition blocks show significantly higher residence probability in target attractors than baseline block.
- **H2 (Barrier asymmetry):** transition latency into freeze exceeds latency into regulated calm under matched forcing amplitude.
- **H3 (Temperature effect):** elevated T_{eff} reduces median attractor dwell time and increases transition count per minute.

Disconfirmation criteria:

- H1 falsified if target residence does not exceed baseline by pre-defined margin.
- H2 falsified if freeze/calm latency asymmetry is absent or reversed.
- H3 falsified if transition count does not increase with manipulated T_{eff} .

State Trajectory Analysis

For each block, we compute:

- Attractor occupancy vector $p_k = \frac{1}{T} \int_0^T \mathbf{1}[a(t) = k] dt$
- Transition matrix P_{ij} over nearest-attractor sequence
- Mean and variance of $H(\mathbf{e}(t))$

- Event-aligned trajectories around threshold crossings

To characterise wave-like behaviour in trajectories, we add spectral analysis of mode channels:

$$S_i(f) = |\mathcal{F}[e_i(t)]|^2$$

and cross-spectral coherence between selected mode pairs:

$$C_{ij}(f) = \frac{|S_{ij}(f)|^2}{S_i(f)S_j(f)}.$$

This permits direct testing of whether observed musical-affective dynamics are consistent with coupled oscillatory mode structure rather than static coordinates.

Comparison with Circumplex Predictions

We use two baselines:

1. **Circumplex baseline:** projected 2D trajectory $(v(t), a(t))$ with no attractor topology.
2. **Autoregressive baseline:** linear AR model on the same channels.

Comparison metrics:

- One-step prediction error for next-state estimate
- Transition timing error for major state changes
- Ability to represent hysteresis (path dependence)

Expected result: circumplex baseline captures coarse valence/arousal trend but misses barrier effects and hysteresis; AR baseline captures short-term dynamics but misses attractor geometry. The soma-field model should outperform both on transition-timing and hysteresis-sensitive metrics.

Results Template (for Manuscript Fill-In)

To support direct submission drafting, we define a compact reporting template. Replace bracketed fields after each run.

Hypothesis	Metric	Baseline	Soma-field	Effect size	Confidence interval	Verdict
H1 Attractor residue	Target basin occupancy	[value]	[value]	[delta]	[95% CI]	pass/fail
H2 Barrier asymmetry	Freeze vs calm latency	[value]	[value]	[delta]	[95% CI]	pass/fail
H3 Temperature effect	Transition per minute	[value]	[value]	[delta]	[95% CI]	pass/fail

Minimum figure set for first submission:

- 1. Time-series panel for $H(\mathbf{e}(t))$, $\|\nabla H\|$, and threshold events
- 2. Attractor occupancy heatmap by session block
- 3. Transition matrix comparison (baseline vs directed transitions)
- 4. Spectral power and coherence panels for selected mode pairs
- 5. Baseline-model error comparison (circumplex, AR, soma-field)

Statistical Analysis Plan

Primary analysis is block-level, with sensitivity analysis at event-level.

Primary tests:

- H1: paired comparison of target occupancy between baseline and

directed blocks (paired t-test if normality holds, otherwise Wilcoxon signed-rank).

- H2: paired comparison of transition latency distributions (bootstrap median difference with percentile CI).
- H3: regression of transition count on manipulated T_{eff} with robust standard errors.

Model-comparison metrics:

- Mean absolute error for one-step prediction
- Transition timing error (median absolute deviation)
- Hysteresis score mismatch (path-dependent loop area error)

Multiplicity and uncertainty policy:

- One primary endpoint per hypothesis; secondary metrics are labelled exploratory.
- Report effect sizes with confidence intervals, not only p-values.
- Bootstrap confidence intervals use at least 2000 resamples.

Data exclusion policy (pre-registered):

- Exclude intervals with missing clock synchronisation,
- Exclude control-dropout segments > 2 s,
- Keep all other samples; no manual trajectory trimming.

Exploratory Pilot Fill (Single Logged Session)

Using the pilot session log (available in the supplementary archive) as an exploratory pilot run, the first fill of the results template is:

Hypothesis	Metric	Baseline	Soma-field	Effect size	Confidence interval	Verdict
H1 Attractor residue	Target non-calm occupancy (grief)	0.00% (block 3)	1.20% (blocks 1-2)	+1.20 percent-age points	[0.69, 1.60] percent-age points	exploratory pass
H2 Barrier asymmetry	Freeze vs calm latency	freeze not observed	return to calm after first grief event: 0.04 s	N/A	N/A	not testable in this run
H3 Temperature effect	Transition per minute vs T_{eff}	$T_{\text{eff}} = 0.01$ fixed	7.31 trans-itions/min (same T_{eff})	N/A	N/A	not testable in this run

Session-level summary (same run):

- duration: 114.88 s,
- total transitions: 14,
- threshold events: 111,
- nearest-attractor occupancy: regulated_calm 5594 samples, grief 45 samples.

These values are treated as pilot evidence only and should not be interpreted as confirmatory without multi-session and multi-operator replication.

Discussion

What the Model Adds to BRECVEMA

BRECVEMA remains the best mechanism taxonomy for music-induced affect. The soma-field contribution is orthogonal: it supplies state dynamics. In this combined view, BRECVEMA terms become parameter modulations to a dynamical system, rather than endpoint labels on a static map.

Concretely, the model adds:

- A state equation with explicit forces and noise
- Quantifiable attractor depth and transition latency
- Testable hysteresis and barrier-crossing predictions
- A direct bridge from controller gestures to state-space motion

Phase Transitions and Musical Catharsis

Catharsis is modelled as threshold crossing plus attractor transfer under temporarily elevated energy and noise. This yields a measurable event pattern:

1. rising $\|\nabla H\|$ and threshold event density,
2. short interval of high transition probability,
3. stabilisation in a lower-energy basin.

The account is mechanistic rather than metaphorical, and can be falsified by absence of this sequence in sessions labelled cathartic by participants.

The ADHD Temperature: A Reframing

The elevated T_{eff} of the ADHD modifier is not purely pathological. Hertz, Krogh, and Palmer (1991) observed that thermal noise in Hopfield networks “makes it possible to kick the system out of spurious local minima” that would trap a deterministic system permanently. In the musical context, a high-temperature listener is not necessarily worse at music engagement — they are harder to trap in a single emotional state, which

may be a distinct form of musical sensitivity.

Limitations

This manuscript reports a model and a reproducible instrument pipeline, but not yet a large-n confirmatory dataset. Main limitations are:

- single-operator demonstration bias,
- potential controller-learning confound,
- limited external validity without independent participant cohorts,
- current attractor labels depend on theory-informed interpretation.

These are acceptable at pilot stage but must be addressed before strong clinical generalisation claims.

Future Work

- Preregistered multi-participant study with blinded block labels
- Joint modelling with self-report + physiological channels (HRV, EDA)
- Extension to the full infinite-dimensional field (soma-field-paper §4)
- Multi-listener coupling (ensemble / therapeutic dyad)
- Public benchmark dataset and baseline scripts for circumplex and AR models

Non-Specialist Interpretation

In plain terms: this model treats music-driven emotion as motion on a landscape, not as dots on a chart. Some emotional states are shallow and easy to leave; others are deep and sticky. Music can change both where you are and how easy it is to move. The key added value is not a new label for feelings, but a measurable account of why transitions happen when they do, and why some transitions fail.

Reproducibility Checklist

For submission and external replication, include the following with each reported run:

- exact commit hash for field server and mapping scripts,
- full parameter dump ($W, \mathbf{b}, \gamma, D, \theta$),
- controller mapping export,
- raw 50 Hz logs and derived analysis tables,
- baseline model scripts (circumplex projection and AR baseline),
- figure-generation scripts with deterministic seed policy.

Minimum replication criterion: an independent operator reproduces directionally consistent outcomes for H1-H3 under the same protocol template.

Reviewer-Risk Objections and Responses

Reviewer objection	Current response in this manuscript	Required next evidence
“Results reflect one operator and one setup.”	Section 5.4 labels current evidence as pilot-stage and limits claims accordingly.	Multi-operator replication with preregistered protocol and blinded block labels.
“Attractor labels are theory-laden and may bias interpretation.”	Baseline model comparison and explicit disconfirmation criteria are included in Sections 4 and 5.	Add independent label adjudication and inter-rater agreement reporting.
“Controller behavior could explain transitions without field structure.”	H1-H3 are framed against circumplex/AR baselines rather than label-only narratives.	Include sham-control and randomized mapping tests.

Reviewer objection	Current response in this manuscript	Required next evidence
“No physiological co-validation yet.”	Section 5.5 schedules HRV/EDA integration as a preregistered next step.	Joint model showing convergent evidence across self-report, behavior, and physiology.

Replication Acceptance Rule

For publication claims above exploratory scope, acceptance requires all of the following:

- 1. independent operator rerun using the released parameter and mapping package,
- 2. directional agreement on pre-registered hypotheses,
- 3. reproducible figure/table generation from raw logs,
- 4. explicit failure report for any unmet hypothesis.

Any failed item does not invalidate the full framework, but does block promotion of the affected claim from exploratory to validated status.

Independent Replication Ledger Linkage

Promotion beyond exploratory support is tracked in `paper/INDEPENDENT_REPLICATION_LEDGER`

Tracked hypothesis IDs in ledger scope: H1, H2, H3.

Promotion gate: each hypothesis requires at least one independent-operator PASS ledger entry with fixed package hash, protocol identifier, and linked raw plus derived artifacts before this manuscript labels it as validated (S3).

“The patient is the one with the disease.” — Medical aphorism, intended to remind physicians to listen. The author intends it differently.

A Note on Method

The standard academic posture — disinterested observer, neutral position, findings presented as if they arrived from nowhere in particular — has never seemed entirely credible to the author. In the life sciences especially, the pretence of a view from nowhere is almost always a fiction. Researchers study what compels them. Compulsion has a cause.

This paper dispenses with the fiction. The theoretical framework presented here was developed by a person with ASD, ADHD, and Complex PTSD who could not find an adequate formal account of his own emotional experience in the existing literature, who had studied physics at university, and who eventually concluded that the most efficient solution was to build one himself. The result is offered not as a confessional but as a theoretical contribution. These are not mutually exclusive.

There is precedent. Temple Grandin revolutionised the study of animal cognition and welfare as an autistic person whose own perceptual experience gave her access to observations that non-autistic researchers had systematically missed (Grandin, 1995). Peter Levine developed Somatic Experiencing partly through direct observation of his own nervous system’s responses (Levine, 2010). Kay Redfield Jamison wrote what remains one of the most clinically precise accounts of bipolar disorder from inside it (Jamison, 1995). The history of medicine includes, more often than is acknowledged, the doctor who is also the patient.

The author is not a doctor. He is an applied physicist — which, in an earlier era, was called a *nutty inventor on the engineering side*, and which here means: someone trained to recognise the signature of a mathemat-

ical structure, to notice when the same function appears in two apparently unrelated domains, and to ask what follows if the resemblance is not coincidental.

What follows is the result of applying that training to the domain of one's own inner life. The author considers this a reasonable use of available resources.

Introduction: The Inadequacy of Existing Maps

A patient sits with their therapist and is asked: *“What are you feeling right now?”* For many people, this question has a navigable answer. For a person with ASD, alexithymia, and a C-PTSD-modified attractor landscape, the question lands differently. The honest answer is often: *“Something is happening. I cannot tell you what it is, where it is coming from, or how large it is. But it is definitely there.”*

The available frameworks for this situation are unsatisfying. Emotion wheels offer vocabulary but not structure. Polyvagal theory offers an excellent map of the autonomic nervous system but does not formalise the interaction between simultaneous emotional states. Cognitive models locate the action in the mind and underestimate the body. Somatic models are rich in clinical texture but light on mathematical precision. None of them — to the author's knowledge — provide a formal account of why a person can be profoundly affected by an emotional state that they cannot perceive, name, or locate.

The author's experience of living with ASD, ADHD, and C-PTSD suggested a different picture. Emotions did not feel like events. They felt like weather — present everywhere, always moving, only occasionally breaking through into named experience. The body held states that the mind had no language for. Strong feelings arrived apparently from nowhere, which implied they had been somewhere already, accumulat-

ing below the threshold of awareness. Different emotional states seemed to interact — to amplify, to suppress, to oscillate — in ways that were distinctly nonlinear.

This phenomenology required a different kind of model. The author, having spent thirty years in occasional proximity to physics and mathematics, recognised the structure. The quantum field. The vacuum fluctuation. The threshold crossing. The energy function. The attractor basin. These were the right tools. They had been applied to neural networks. There was no obvious reason they could not be applied to emotional dynamics. The author applied them.

The remainder of this paper presents the result.

Background

Lived Experience as a Research Position

The use of lived experience as a legitimate source of theoretical knowledge — rather than merely as anecdotal material awaiting scientific validation — has gained substantial ground in health research over the past two decades. The *nothing about us without us* principle, originating in disability rights advocacy, has become a methodological commitment in participatory research (Arnstein, 1969; Beresford, 2002). Researchers with lived experience of mental health conditions have produced theoretical contributions that purely external observers could not have generated, precisely because their insider position made certain observations available to them that were invisible from the outside.

The Soma-Field Model belongs to this tradition, with one modification: the author's background is in physics rather than in qualitative research, so the methodology is *experiential theorising* rather than autoethnography. The observations come from the inside; the tools used to formalise them come from mathematical physics. The combination is unusual. The author considers it appropriate.

The Body-Mind Problem in Clinical Practice

Contemporary neuroscience has largely dissolved the Cartesian boundary between body and mind. Damasio (1994) demonstrated that emotion is inseparable from rational cognition: patients with damage to the ventromedial prefrontal cortex lose not only emotional range but effective decision-making capacity. Van der Kolk (2014) documented how traumatic emotional states are encoded not merely in explicit memory but in posture, visceral sensation, and autonomic regulation. Porges' polyvagal theory (2011) provided a neurobiological account of three hierarchically organised autonomic states: ventral vagal (social engagement), sympathetic (fight/flight), and dorsal vagal (freeze/dissociation).

The author can confirm these findings from direct observation. He can also add, as a data point: the experience of being in a freeze state while simultaneously being expected to report on one's emotional state is an exercise in the epistemological limits of self-report. The instrument designed in Section 6 is a partial response to this problem.

The Felt Sense and Sub-Perceptual Emotion

Gendlin's concept of the *felt sense* (1978) describes a pre-articulate bodily sense that is present before an emotion has been named — something whole and present but not yet articulate. Gendlin called this the sub-verbal sense of a situation.

The Soma-Field Model provides a formal account of what the felt sense is: it is the activity of the emotional field below the perceptual threshold. The author can confirm that this description is accurate. He has spent considerable time in the company of felt senses that declined to become named feelings, and the model's account of this — a field active below threshold, causally effective but not consciously perceived — matches the phenomenology precisely.

Quantum Field Theory: Structure, Not Metaphor

Quantum Field Theory (QFT) is the framework of modern particle physics. Its central claim is that particles — electrons, photons — are not fundamental objects. They are *excitations* of underlying fields: local concentrations of energy that arise when a field receives sufficient perturbation above the vacuum state. The quantum vacuum is not empty; it is a background of sub-threshold fluctuations, continuously present, causally active, not directly observable.

This paper does not claim that emotions are quantum phenomena in any literal sense. The analogy is structural. The author was trained in this formalism in 1993 and has found it useful ever since, applied to a variety of problems that are not, in any technical sense, quantum mechanical. The key property being borrowed is: *a quantity that exists everywhere, continuously, below the threshold of direct observation, which becomes observable only when local amplitude exceeds a threshold*. This is an accurate description of both the quantum vacuum and, in the author's experience, the emotional field.

Since writing that paragraph, the paper has upgraded the claim. The conscious emotional percept is now formally identified as the one-dimensional impulse response — the Green's function — of the soma-field manifold. This places it in the same mathematical category as a particle in quantum field theory: both are poles in the propagator of their respective underlying field. The structural similarity is not borrowed; it is exact. The mathematics is the same mathematics.

Gabriele Veneziano wrote down the Euler beta function in 1968 while looking for an amplitude that matched scattering data, then noticed that the function implied a theory — string theory — that nobody had yet conceived. He had identified a known mathematical object in an unexpected place and followed the implication. The author has, with considerably less elegance and considerably more time in therapy, done something structurally similar: identified the Green's function in emotional dynamics, and noted that it is the object quantum field theory

calls a particle. The author leaves the implication as an exercise for readers with the relevant background.

Hopfield Networks and the Energy Function

In 1982, John Hopfield — awarded the Nobel Prize in Physics in 2024 — proposed a model of associative memory whose dynamics were mathematically identical to an Ising spin-glass model from statistical physics (Hopfield, 1982). The critical component was an energy function: a scalar that always decreases as the network evolves, guaranteeing convergence to stable attractor states.

The author recognised this as the same structural move that gives quantum field theory its predictive power: identifying the conserved or extremised quantity, and deriving the dynamics from it. In physics, this is Noether's theorem applied as a design principle. In Hopfield networks, it is an energy function borrowed directly from condensed matter physics. The author's proposal is to apply the same move to emotional dynamics.

The observation that underwrites this is simple: emotional states feel like they have energy. Some states are high-energy and unstable — fight, flight, acute anxiety. Others are low-energy and stable — calm, regulated, present. Some are low-energy and *stuck* — freeze, dissociation, collapse. If these states have an energy ordering, there is likely an energy function. If there is an energy function, the dynamics can be derived from it. The author found this reasoning persuasive.

The Soma-Field Model

Emotions as a Persistent Wave Field

The foundational claim is this: emotions are not events. They are a *field* — a distributed, continuous quantity defined over the entire soma (body-mind system) at all times.

This is not a metaphor. It is the most accurate description the author can offer of his own experience. The emotional field is always there. It does not begin when a feeling becomes conscious and end when it subsides. It precedes conscious awareness and continues after it. What changes is not the field's existence but its local amplitude: whether, at a given moment, the field in a given mode exceeds the threshold required to surface as a named experience.

The field has two coupled components:

1. **The somatic wave $E_{\text{body}}(x, t)$:** distributed across the body as patterns of visceral sensation, muscle tone, proprioception, and autonomic state.
2. **The neural wave $E_{\text{neural}}(x, t)$:** distributed across the nervous system as patterns of cortical, subcortical, and peripheral activation.

These are not separate systems:

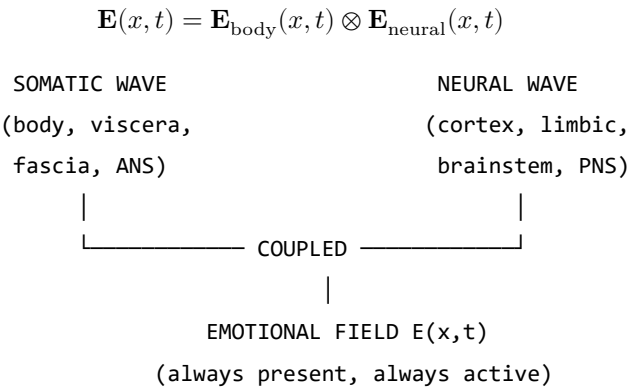


Figure 1. The Soma-Field: two coupled waves constituting a single unified emotional field.

The Perception Threshold

Not all field activity is consciously perceived. Each emotional mode i has a threshold T_i :

$$\text{Emotion } i \text{ is consciously perceived} \iff |\mathbf{E}_i(t)| > T_i$$

Below threshold: the emotion is sub-perceptual. It exists, it influences behaviour and physiology, but it does not surface as a named conscious feeling. This is the author’s most frequent relationship with his own emotional field — something is happening, below the line, shaping everything, unidentified.

Clinical Observation	Soma-Field Account
Field active but no named feeling	Sub-threshold: $ \mathbf{e}_i < T_i$
Sudden unexplained flood of emotion	Rapid threshold crossing after accumulation
Somatic signal without cognitive name	Threshold crossed in body component, not neural
Alexithymia	Elevated T_i — high energy required to cross
Hypervigilance / flooding	Lowered T_i — reduced threshold

Table 1. Clinical observations mapped onto the perception threshold model.

The author notes that all five rows in Table 1 are, in his clinical history, simultaneously applicable. This is, admittedly, a challenging configuration. It is also why this model was necessary.

A note on the intelligence quotients

McCulloch and Pitts built the mathematical brain in 1943. What they built — what every artificial neural network since has been — is the **IQ machine**: the neocortex, pattern recognition, sequence prediction, error minimisation. The field of AI has, for eighty years, been building increasingly sophisticated versions of this one component.

The soma-field adds what was missing: the **AQ machine**. AQ is to limbic dynamics as IQ is to cortical dynamics. Not a score; a formal

model of the system that produces it.

Quotient	System	First	Comment
		Formalised	
IQ	Neocortex: pattern recognition, prediction	McCulloch & Pitts, 1943	The entire AI industry
EQ	Limbic: valuation, attachment, empathy	Goleman, 1995	Described; not yet formally modelled
AQ	Soma-field: field-theoretic limbic dynamics	This paper, 2026	The formal model EQ has always needed
SQ	Relational field: dyadic and social resonance	Future work	Requires AQ as prerequisite

Table 3. The four intelligence quotients and their formal status.

The author observes — with a wryness he trusts the reader will share — that his IQ is in the column labelled 1943. His AQ is in the column he has just written. His EQ is what brought him to this desk in the first place.

A note on brane thickness

The threshold parameter T_i is not merely a number. The technical paper identifies it with the thickness of an extra dimension — the metaphorical ‘brane’ separating the limbic system from conscious awareness. Alexithymia is a thick brane: the field can be highly active and almost nothing crosses the threshold into named conscious experience. Hypervigilance

is a thin brane: everything crosses, simultaneously, at high amplitude. The author confirms personal experience of both states. He notes that neither is a character flaw; both are calibration states of a physical parameter in a system that was trying, with the information available, to keep him safe.

The Interaction of Emotional Modes

Multiple emotional modes are simultaneously active at all times. Their interactions are encoded in the **emotional coupling matrix** W , where W_{ij} is the influence of mode j on mode i :

- $W_{ij} > 0$: mode j amplifies mode i
- $W_{ij} < 0$: mode j suppresses mode i

The field evolves according to the energy gradient plus noise:

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}) + \eta(t)$$

The noise term $\eta(t)$ represents the continuous sub-perceptual fluctuations. The field is never still. This is not pathology; it is physics.

The Energy Landscape

The Hopfield Energy Function

$$H(\mathbf{e}) = -\frac{1}{2} \mathbf{e}^\top W \mathbf{e} - \theta \cdot \mathbf{e}$$

The field always moves toward lower H . The stable states of the system are the local minima of H — the attractor basins.

Attractor States: Fight, Flight, Freeze, and Regulated Calm

ENERGY

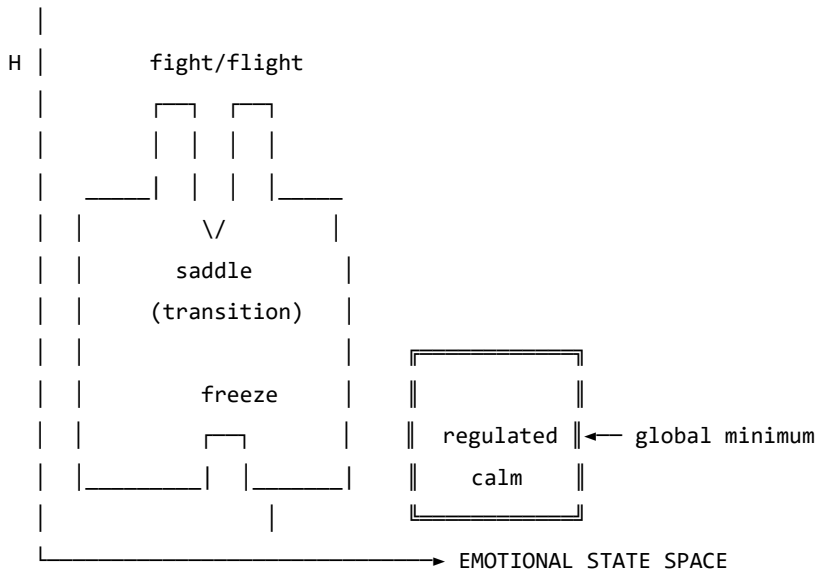


Figure 2. The emotional energy landscape. The freeze state is not high-energy — it is isolated. This distinction matters enormously. The author is aware of this from personal experience, over many years, and from the other side.

Attractor	Energy	Polyvagal Correlate	Clinical Presentation
Regulated Calm	Global minimum	Ventral vagal	Present, flexible, connected
Fight	High, unstable	Sympathetic	Agitation, urgency
Flight	Saddle point	Sympathetic	Anxiety, avoidance
Freeze	Deep, isolated	Dorsal vagal	Dissociation, numbness

Table 2. Attractor states and their polyvagal correlates.

The coupling matrix W is not merely a parameter. It is the *shape* of

the emotional manifold — a seven-dimensional space with the mathematical structure of a G manifold. Trauma does not adjust a dial on this space; it deforms the manifold itself. The therapist doing somatic work is, without needing to know this, doing differential geometry on the patient's G manifold: reshaping a seven-dimensional space by modifying the structure tensor. This is a precise technical statement. The author considers it a more honest account of what a skilled practitioner actually does than any narrative framework currently available. The practitioner is a geometer. The patient is a manifold that is learning to remember its own natural curvature.

The therapeutic and personal significance of the freeze attractor's structure cannot be overstated. It is not high-energy — it does not feel dramatic or intense. It is *isolated*: surrounded by energy barriers. Escape requires first *increasing* the field's energy before it can flow toward calm. This is counterintuitive from the outside and well-known from the inside.

Dissonance and Resolution

When two emotional modes are in an incompatible phase relationship, the field is far from equilibrium. This is felt as tension. The acoustic analogy is precise: just as two tones in a dissonant interval generate a beating, unstable interference pattern, two emotional modes in an incompatible configuration generate a gradient that drives toward resolution.

Dissonance is not pathological. It is the field's communication that resolution is available. The therapeutic process is guided voice-leading: finding the path that transforms the dissonant configuration into a consonant one. Avoidance keeps the field in dissonance. The energy minimum lies on the other side of the tension, not around it.

The author has spent considerable time attempting the route around it. He does not recommend it.

The Neurodivergent Field: ASD, ADHD, and C-PTSD as Operator Modifications

This section addresses the author's specific clinical picture. It is presented not as a case study but as a theoretical elaboration: three structural modifications to the standard Soma-Field dynamics, each defined by the operator it adds to the governing equations.

The key architectural principle — and the author considers this the most important contribution of this paper — is the following:

These conditions are not parameter settings. They are operator modifications.

A parameter change adjusts a coefficient within the existing equations. An operator modification changes the *form* of the equations themselves. The distinction is not semantic. It determines what kind of therapeutic intervention is possible and at what level it must operate.

Each condition is a functor that wraps the standard dynamics. The composed condition — ASD + ADHD + C-PTSD — is their composition. The composition does not commute; order matters; the joint presentation is structurally different from any of the individual conditions or from their sum.

Complex PTSD: Memory Kernel and Asymmetric Coupling

C-PTSD adds a **memory kernel**: past activations leave exponentially decaying echoes.

$$\dot{\mathbf{e}}(t) = -\nabla H(\mathbf{e}(t)) + \int_0^t K_{\text{trauma}}(t-s) \mathbf{e}(s) ds + \eta(t)$$

$$K_{\text{trauma}}(\tau) = \sum_k A_k e^{-\tau/\tau_k}$$

This is a damped oscillating kernel. The past does not vanish; it rings. Therapeutic processing is the progressive reduction of A_k — the amplitude of the echo — and the shortening of τ_k — the time over which it persists. The author notes that this description is a more accurate account of what trauma processing actually feels like, from the inside, than most of the narrative accounts available to him.

C-PTSD also breaks the symmetry of the coupling matrix W , admitting **limit cycles**: the oscillation between hyperarousal and shutdown that characterises the PTSD symptom cycle is, in this model, a limit cycle generated by the antisymmetric component of W . It is not a choice, a habit, or a failure of willpower. It is a topological consequence of an asymmetric coupling matrix.

ADHD: High Temperature, Low Damping, Pink Noise

ADHD modifies the **effective temperature** of the field:

$$\gamma_{\text{ADHD}} \dot{\mathbf{e}}(t) = -\nabla H + \sqrt{2D_{\text{ADHD}}} \xi_{1/f}(t)$$

with $\gamma_{\text{ADHD}} < \gamma_0$ (less damping) and $D_{\text{ADHD}} > D_0$ (more noise). The noise has $1/f$ spectral structure — long-range temporal correlations that produce the characteristic slow drift of attentional state.

The practical consequences: shallow attractor basins cannot hold the field at high temperature (distractibility). When a high-salience stimulus deepens a specific basin far beyond its baseline depth, the field falls in and is held (hyperfocus). The system is not broken. It is a different thermodynamic regime, with different costs and different affordances — including, at the right temperature, a capacity to explore the energy landscape at speed that a low-temperature system does not have.

The author considers this framing considerably more useful than “difficulty sustaining attention.”

Autism Spectrum Condition: Sparse Coupling and Modified Projection

ASC modifies the **projection kernels** and the **coupling matrix sparsity**.

The projection kernel $K_i(x)$ determines which somatic regions contribute to the i -th emotional mode. In ASC, some regions are over-weighted (sensory sensitivity) and others under-weighted (interoceptive under-registration). The named-feeling state vector is produced from a differently sampled version of the same somatic field.

The coupling matrix is sparser — fewer strong cross-modal connections — producing deeper individual attractor basins with higher inter-basin barriers. This is monotropism: the field settles deeply into one attractor at a time and requires disproportionate energy to transition. The author confirms that this is an accurate description of his attentional and emotional experience, and that it has both significant disadvantages (transitions are hard, unexpected context changes are physiologically costly) and significant advantages (depth of engagement, reliability of focus once established, resistance to shallow distractors).

The Composed Condition

$$\gamma_{\text{ADHD}} \dot{\mathbf{e}}(t) = -\nabla H_{\text{ASC}}(\mathbf{e}(t)) + \int_0^t K_{\text{trauma}}(t-s) \mathbf{e}(s) ds + \sqrt{2D_{\text{ADHD}}} \xi_{1/f}(t)$$

The interaction effects are non-trivial:

Interaction	Clinical Consequence
ADHD noise + C-PTSD limit cycles	Rapid oscillation between hyperarousal and shutdown; hard to titrate

Interaction	Clinical Consequence
ADHD noise + ASC deep basins	Long wind-up time; fast exit once perturbed from hyperfocus
C-PTSD echoes + ASC sparse coupling	Trauma triggers are specific, apparently disproportionate, difficult to anticipate
All three composed	Wide tolerance window required; regulation is genuinely structurally harder

Table 3. Interaction effects of composed neurodivergent modifiers.

The author wishes to note, for the record, that Table 3 is not a complaint. It is a description. These are the equations. The field is doing what the equations predict. Understanding this has been, in practice, more useful than most of the alternative framings on offer.

The Soma-Field Instrument

Rationale

The emotional field is normally invisible to its host. It operates below the threshold of conscious awareness, shaping behaviour and physiology without being available for reflection. The author found this situation suboptimal and designed an instrument to address it.

The instrument externalises the emotional field — renders it as sound, image, and signal — so that it becomes available as an object of attention. This is a therapeutic biofeedback instrument. It is also, unavoidably, a musical instrument. The author considers these compatible.

Design

A MIDI controller with 16 rotary knobs. Eight emotional dimensions. Two knobs per dimension — one for the somatic component, one for the neural/cognitive component. The act of setting a knob is the act of reporting an emotional state: it is the quantum measurement, the collapse of the distributed field onto a specific coordinate.

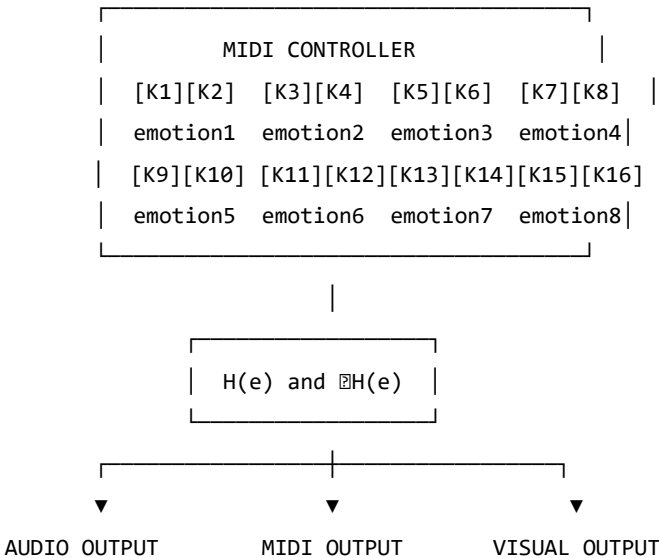


Figure 3. The Soma-Field Instrument.

The Feedback Loop

The instrument creates a closed feedback loop: the person expresses a state, the system reflects it back as sound and image, the person responds. The system does not tell the user what they are feeling. It shows them what the field looks like when they report what they are feeling. The difference is significant.

Pluggable Emotion Models

No single emotion model is assumed. The coupling matrix W is loaded from a configuration file. Plutchik, Ekman, the valence-arousal-

dominance dimensional model, and custom user-defined models are available as defaults. The author's own W has been refined over time and is not identical to any standard model. This is, on reflection, unsurprising.

Clinical Implications

Assessment

The model suggests asking not “What emotion do you feel?” but “What is present in the body right now, even if it cannot be named?” This aligns with Focussing-oriented and sensorimotor approaches, and is considerably more productive, in the author's experience, for anyone whose T_i values are elevated or whose somatic-to-neural projection is modified.

Intervention

The energy function provides formal grounding for titration, pendulation, somatic resourcing, and felt-sense work. In each case, the therapeutic action can be described as: adding energy to approach a frozen state, establishing a stable low-energy region, or attending to sub-threshold field activity in a supported context.

Psychoeducation

“Your emotions are like waves — they are always there, even when you cannot feel them, and they are always moving.”

This sentence is both clinically useful and technically accurate. The author has found it more useful than most alternative formulations, including several that were provided to him by qualified practitioners. He offers it here as a contribution to the field.

Neurodivergent Profiles as Structural Realities

The most important clinical implication of Section 6 is this: for people with ASD, ADHD, and C-PTSD, the challenge of emotional regulation is not a motivational or characterological failure. It is a structural consequence of specific operator modifications to the dynamics. The composed modifier produces a field that is genuinely harder to regulate — not by a small margin, not as a matter of subjective experience, but mathematically, as a consequence of higher noise temperature, memory echoes, sparse coupling topology, and the possibility of limit cycles.

Knowing this does not solve the problem. It does, however, locate it correctly. The author has found that locating a problem correctly is a necessary precondition for solving it, and that a great deal of time and distress can be saved by not attempting to solve problems that are located in the wrong place.

Limitations and Future Directions

The model is theoretical and requires empirical validation. Its QFT analogies are structural rather than ontological. The coupling matrix W is idealized as fixed when it is in practice dynamic. The acoustic analogy is a hypothesis.

The author also acknowledges a methodological limitation: this paper is written by someone who is simultaneously the theorist and the primary data source. This is either a significant advantage (direct access), a significant limitation (potential confirmation bias), or both. The author suspects both.

What is needed: empirical work with physiological sensors, user studies with the instrument, collaboration with practitioners, and independent theoretical review. The author is, by training and disposition, an applied physicist — an engineer with a tolerance for abstraction. The clinical refinement of this model will require people with different skills, and the

author welcomes their involvement, provided they read the appendices.

Conclusion

The wave is always there. This is not a metaphor; it is a description of how the emotional field actually behaves, as far as the author can determine from the inside. Therapy — and the instrument described in this paper — is the practice of learning to hear it: to extend awareness downward, below the threshold, into the field's continuous activity, and to make that activity available as information rather than overwhelming noise.

The Soma-Field Model is offered as a tool for this practice. It was built because it was needed. It uses the best mathematical tools available for describing distributed, dynamic, energy-minimising systems, because those tools are, in the author's assessment, appropriate to the problem.

The author is aware that this is an unusual paper. A formally trained physicist with three neurodivergent conditions developing a quantum-field-inspired model of his own emotional dynamics and presenting it as a contribution to clinical psychology is not, strictly speaking, the standard academic pipeline. The author does not find this troubling. The standard academic pipeline has had some time to address the problem and has not yet done so to his satisfaction.

He therefore took the matter in hand.

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